Prevention, treatment and lifecycle health types

Paper skeleton — first full draft, bullet form

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1 Introduction

[**TODO:** Deferred.]

1.1 Related literature

- Include a discussion here of the earnings dynamics literature and how they use the covariance matrix
- Also a discussion of health measurement in economics.
- On the stochastic specification we are close to Hosseini et al. (2022), who model log frailty in the PSID as a fixed effect plus an AR(1) plus a transitory shock, with persistence 0.991. Our log(frailty + 1/31) on UKHLS reproduces their hump-shaped variance profile, and it behaves like θ throughout (Section 3.2).

2 Conceptual framework

[Condensed from `conceptual_framework.tex`, which holds every derivation, proof and simulated figure. Result and equation numbering here is local; cross-reference the note for proofs.]

2.1 Setup: deterministic type and stochastic noise

- Health of person i of latent type j at age a (age counted from a base age, not birth):

$$h_{i,a} = \bar{h}_{j,a} + u_{i,a}, \qquad \bar{h}_{j,a} \equiv H_j + \sum_{a'=1}^{a} d_{j,a'}. \tag{1}$$

  $(H_j, d_{j,a})$ is the *deterministic* component: the type's level at the base age and its expected change at each age. $u_{i,a}$ is the *stochastic* component, mean zero within type, otherwise unrestricted (could be transitory, persistent, or measurement error).
- Three kinds of information in a health history:
  1. what *stays* (the level, including permanent shocks that have landed),
  2. what is *in the moment* (transitory variation and measurement error),
  3. how one is *evolving* ($d_{j,a}$, the only component that predicts future *changes*).

Prevention targets the third; the first two are what is paid for, as treatment, once realised.

- Only what is carried from one wave to the next produces covariance between waves: the three components leave different traces on the covariance matrix, which is the basis for our identification.
- Two-by-two matrix of possibilities: dispersion is deterministic (across types) or stochastic (within type), and actionable or not. Deterministic and actionable is *targetable* prevention; stochastic and actionable is untargeted prevention, or treatment once the shock has arrived. Predictability of levels $\neq$ predictability of changes.
- Health types are thus the object of interest: they are the support of (H, d), and clustering estimates it. Between/within variance maps to type/individual. Result 2 below is the reason: the return to prevention for a type is a function of that type's mean path alone, so a policy needs the type shares and each type's path, which is what Section 4 estimates.
- Link to other life course: With the base age at birth, the life-course accounts are second moments: critical period = $\mathrm{Var}(H)$, accumulation = $\mathrm{Var}(d)$, pathway = $\mathrm{Cov}(H, d)$; at a later base age the triple's evolution with the base age separates them.
- Throughout this section, we will use a linear illustration (constant type slopes, $d_{j,a} = d_j$): a random-coefficients growth model with V_H, V_d, C_{Hd} and noise variances v_a,

$$\begin{aligned} \bar{h}_a &= \bar{H} + a\bar{d}, \\ \mathrm{Var}(h_a) &= V_H + 2aC_{Hd} + a^2 V_d + v_a, \\ \mathrm{Cov}(h_s, h_t) &= V_H + (s+t)C_{Hd} + st\, V_d + \mathrm{Cov}(u_s, u_t). \end{aligned} \tag{2}$$

- A key issue that will come up again in relation to costs and when we bring to the data is what the units of health are. The covariance matrix treats health as cardinal, so transformations of a health index that are rank neutral can change the covariance matrix. We report the unbounded latent scale θ in the main text; its bounded expected-score twin h and two frailty indices are in the appendix, and these are all discussed in more detail in Section 3.

2.2 Policy: the treatment bill, and prevention as avoiding it

- Cost of poor health $c(h) = (1-h)^2$, full health normalised to one. The quadratic is for illustration: it is the second-order approximation to any smooth increasing convex cost, and it makes the first two moments of health sufficient for every policy object below. What matters for prevention is not the quadratic itself but marginal cost times the decline prevention holds back ((7) below). The quadratic is also not far from the data: in pounds, cost is close to quadratic in poor health (Section 3, Figure 1), with a cubic term that only steepens the sickest end. The prevention exercises in Section 5 price with the cost curve estimated in pounds on θ rather than the quadratic.
- Population cost at age a: mean gap plus between-type plus within-type variance,

$$\mathbb{E}[c(h_{i,a})] = (1-\bar{h}_a)^2 + \mathrm{Var}_j(\bar{h}_{j,a}) + \textstyle\sum_j \pi_j \mathrm{Var}(u_{i,a} \mid j). \tag{3}$$

- Cost is treatment: $c(h)$ is what the health system spends on a person in health h, and the cost index of Section 3 measures exactly that (utilisation at unit costs). The object a policy acts on is the **treatment bill still to come** from age r,

$$C(r) = \sum_{a \geq r} \mathbb{E}[c(h_{i,a})] = \sum_{a \geq r} \Big[\underbrace{c(\bar{h}_a)}_{\text{mean path}} + \underbrace{\textstyle\sum_j \pi_{j,a} c(\bar{h}_{j,a}) - c(\bar{h}_a)}_{\text{type differences}} + \underbrace{\mathbb{E}[c(h_{i,a})] - \textstyle\sum_j \pi_{j,a} c(\bar{h}_{j,a})}_{\text{realised deviations}} \Big], \tag{4}$$

which for the quadratic is (3) summed over ages: the mean gap, the between-type variance and the within-type variance. The third part is paid as treatment but is not preventable: it is the in-the-moment variation, including the transitory shock.

- One instrument. **Prevention** is ex ante and acts on change: from age r at intensity μ it scales every future mean change of a type by $(1-\mu)$, moving type j's path to

$$\bar{h}^{\mu}_{j,a} = \bar{h}_{j,a} + \mu\,(\bar{h}_{j,r-1} - \bar{h}_{j,a}) \quad \text{for } a \geq r,$$

and leaving $u_{i,a}$ untouched, so health gained is μ times the type's preventable decline $\bar{h}_{j,r-1} - \bar{h}_{j,a}$, and its return is the treatment cost that avoids.

Result 1 (The treatment bill; note Result 1)**.**

$$C(r) = \sum_{a \geq r} \big[(1-\bar{h}_a)^2 + \mathrm{Var}(h_{i,a})\big]$$

under the quadratic. Cross-sectional moments only; no identification problem. (The note's "return to treatment" $T(r)$ is $2C(r)$: restoring a share τ of every realised gap is worth τ times twice the bill.)

Result 2 (Return to prevention by type; note Results 2 and 3)**.** *Under the quadratic, the return to prevention from age r for type j, per unit of μ, is*

$$P_j(r) = 2\sum_{a \geq r} \underbrace{(1-\bar{h}_{j,a})}_{\text{gap at } a} \underbrace{(\bar{h}_{j,r-1} - \bar{h}_{j,a})}_{\text{preventable decline by } a} \equiv \sum_{a \geq r} s_j(a \mid r). \tag{5}$$

Both factors are functions of the type's mean path alone: nothing within type enters. A type pays back only where it is both *in poor health and still declining. A sick but flat type is a treatment case; a declining but healthy type pays back only once its decline has opened a gap, which is why the per-age saving $s_j(a \mid r)$ rises with age.*

Corollary 1 (The uniform policy)**.** *Giving every type the same μ returns $P(r) = \sum_j \pi_j P_j(r)$, and the law of total covariance applied to the product in* (5) *splits it as*

$$P(r) = 2\sum_{a \geq r} \Big[\underbrace{(1-\bar{h}_a)(\bar{h}_{r-1} - \bar{h}_a)}_{\text{mean channel}} + \underbrace{\mathrm{Cov}_j\big(1-\bar{h}_{j,a},\, \bar{h}_{j,r-1} - \bar{h}_{j,a}\big)}_{\text{between-type channel}}\Big], \tag{6}$$

the between-type channel splitting further as $\mathrm{Cov}_j(1-H_j,\, \bar{h}_{j,r-1} - \bar{h}_{j,a}) + \mathrm{Cov}_j(H_j - \bar{h}_{j,r-1},\, \bar{h}_{j,r-1} - \bar{h}_{j,a}) + \mathrm{Var}_j(\bar{h}_{j,r-1} - \bar{h}_{j,a})$*, the pathway, persistence and accumulation margins. In the data the mean channel is most of the uniform return (three-fifths from 25, nine-tenths from 70; Section 5), so the cross-section prices uniform prevention nearly on its own; what the types add is the dispersion of $P_j(r)$ across them, which is what targeting buys (Result 3).*

- **What matters is marginal cost times decline.** For a general increasing convex cost, to first order in μ,

$$P_j(r) = \sum_{a \geq r} G_{j,a}\,(\bar{h}_{j,r-1} - \bar{h}_{j,a}), \qquad G_{j,a} \equiv \mathbb{E}[-c'(h_{i,a}) \mid j], \tag{7}$$

the marginal cost of a unit of health to type j at age a times the decline prevention holds back by then. The quadratic's gap is this marginal cost, since $-c'(h) = 2(1-h)$. The uniform return splits as in Corollary 1 with G in place of the gap: $P(r) = \sum_{a \geq r} \big[\bar{G}_a\,\bar{d}_a + \mathrm{Cov}_j(G_{j,a}, d_{j,a})\big]$, with $d_{j,a} = \bar{h}_{j,r-1} - \bar{h}_{j,a}$. Two features are specific to the quadratic: within-type dispersion drops out exactly (for a cubic it enters exactly, $G_{j,a} = -c'(\mathbb{E}[h_{i,a} \mid j]) - \frac{1}{2}c'''\mathrm{Var}(h_{i,a} \mid j)$, and on the estimated curve the dispersion term is 8–10% of the uniform return), and the between-type channel splits into the pathway, persistence and accumulation margins.

- **Why the evolution of the mean by type.** Had prevention instead lifted the path by a constant δ from r, the return would be $2\delta \sum_{a \geq r}(1 - \bar{h}_{j,a})$: gap only, priced by levels, and a cross-section would do. The product form of (5) is specific to prevention that acts on the *change*, and under it the object a policy needs is $(\pi_j, \bar{h}_{j,\cdot})$, the support of the types and each type's path, not the covariance structure. The covariance structure matters for two other things: separating the deterministic path from the noise (Section 2.3), and recovering a person's type when it is not observed (the posterior, Section 5).
- For a general increasing convex cost $c(\cdot)$ the bill is (4) as written and the return keeps the form in Result 2 with the derivative as the weight; the quadratic reproduces Result 1 and (5). $P(r)/C(r)$ sets the return against the remaining bill. $P(r)$ is a first-order return, the saving per unit of μ as μ rises from zero, so on a convex curve it overstates what a larger dose saves and $P(r)/C(r)$ can exceed one. For a cubic cost, the curve Section 5 estimates, the expansion stops at the third term and the saving at any μ is exact: with $d_{j,a} = \bar{h}_{j,r-1} - \bar{h}_{j,a}$,

$$\begin{aligned} S_j(r;\mu) &\equiv \sum_{a \geq r} \mathbb{E}\big[c(h_{i,a}) - c(h_{i,a} + \mu d_{j,a}) \mid j\big] \\ &= \mu P_j(r) - \frac{\mu^2}{2} \sum_{a \geq r} c''\big(\mathbb{E}[h_{i,a} \mid j]\big)\, d_{j,a}^2 - \frac{\mu^3}{6}\, c''' \sum_{a \geq r} d_{j,a}^3, \end{aligned} \tag{8}$$

  since c'' is linear and c''' a constant. Section 5 reports $S(r) \equiv \sum_j \pi_j S_j(r;1)$, the saving from holding back the whole decline, beside $P(r)$; unlike $P(r)$, it cannot exceed the bill.
- Prevention averts future gaps but must be assigned ex ante; the uniform policy pays the population-average preventable decline and its between-type channel is bounded by deterministic heterogeneity. What it can reach at most is the first two parts of (4): the realised deviations are treated, not prevented.
- Measurement error sits in the third part of the bill and in no prevention return; Case 4 below removes it from the type dynamics.

Result 3 (Targeting; note Results 5–7 and Lemma C.1)**.**

- *Any allocation $\mu(z)$ with the same average intensity saves* $\mathrm{Cov}(\mu(z), \mathbb{E}[P_i \mid z])$ *over the uniform policy, at most* $\mathrm{sd}(\mu)\,\mathrm{sd}(\mathbb{E}[P_i \mid z])$*: a signal is worth the return dispersion it reveals.*
- *We can consider two forms of targetting:*
- *Targeting on initial health $h_{i,0}$ has a gap channel and a compounding channel, both cells of the first row of the covariance matrix, attenuated by the level/noise share λ_0.*
- *Targeting on type* $\max_j P_j(r) - \sum_j \pi_j P_j(r)$.

2.3 Identification

- Every observed covariance is a deterministic (i.e. between-type) part plus a noise part, $\mathrm{Cov}(h_s, h_t) = \mathrm{Cov}_j(\bar{h}_{j,s}, \bar{h}_{j,t}) + \mathrm{Cov}(u_{i,s}, u_{i,t})$. The returns need the deterministic part but we only observe the sum.
- Start with some broad assumptions:
  - A1 window linearity ($d_{j,a} = d_j$ over the waves used),
  - A2 orthogonality of the noise to (H, d),
  - A3 a noise class that puts some restrictions on the covariance matrix

- In the linear case
$$\mathrm{Cov}(h_s, h_t) = V_H + (s+t)C_{Hd} + st\,V_d + \mathrm{Cov}(u_s, u_t). \tag{9}$$
which means that on the diagonal $\mathrm{Var}(h_a) = V_H + 2aC_{Hd} + a^2V_d + v_a$
- If the noise variances v_a are unrestricted the variance profile can never rule out pure noise.
- With stationary noise, $\mathrm{Var}(h_a)$ can identify (C_{Hd}, V_d) but never split V_H from σ_u^2. Identification thus needs restrictions on how the noise variance moves with age and at least some off-diagonals.

Terminology. We call the noise class of Proposition 1, a persistent AR(1) component plus a one-period shock, the *persistent–transitory* specification, and the special case with no persistent component *transitory only*. This is a standard specification, under a standard name, in the literature on earnings and income dynamics (MaCurdy, 1982; Abowd and Card, 1989; Meghir and Pistaferri, 2004; Blundell et al., 2008), and health economists use the same language: French and Jones (2004) and De Nardi et al. (2010) split medical spending into a persistent AR(1) component and a transitory one, and Hosseini et al. (2022) model frailty as an AR(1) plus a transitory shock. The transitory shock can be thought of as including both pure noise, that is measurement error, and an *episodic* shock: a genuine change in health that is gone by the next period. The covariance matrix cannot tell the two apart, since neither is carried into the next period.

Proposition 1 (Identification within the persistent–transitory class; note Proposition 1)**.**
Assume A1–A2 and $\mathrm{Cov}(u_s, u_t) = \rho^{t-s}v_s + \mathbf{1}\{s=t\}\sigma^2_{\varepsilon,s}$ *for* $s \le t$, $\rho \in [0,1)$*: a carried component with memory* ρ *and variances* v_s*, plus a one-period transitory shock. Then*
(i) five waves identify V_H, C_{Hd}, V_d, ρ *and both noise variances at the waves used;*
(ii) four waves are sufficient without the transitory shock (Case 3) or with both noise variances stationary (Case 4);
(iii) three waves are sufficient without the transitory shock and with stationary variance, for any ρ *(Cases 1–2);*
(iv) two waves are sufficient if in addition $C_{Hd} = 0$ *and the noise is memoryless.*

- The forces underlying , one per region of the matrix. This is the precise content of I(i) and I(ii):

I(i) **Diagonal** $V(a)$: every component adds variance. Stationary noise cancels in differences and leaves C_{Hd} (slope) and V_d (curvature). Fails when v_a drifts.

I(ii) **First row** $C_0(a)$: the deterministic part is the line $V_H + aC_{Hd}$, carried noise a decay $\rho^a v_0$; a line cannot mimic a decay, which splits the level from the noise and measures the memory; the row's slope beyond the noise is C_{Hd} itself, locally $C_{Hd}(s)$ for the row anchored at s. A transitory shock is invisible to the row and is read as the anchor's excess over the row's back-extrapolation. Limit: $\rho \to 1$ merges permanent shocks with the level.

I(iii) **Interior**: $st\,V_d$ needs both indices to move; delivers V_d when the diagonal cannot be read, overidentifying tests otherwise.

- *[There are a whole bunch of cases in the conceptual framework note, but we certainly don't want to go through all of them here, so we should think about what we want to include here. At least one or two are probably useful to help illustrate the identification (but maybe they can also wait until Section 3... In the illustrative cases on real data, the sign of the small* C_{Hd} *does depends on the noise treatment (although the gradient from positive to negative is relatively consistent.]*

- The linearity assumption is local, so A1 holds only over a window. But given this local linearity, $C_{Hd}(a) \equiv \mathrm{Cov}_j(\bar{h}_{j,a}, d_j)$ and so the deterministic variance increases by $C_{Hd}(a) + C_{Hd}(a+1)$ per year: a variance profile that flattens requires $C_{Hd}(a)$ to change sign, which the global linear model cannot produce. Figure 8 is the empirical counterpart: types refitted on age windows recover the

lifecycle classes once the window reaches back to 40, and lose the declining type when it does not, because a slope needs the years over which it acts.

- The between-type channel of $P(r)$ is the exposure-weighted sum $\sum_{a \geq r}(a - r + 1)C_{Hd}(a)$ and so depends crucially on C_{Hd}.
- Everything can be represented as person-level OLS of h on $(1, a)$, except for the noise correction which is case-specific (note Appendix E).

What the covariance surface prices without types. In the linear case of Equation 2, $\bar{h}_{j,a} = H_j + a\, d_j$, the between-type channel of the uniform return (Corollary 1) is $(a - r + 1)(C_{Hd} + aV_d)$, so

$$P(r) = 2 \sum_{a \geq r} \big[(1 - \bar{h}_a)(\bar{h}_{r-1} - \bar{h}_a) + (a - r + 1)\, C_{Hd} + a(a - r + 1)\, V_d\big]. \tag{10}$$

The uniform return is therefore priced by the surface's moments alone, without recovering the types, and each object reads off one region of the surface:

Policy object	Moments it needs
$C(r)$, and the mean channel of $P(r)$	the mean path $\bar{h}_a$ and the cross-sectional variance $\mathrm{Var}(h_a)$: Figure 2 panels (a) and (b)
the accumulation margin of $P(r)$; the timing profile $r \mapsto P(r)$	V_d (with C_{Hd}): the curvature of the surface
the pathway margin; targeting on a level	C_{Hd}, locally $C_{Hd}(a) \equiv \mathrm{Cov}_j(\bar{h}_{j,a}, d_j)$, whose sign says diverging or converging: Figure 2 panel (c)
targeting on a type	$P_j(r)$ across types, i.e. the paths of Section 4

Read through (10), Figure 2 says how the return moves with the start age: an accelerating mean raises the mean channel for later starts; a V_d that falls with age puts the accumulation margin early; a $C_{Hd}(a)$ that changes over time works through the pathway margin, and its sign is what targeting on initial health turns on. The between-type channel this prices is also the moments-only check on the clustering: it should match the channel the estimated paths of Section 4 imply.

- [**TODO:** We should decide which noise class to focus on. The data generally pushes us towards a transitory shock and pretty high ρ (maybe the high rho is also making up for some individual fixed effect though?). This would be Case 4 getting close to its $\rho \to 1$ limit. We do have a random walk case in the conceptual note (Proposition F.1, random walk plus a transitory shock), but here the diagonal becomes useless which sort of undercuts our identification forces...]
- [**TODO:** For the covariance surface, a permanent shock and a type level are the same thing, all bundled into H. The clustering models don't really have this as they require everyone in the cluster to have transitory noise around a shared intercept (related to your point about insufficient power to identify all clusters)... Is the AR(1) a "noise within class", or is it part of "what stays"?]
- [**TODO:** Should maybe already mention selection here as every row conditions on being observed later (and return to this with the multidimensional model).]

3 Data and motivating descriptives

3.1 Data and the health measure

Data:

- Understanding Society (UKHLS), waves 1–15, 2009–2024 (University of Essex, Institute for Social and Economic Research, 2025); adults aged 20–90. The SF-12 is asked every wave; the impairment questions every wave; the choronic conditions are asked at a person's first interview and updated new diagnoses (but do not register if the condition is no longer present).
- We choose these variables as they are consistently reported over the UKHLS and they all focus on physical health (mental health displays quite different lifecycle dynamics so we look at that separately in Section 4.3).[1]
- 498,424 person-waves on 84,759 people aged 20–90 are complete on the SF-12 physical items. Of these, 387,599 person-waves on 69,958 people (5.5 per person) are also complete on the condition inventory, and the graded-response model is estimated and scored on that sample. The lifecycle analysis restricts further to people with at least three observed ages between 20 and 89, one row per person-age: 334,194 person-ages on 38,963 people.[2]

[**TODO:** I have (obviously with Claude's help) replicated most of this in the PSID as well, so we can look at those as well. I will send this as a separate document. Generally most things carry over - θ looks very similar on both, but h has variance slowing between 60 and 70 but then accelerating after 70...]

Table 1 shows the underlying health variables that we use for our main health measure. The first four are the SF-12 physical compoments of the commonly used PCS metric; the next three are grouped counts the UKHLS impairment areas that relate to physical limitations; the last counts diagnoses of chronic conditions. None of the items is tied to employment: the role-limitation items ask about problems "with your work or other regular daily activities", and the others make no reference to work. Two checks confirm that the measure is not picking up employment status.[3]

[1] We test for local dependence with Yen's Q_3 and find no evidence that any of these variables are principally capturing a second factor.

[2] UKHLS interview dates within a wave spread over up to two years, so a person can be interviewed in two consecutive waves at the same integer age. 20,114 such person-age rows are collapsed to one by averaging the scores and keeping the earlier wave. The K-means types and the Bayesian mixtures are fitted on exactly these rows.

[3] First, at the state pension age of 65 every item improves by about 0.05 pooled standard deviations (a linear trend either side of 65 on ages 58–72, person-clustered t between 2 and 5), and the role-limitation testlet's jump, 0.047, sits in the middle of the range, so retirement does not move the work-related items specifically. Second, at ages 25–64 the employed cost about a third less than the non-employed at every level of health, but the shape of the cost–health curve is the same: in a quadratic in θ interacted with an employed indicator, the interactions with the slope and the curvature have $|t| \leq 2.8$ against everyone else and $|t| \leq 1.7$ once the long-term sick are excluded from the comparison. We find no evidence that retirement changes the way people answer these questions.

Table 1: Underlying health variables

Item	What it records	Categories
GH	self-rated general health	5
PF	moderate activities and climbing stairs	5
RP	accomplished less, limited in kind of work or other activity	9
BP	pain interference with normal activity	5
LIM_FL	functional limitations: mobility, lifting, dexterity, co-ordination	5
LIM_SELF	self-care: personal care, continence	3
LIM_SENS	sensory: hearing, sight	3
COND	ever-diagnosed physical conditions, 0–5 and 6 or more	7

[Maybe here (or back in the introduction) we can already discuss that there is no single cardinal health measure and so, arguably, any monotonic transformation of a health measure is a valid health measure [see Cunha and Heckman (2008) on test scores; Cunha et al (2021) on "Cardinality"; Bond and Lang (2013, 2019) for test scores and happiness scales]. However, if we want to use the covariance matrix to identify dynamics, then the scale does matter as these covariance are cardinal. So what we would need is some cardinal anchor and fortunately our framework gives us two obvious candidates: the mapping to costs and the mapping to interventions. Given that the focus of this paper is the potential returns to prevention rather than an evaluation of specific interventions it probably makes sense to focus more on the costs (and we can construct a proxy measure for these from the data on utilisation). If we had constructed the framework around mortality or QALYs or employment/productivity then this would imply an alternative anchoring.]

To construct a continuous health metric from these underlying variables we use a graded-response model Samejima (1969) - a one-factor model for ordinal indicators (with continuous and unbounded indicators this would be equivalent to the linear factor models common in economics, e.g. Cunha et al., 2010; Attanasio et al., 2020; Agostinelli and Wiswall, 2025). Let $Y_j \in 1, ..., k_j$ be the response to item j, which has k_j ordered categories. Given the respondent's underlying health state is θ, the probability of answering at or above category c is logistic in θ,

$$P(Y_j \geq c \mid \theta) \;=\; \frac{1}{1 + \exp\{-a_j(\theta - b_{j,c-1})\}} \tag{11}$$

where a_j and the $k_j - 1$ thresholds $b_{j1} < \cdots < b_{j,k_j-1}$ are estimated item-specific parameters. The latent variable θ thus capture the respondent's underlying health that determines their expected answers across all items. The item parameters are estimated by marginal maximum likelihood. Latent health is assumed standard normal in the pooled sample, which fixes the scale; θ is integrated out of the likelihood of each answer pattern over a grid of 121 points, and the discriminations and thresholds are chosen to maximise the resulting likelihood by EM, over the 28,360 distinct answer patterns. Scoring is a second step with the item parameters held fixed: each person-wave's posterior over θ is the standard normal prior times the likelihood of its eight answers, and the reported θ is the mean of that posterior, with its standard deviation kept alongside. Each person-wave is scored on its own, with the same prior, so a respondent's other waves and age do not inform their score.

[we never take the posterior uncertainty around θ into the clustering models, which it might be useful to do (and I think relatively straightforward).]

The model allows thresholds the be unevenly spaced and for different items to separate observations in different parts of the distribution. In our case PF, RP and LIM_FL discriminate the most (i.e. have the highest values of a_j) and their thresholds sit between -2.5 and 0, so they separates people in the

unhealthy half of the distribution. GH and BP discriminate less but their thresholds are further up in the distribution so they separate out the healthy half. LIM_SENS and COND have the lowest values of a_j and their thresholds go down below -4.0 so they prevent the very sick from piling up at a floor.

The underlying health factor θ is designed to be continuous, unbounded and distributed $N(0,1)$. However, we can map it back into an expected share of the measure's total scale, i.e. summing overall all J items as a share of the maximum possible score(34). We define this as

$$h(\theta) \;=\; \frac{\sum_{j=1}^{J}\sum_{c=2}^{k_j} P(Y_j \geq c \mid \theta)}{\sum_{j=1}^{J}(k_j-1)}, \qquad h_i = h(\hat{\theta}_i), \quad \hat{\theta}_i = E[\theta \mid \mathbf{y}_i]. \tag{12}$$

The map is monotone, so θ and h order people identically, but not linear so the variance profile and cost function differ across the two. In practice the $h()$ transformation compacts the health scores at the healthy end of the distribution and spreads them out more at the unhealthy end. Figure B.1 compares the measures and illustrates how they relate to one another.

The h score is close to a weighted sum of the coded answers: regressing h on the standardised codes gives $R^2 = 0.993$. The parallel is not perfect for two reasons: the uneven gaps between the categories of an ordered item; and an item's best and worst categories are open-ended so they only tell us that a person lies beyond the last threshold.[4] A linear one-factor model fitted to the same eight codes, treated as continuous, gives a score with a Pearson correlation of 0.995 with h and 0.998 with that weighted sum, so on the $[0,1]$ scale the graded-response model's practical content is the weighting rather than the functional form.

In addition to θ and h we construct a Rockwood-style frailty index (Mitnitski et al., 2001; Searle et al., 2008): the share of 31 equal-weight deficits a person has, namely the six SF-12 physical items graded 0 to 1 by response step, each of the eight physical impairment areas, and each of the 17 ever-diagnosed conditions (clinical depression among them), so that 0 is no deficit and higher is frailer (Appendix B.3). 7% of person-waves have none. Following Hosseini et al. (2022), who model the log of frailty, we also report $\log(\text{frailty} + 1/31)$: adding one deficit's worth keeps the zeros in the sample, where Hosseini et al. (2022) drop them into a separate probit.[5] Both indices rank people much as θ and h do (Spearman 0.92 with each), and the two pairs line up: h behaves like the frailty index and θ like its log (Section 3.2).

Relation of our health metric to the economics literature.

- Why this measure - the underlying data are ordinal and bounded, so our model respects this.
- Factor methods are common in health economics: Poterba et al. (2013, 2017) take the first principal component of 27 HRS items, and Blundell et al. (2023) of the three subjective health measures common to HRS and ELSA.
- In the skill-formation literature, Cunha et al. (2010) estimate a closely related dynamic factor model, on continuous test scores that enter linearly in the log of the latent skill.
- Item-response models are standard in health measurement: the PROMIS item banks, now widely used in health research and clinical care, were calibrated with the graded-response model (Cella et al., 2010), including the 124-item physical function bank (Rose et al., 2014).

[4] The latent scale θ departs is somewhat further from a weighted sun: regressed on the same codes gives $R^2 = 0.925$. Most of the gap comes from the model not scoring an item's categories as equally spaced: letting each category carry its own score raises the R^2 to 0.979. The rest comes from the items discriminating in different parts of the scale, so that a step on one item counts for more where few others still move.

[5] Adding half the smallest positive value, 1/248, instead puts the zeros so far below the next group that they drive the variance at young ages; with 1/31 the variance profile is smooth.

- Item-response models themselves are rare in health economics: Graeber (2023) scores latent health from five SOEP items with the graded-response model, Wallace et al. (2017) build an item-response index from 21 HRS items, and Meijer et al. (2011) estimate the same ordinal factor model on SHARE without calling it one. They are common in the economics of education, for scaling and linking test scores (Das and Hammer, 2005; Das and Zajonc, 2010; Singh, 2015; Muralidharan et al., 2019; Jacob and Rothstein, 2016), and in the measurement of poverty and deprivation (Moffitt and Ribar, 2016; Corsini and Martelli, 2020; Farcomeni et al., 2025; Davis et al., 2026).
- The closest work to ours in health dynamics uses five-category self-assessed health kept categorical and usually collapsed: to three states by De Nardi et al. (2025) and Capatina (2015), to a bad-health indicator by Halliday (2008), and kept at five by Contoyannis et al. (2004). But this would really limits our study of dynamics and in particular the timing and targetting of prevention.
- In our case, the choice of methodology relative to a more familiar principal component or linear factor model is innocuous, if we focus on h - a linear factor model estimated on the same data has a Pearson correlation of 0.995 (Spearman 0.992) with h.
- There is a precedent for modelling the log of a bounded health index, the log frailty of Hosseini et al. (2022); on our data $\log(\text{frailty} + 1/31)$ has a variance profile by age like θ's, and the frailty index in levels one like h's. Table B.3 shows the correlation with alternative metrics (UKHLS PCS, MCS, raw chronic count). All of these have theoretical disadvantages over our measure, but PCS is closest.

[I think overall conclusion here is that there is no one way to measure health, but for dynamics we kind of need cardinality which we can maybe pin down with costs.]

As our preferred case, we report θ in the main text. It is the scale the measurement model is defined on; it is unbounded, so the Gaussian dynamic models of Section 4 fit it without a ceiling (on h the healthiest class collapses under the persistent–transitory specification); and its cost curve in pounds is convex. Appendix I repeats the main-text figures on h, the frailty index and log frailty, and the text flags where the measure changes a conclusion.

In addition to our health metric, we also construct a proxy for healthcare cost based on the healthcare utilisation information in UKHLS.

- Per-person-wave estimate of annual NHS cost from in-patient, out-patient and GP contacts, waves 7–15, in 2024/25 prices. UKHLS records the nights spent in hospital as an in-patient over the past year, and in bands the number of GP consultations and hospital out-patient attendances (none, 1–2, 3–5, 6–10, more than 10, which we score 0, 1.5, 4, 8 and 15). Stays for childbirth are left out.
- The construct is

$$\widehat{C}_{it} \;=\; \underbrace{\mathrm{adm}_{it}\big[\,c_{\text{short}}\,\mathbf{1}\{n_{it}\le 1\} + c_{\text{bed}}\,n_{it}\,\mathbf{1}\{n_{it}\ge 2\}\big]}_{\text{in-patient}} \;+\; \underbrace{c_{\text{op}}\cdot\widehat{n}^{\text{op}}_{it}}_{\text{out-patient}} \;+\; \underbrace{c_{\text{gp}}\cdot\widehat{n}^{\text{gp}}_{it}}_{\text{GP}}, \tag{13}$$

  with adm_{it} any in-patient stay and n_{it} the nights in hospital.
- The unit costs are national averages for England in 2024/25. The three hospital prices come from NHS England's National Cost Collection (NHS England, 2025), which costs every admission and attendance in English NHS trusts:
  - $c_{\text{short}} = £826$, the average non-elective short-stay episode (0–1 nights).
  - $c_{\text{bed}} = £791$, the average cost per bed day of a non-elective long stay (2 or more nights).[6]

[6]From the collection's patient-level costs, given in answer to written question 100046 (UK Parliament, 22 December 2025) `https://questions-statements.parliament.uk/written-questions/detail/2025-12-15/100046`. A

  - $c_{\text{op}} = £191$, the average out-patient attendance across consultant-led (£203) and non-consultant-led (£132) attendances and out-patient procedures (£232).

  The GP price is not in the collection. $c_{\text{gp}} = £40$ is a 10-minute surgery consultation, including direct care staff and excluding qualification costs (£48 with them), from PSSRU's *Unit Costs of Health and Social Care 2025* (Jones et al., 2026, Table 9.4.2).

- None of these prices depends on age. The NHS costs an admission by its Healthcare Resource Group (HRG), a group of similar diagnoses or procedures. Some HRGs have separate versions for children (18 and under), but none has an older-age band. Within a condition, however, an admission with more or worse complications and comorbidities falls into a dearer HRG. An older person's stay therefore costs more because they are on average sicker, not because they are older. Our index prices every short stay and every night the same, so it picks up severity only through length of stay and misses the rest of this gradient. The convexity of cost in poor health that we report is therefore probably understated.
- Pricing every night at the average works the other way for the longest stays, whose later nights probably cost less than the first: 42% of reported nights fall in years with more than 30 nights in hospital. A two-part price with the same £5,449 for the average long stay, a fixed £3,038 per stay plus £350 a night, lowers the sickest-to-healthiest ratio from 18.6 to 13.9 but leaves the convexity intact (quadratic $t = 22.4$; Appendix C).
- The index averages about £900 a year, against NHS spending of roughly £3,000 per head, as it leaves out prescribing, A&E, community services and social care. It is wrong in level but roughly right in *shape*, and the variation of cost is all we need. Underlying data, construction and validation against published statistics are in Appendix C.

Figure 1 shows that:

- Cost rises convexly with poor health: the quadratic term in standardised θ, person-clustered, has $t = 17.4$ (33.5 on costs capped at the 99th percentile). A cubic term steepens the sick end further. It is significant on the cost index ($t = -4.5$) and, with a much smaller coefficient, on capped costs ($t = -2.3$). Section 5 prices with the cubic, drawn in Figure 1(a), and Appendix G.4 with the capped fit, which is a quadratic.[7]
- Over most of the range the relationship is close to age-invariant. The binned curves for the three age bands lie on top of one another down to $\theta \approx -2$; in the sickest pooled tenth mean cost is £4,075 at 20–34 and £4,392 at 75–90; and the quadratic term is positive within every one of six age bands. Beyond -2 the over-65s cost more. The 75–90 band's curvature is roughly double the others' ($t = 4.4$) and still about 40% higher when costs are capped at the 99th percentile ($t = 3.6$). This comes from in-patient nights: below $\theta = -2$, 35% of the over-65s are admitted in a year against 26% at 45–64, and those admitted spend 20 nights in hospital against 16. Longer stays at the same measured health could reflect severity that θ misses, or slower discharge.[8]
- Mortality also rises steeply with poor health, roughly log-linearly: pooled across ages with a quadratic in age, the log-odds of death fall by 0.90 per standard deviation of θ.
- The health-mortality gradient is relatively constant with age, although the slight flattening after 45 is statistically significant ($t = -2.3$); the level, however, is not age-invariant. Age thus carries mortality risk that the health measure does not capture, which matters for survival but not

non-elective long stay averages £5,449 per episode, which is 6.9 nights at £791. The 2024/25 collection no longer reports excess bed days separately, so we price every night. The cost per bed day of a short stay is almost the same, £806, so in the collection's own figures a stay's cost rises in proportion to its length.

[7] A quadratic coefficient is driven by the top of the cost distribution, so we also report results on costs capped at the 99th percentile. The cost index is uncapped throughout unless we say it is capped.

[8] The linear term's interaction with age is insignificant on θ in every specification we try ($|t| \leq 1.7$).

for cost. *[This kind of relates to what we were thinking about right at the start in terms of the age-health-mortality relationship...]*

- The same holds on h, with one difference on the frailty indices (Appendix I): the curvature has $t = 10.4$ on h and 20.0 on log frailty, but only 6.5 on the frailty index in levels (13.1, 38.3 and 4.6 capped), which is the least convex of the four.

[We can move this figure to the appendix or simplify it, but I wanted to include something for you to have a look at...]

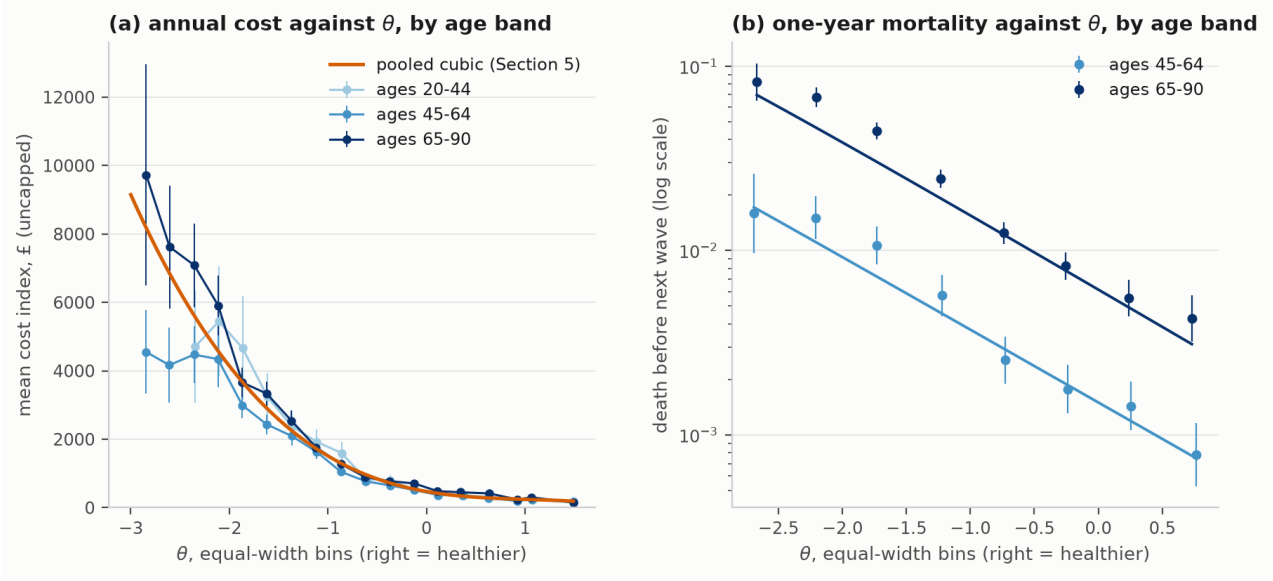


Figure 1: Cost and mortality against θ, by age band. (a) Mean annual cost index in equal-width bins of θ within three age bands, with 95% intervals; the orange line is the pooled cubic in θ that Section 5 prices with. (b) The one-year death rate in the same bins on a log scale, with the within-band logit drawn through it; ages 20–44 are left out of (b), as there are too few deaths in that range to identify the curve. The measure is standardised on the pooled sample, so a bin is the same health at every age. The same figure on h and the frailty indices is in Appendix I.

3.2 The moments of health over the life course

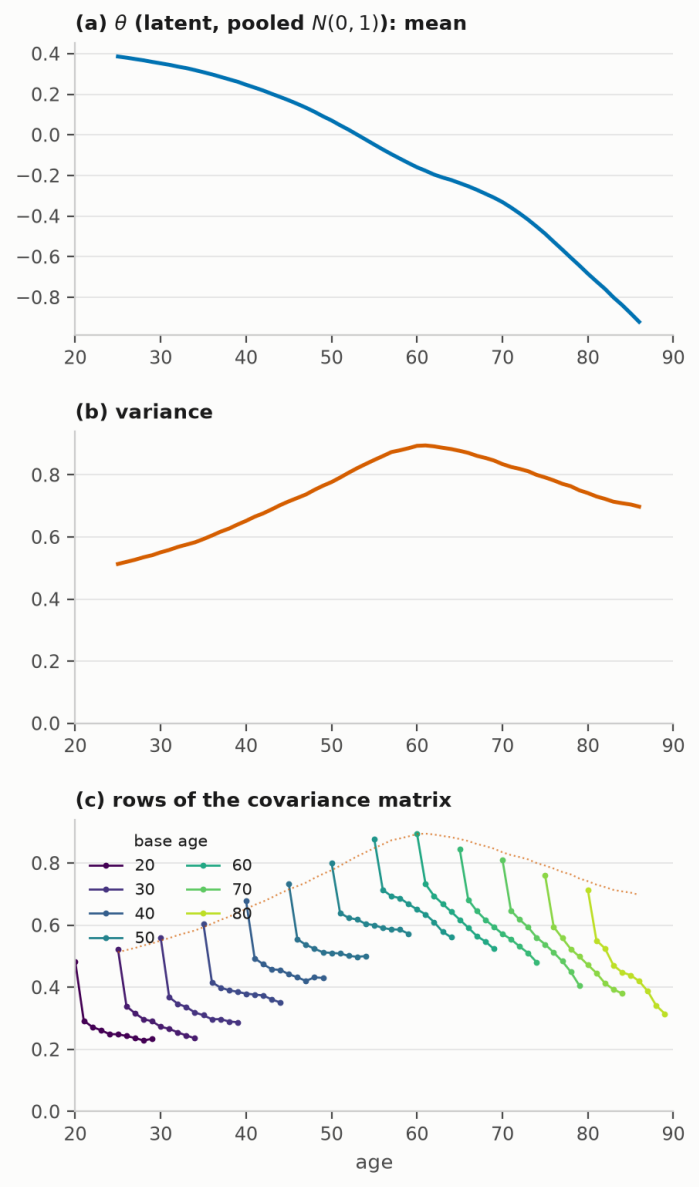


Figure 2: The measure's mean path, variance profile and covariance rows, model-free, on θ. 387,599 person-waves; the mean and variance profiles are ten-year centred rolling means; each covariance row starts on the diagonal at a five-year base age and runs nine years. The same figure on h, the frailty index and log frailty is in Appendix I.

- From panel (a) the mean accelerates down: θ falls by about 0.014 standard deviations a year between 30 and 50, 0.020 between 50 and 70, and 0.038 after 70. $\mathbb{E}(d)$ rises with age, which on its own says intervene late: the mean channel of $P(r)$ is largest where the preventable decline is largest.
- From panel (b) the variance rises by about 60% between 30 and 60, peaks at 62, and falls by a fifth by 85. Under stationary noise this says the local covariance $C_{Hd}(a)$ has reached zero around 60 and turned negative after it: interventions on dispersion pay early.

- From panel (c) we see divergence then convergence: every row drops sharply in its first year, by 40% at base age 20 and about 20% at 50–60 (the transitory shock, health not carried even one year). After this the younger rows run flatter while the older rows fall more steeply.
- The shape of the variance profile depends on the measure as Section 3 anticipated (Appendix I).
- On log(frailty + 1/31) the pattern is very similar, with variance rising by a factor of 1.70 from 30 to 60 and falling to 0.70 of its 60 level by 85, against 1.62 and 0.78 on θ.
- On h and the frailty index, variance continues to increase after 60, but at a markedly slower rate. The bounded measures keep dispersion growing into old age; the unbounded ones have it peak around 60 and then compress.
- Across all three alternative indices, panels (a) and (c) show the same stylised facts.
- One stationary noise process cannot produce both. Either the noise moves with age or the local C_{Hd} has changed sign: divergence among the young, compression among the old.
- This has implications for timing and targeting: intervene early and target the low early in life; later in life the covariance turns against targeting on levels so not much use targetting on low.
- Cautions carried to the next section: composition (who survives to be measured), floors and cohort pooling are all in these curves; the type models will use within-person information.
- The covariance rows in Figure 2 are based on pairwise covariances. Covariance rows on balanced samples, i.e. each row over the people observed at all of its ages, are shown in Appendix D (Figure D.1).

3.3 The linear approximation on real data

[**TODO:** Decide which cases, if any, that we should present here. We have four main cases and also a decision on whether we should use pairwise or balanced moments.]

- Figure 3 shows the V_H, C_{Hd} and V_d identified from pairwise moments on four ages four years apart (so over a 12 year span. To account for the non-linearity we show this pooled within four bands. We show Case 4 (persistent–transitory), the most general case we've looked at in detail; Case 1 (transitory only), the simplest, is in Appendix D (Figure D.3).
- These are not computed by solving the just-identified propositions cell by cell, but by least squares on the ten pooled moments of each band.
- Given $H_a = H + a\,d$ the covariance of the measure at offsets i and j years from the base age is $V_H + (i + j)\,C_{Hd} + ij\,V_d$ plus the noise.
- Under Case1, the noise is one variance on the diagonal;
- For Case 4 the noise is $\rho^{|i-j|}$ times a carried variance plus a transitory variance on the diagonal.
- The ten cells are regressed on these columns with V_H, V_d and the noise variances non-negative and $C_{Hd}^2 \le V_H V_d$ imposed, and ρ profiled over a grid.
- Each case is therefore overidentified, ten moments against four or five parameters, and the bars in Figure 3 stack the fitted layers against the observed moment in every cell, so the misfit is visible.
- $\mathrm{Corr}(H, d) = C_{Hd}/\sqrt{V_H V_d}$ stated in each subtitle is calculated from the fitted coefficients.
- There are many different ways to represent this (i.e. the health metric, the gap between ages, pairwise or balanced moments, the noise case) and the details vary, but relatively consistently C_{Hd} gets more negative with age, see a few more in Figures D.4 and D.5.
- For Case 4, on θ, $\mathrm{Corr}(H, d)$ runs +0.45, +0.78, −0.56, −0.53 across the four bands, with ρ 0.85–0.94 and no constraint binding.
- For Case 1 (Figure D.3), $\mathrm{Corr}(H, d)$ is negative in every band (−0.22, −0.02, −0.42, −0.48), but

it clearly gets more negative with age. *[I chose this specification because it works nicely for Case 4, but there are plenty that work nicely for Case 1 as well]*.

- On the other measures the sign path is the same, but whether it fits as cleanly depends on the specification (Appendix I). Under Case 1, log frailty almost reproduces θ (-0.14, 0.00, -0.44, -0.48). Under Case 4, h runs $+0.43$, $+0.63$, -0.31, and at 75–90 V_H goes to zero so nothing is identified; log frailty runs $+0.48$, $+1.00$, -1.00, -0.92, with the admissibility constraint binding at 40–60 and 60–75; the frailty index runs $+0.40$, $+0.98$, -0.22, and at 75–90 the constraint binds with V_H near zero.

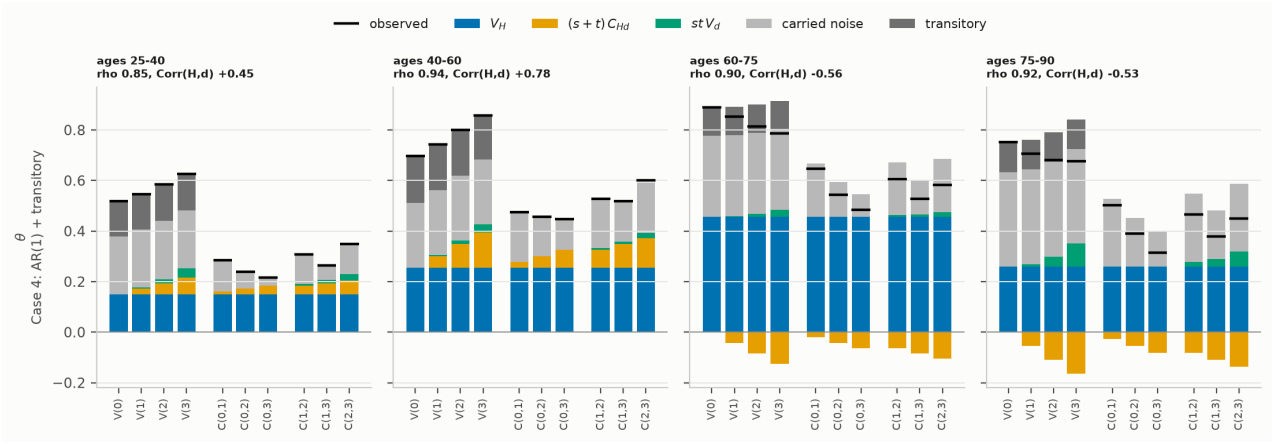


Figure 3: The linear approximation on θ: the ten moments of four ages four years apart in each band, pairwise moments, decomposed into the fitted layers under Case 4. Properly constrained fit: variances non-negative and $|\text{Corr}(H,d)| \leq 1$ imposed in the estimation. A base age enters when every cell rests on at least 50 people observed at both of its ages. Case 1 is in Appendix D, and Case 4 on h, the frailty index and log frailty in Appendix I.

4 Lifecycle types

4.1 Why and how

- The health-dynamics literature generally works with the old (HRS, ELSA) or short windows; De Nardi et al. (2025) splice the PSID early in life with the HRS later, but health is modelled only for the older people; Hosseini et al. (2022) build a frailty index on the PSID.
- The prevention framework says H_{55} is an *outcome* of the dynamics before 55, not an input. And the data say the dynamics differ across the lifecycle (Figure 2): starting in late middle age misses the divergence phase. So this is why we focus on the full lifecycle.
- **How.** Overlapping cohorts observed over 15 waves: cohort-independent types with cohort-dependent shares. First non-parametric (partial K-means), then a parametric refinement (Bayesian mixtures) that separates persistent deviations from transitory shocks, then against observables. Everything is on θ; the h versions, and the K-means types on the two frailty indices, are in the appendix.
- **Four challenges** addressed along the way: (1) power and type categorisation (functional forms, stochastic structure, several dimensions); (2) mortality and attrition; (3) cohort effects; (4) units and bounds (ceiling, floor, cardinalisation), which is why we work on θ and repeat the analysis on the bounded measures in the appendix.

4.2 (A) Non-parametric types: partial K-means

- Method: Included respondents with at least three observed ages between 20 and 89 (38,963), the same rows as the Bayesian mixtures below. Each person is a vector over ages 20-90 with most entries missing.
- Estimate type/cluster by minimising average squared deviations over observed entries only - "partial" K-meanrs. 50 seeded starts to insure that results are not sensitive to initialisation.
- Baseline case is $K = 3$.
- "Equal penalty": every person-wave is punished equally, so the method is neutral about whether the stochastic process is transitory or persistent. *[Open: the person is assigned to one cluster for life, which does impose that the type is permanent, so that does mean that the within person dynamics drive the results.]*
- We focus on means because they are what the policy objects use and what the interventions considered shift.

Figure 4 shows the cluster results on θ; h, the two frailty indices and predicted cost are in Appendix I.

- On θ, three ordered, non-crossing paths, shares 18 / 43 / 40%. The worst type starts below the others at 20 and falls fastest to 60, by 1.2 standard deviations against 0.4 for the best type; after 60 the order reverses, the worst type falling 0.3 by 85 and the other two about 0.8. Divergence and then convergence is visible in the type paths themselves.
- The choice of health metric matters a bit (Appendix I), but the overall shape looks very similar across each of the four we consider.
- On h K-means isolates a 10% sick tail (shares 10 / 28 / 61%) instead of splitting the healthy majority, and agrees with the θ types at ARI 0.37 (71% in the same type): θ stretches the healthy end. The frailty index lands with h (ARI 0.61, 86%) and log frailty with θ (ARI 0.44, 78%, shares 21 / 41 / 38%).
- The stochastic part (middle panel): within-type deviations are persistent, with lag-1 autocorrelation 0.64 in the worst type and 0.37–0.38 elsewhere, decaying by nine years to 0.24, 0.16 and 0.03 (the middle type). Suggests that a final model should include some persistence, and that it might differ by type.
- Composition: on θ the worst type's share of observed person-waves is 22% at 20, falls to 13% at 50 and recovers to 18% by 70. This is selection into the observed sample, not people changing type. *[And for θ in the persistent–transitory model, this pattern does somewhat reverse and we get a falling share of the unhealthy type by age.]*

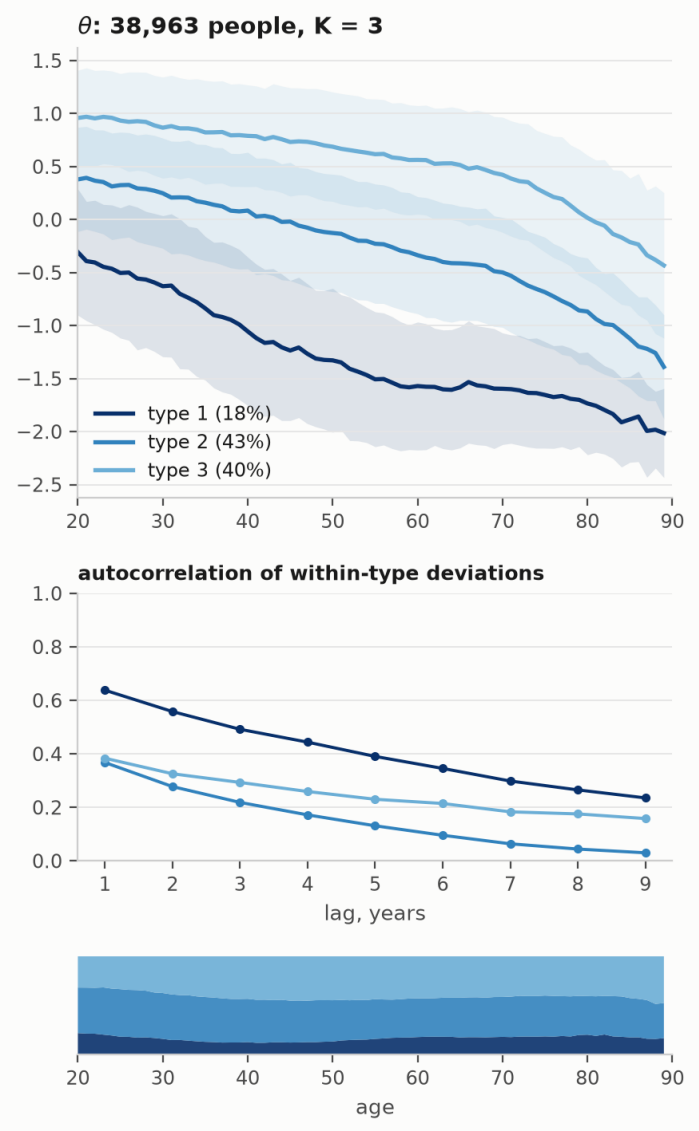


Figure 4: Partial K-means types ($K = 3$) on θ. Top: type means with the within-type ± 1 sd band; middle: autocorrelation of within-type deviations by lag; bottom: type composition of the person-waves observed at each age. The same figure on h and the two frailty indices is in Appendix I, and on predicted cost in Appendix E.1.

Figure 5 shows the key moments of Figures 2 and 3 computed from the K-means types rather than from the covariance surface, with the age-specific type composition as the weights throughout.

- Top panel: the between-type share of the cross-sectional variance rises from 48% at 30 to 65% at 70 and falls back to 57% by 85. The within-type variance is flat on θ. *[The persistent–transitory classes give the same shape from 40 on but a between share of only 19% at 30, because their paths coincide at 20 and separate only after 30; that is the same fact as the windows result of Figure 8.]*
- Middle panel: the spread of the type slopes across types, $\mathrm{sd}_j(d_{j,a})$, is lowest around 60 and rises towards both ends: it peaks at 36, the worst type's early decline, and again after 75, so slopes are most consistent across types in late middle age.
- Bottom panel: $\mathrm{Cov}_j(\bar{h}_{j,a}, d_{j,a})$ goes from positive to negative at 61: divergence and then conver-

gence, the accumulation and pathway margins of Corollary 1 changing sign in the data.

- The key messages are relatively consistent across h and θ (Appendix I).
- *[Between-type shares are partly mechanical: the clusters are chosen to maximise them. The comparison that is not mechanical is with observables in Section 4.4.]*

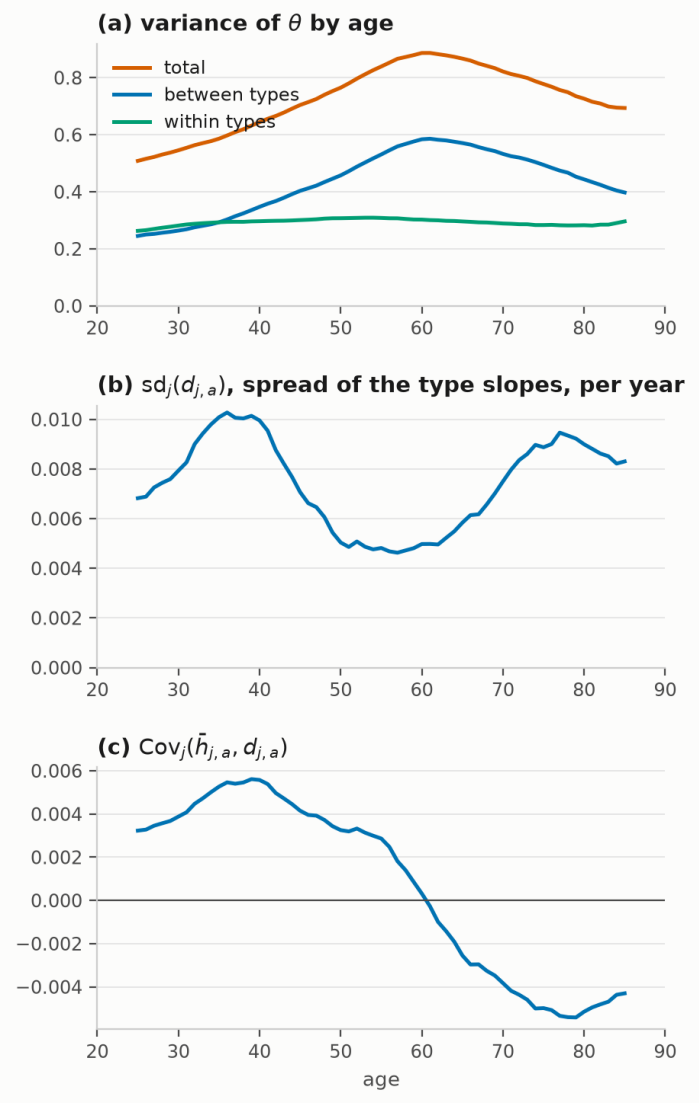


Figure 5: Between versus within by age, on θ, for the K-means types. (a) The cross-sectional variance split into between types, the variance of the type means under the age-specific composition, and within; (b) the spread of the type slopes across types, $\mathrm{sd}_j(d_{j,a})$, in θ per year, the slopes being the one-year changes of the smoothed type paths; (c) the level-slope covariance across types. Ten-year rolling means; age-specific composition shares throughout. The same figure on h and the two frailty indices is in Appendix I.

Small robustness exercise:

- Cohort adjustment within K-means (Appendix F.3): regresses the score on single-year age dummies and birth-decade dummies, subtracting the decade shifts (so identifies the decade effects from the overlapping ages).

- Re-clustering leaves the typology unchanged (adjusted Rand index 0.90 on θ, 0.96 on h; at least 97% of people keep their type). The shifts themselves are zero for cohorts born before 1960 and negative only from the 1970s on. *[worth noting that he Bayesian model does pick up more meaningful cohort effects, which I have not really thought about in depth yet.]*

4.3 (B) Parametric types: Bayesian mixtures

The parametric counterpart of the K-means types is a finite mixture of quadratic growth curves. Let y_{ia} be the measure of person i at age a, standardised on the contract, and $\tilde{a} = (a - 55)/10$. Person i belongs to one of K classes, $k_i \in \{1, \dots, K\}$, with $\Pr(k_i = k) = \pi_k$, and in the baseline specification

$$y_{ia} = \alpha_{k_i} + \beta_{k_i}\tilde{a} + \gamma_{k_i}\tilde{a}^2 + \varepsilon_{ia}, \qquad \varepsilon_{ia} \sim N(0, \sigma^2) \text{ i.i.d.}, \tag{14}$$

so a class is a path and a residual scale common to the classes: the transitory-only specification of Section 2.3. The class indicator is marginalised: person i's likelihood is $\sum_k \pi_k \prod_a N\big(y_{ia} \mid \mu_k(a), \sigma^2\big)$ over their observed ages, and their posterior class probabilities follow from the fitted parameters. Labels are identified by ordering the intercepts, $\alpha_1 < \cdots < \alpha_K$, so class 1 is worst health. Priors are weakly informative on the standardised scale: $\pi \sim \text{Dirichlet}(1.5)$, $\alpha_k \sim N(0, 1.5^2)$, $\beta_k \sim N(0, 1)$, $\gamma_k \sim N(0, 0.05^2)$ and $\sigma \sim N(0.7, 0.5^2)$ truncated below at 0.05. Estimation is by Hamiltonian Monte Carlo (the no-U-turn sampler) in Stan: four chains of 1,000 warm-up and 1,000 sampling iterations, with convergence judged by the split-$\hat{R}$ of the structural parameters.

Figure 6(a) shows the Bayesian equivalent of the partial K-means, with the K-means type means in red:

- Same data as the partial K-means, fitted on θ; the h, frailty-index and log-frailty versions are in Appendix I.
- The baseline $K = 3$ mixture (quadratic paths, transitory only, no AR term) converged (max $\hat{R} = 1.001$, about 3 h).
- The mixture and the K-means types are the same typology. Shares agree to a few points (.15/.44/.41 against .18/.43/.40), the class paths track the type means, and person by person 95% of people are in the same class (ARI 0.85, mean maximum posterior 0.93).
- The same holds on h (ARI 0.82, 94% in the same class): there both methods isolate a 10% sick tail, where on θ both split the healthy majority, so the h–θ split is a property of the scale, not of the method.
- The composition strips move the same way under both methods: on θ the worst class is 13% of the observed person-waves at 30 and 16% at 85 under the mixture, against 16% and 18% under K-means.

The paper's stochastic specification changes only the residual. In place of the i.i.d. ε_{ia} of (14), the deviation from the class path is a persistent AR(1) state plus a transitory shock, the persistent–transitory specification of Section 2.3,

$$y_{ia} = \mu_{k_i}(a) + u_{ia} + \varepsilon_{ia}, \qquad u_{ia} = \rho_{k_i}^{g}\, u_{i,a-g} + \eta_{ia}, \quad \eta_{ia} \sim N\Big(0,\ \sigma^2 \tfrac{1-\rho_{k_i}^{2g}}{1-\rho_{k_i}^{2}}\Big), \quad \varepsilon_{ia} \sim N(0, \sigma_\varepsilon^2), \tag{15}$$

where g is the gap in years since the person's previous observation, the state starts from its stationary distribution $u \sim N\big(0, \sigma^2/(1-\rho_k^2)\big)$, and the persistence ρ_k is class-specific while the innovation scale σ and the transitory scale σ_ε are common. The two variances are identified off the autocorrelation function, ρ^g times the state's share of the variance, so ρ comes from the ratio of successive autocorrelations and the transitory variance from their level; each person's likelihood is computed by

the Kalman filter, class by class, and the rest is as in the baseline. Priors add $\rho_k \sim \text{Beta}(2, 2)$ and $\sigma_\varepsilon \sim N(0, 0.4^2)$ truncated below at 0.01.

Figure 6(d) shows the persistent–transitory specification:

- The same specification on θ (and on h for the appendix), on the same contract; converged (max $\hat{R} = 1.002$, 4–8 h).
- On θ the story carries over. Shares .25/.47/.29, persistence .96/.90/.87, a transitory standard deviation of 0.42 against a persistent innovation of 0.23: between a fifth and a half of the variance around each class's path is transitory (signal shares 0.78, 0.63 and 0.56), the worst class starts within half a standard deviation of the middle and falls away, and the composition drift reverses: the worst class is 26% of the observed person-waves at 30 and 17% at 85, against 13% → 16% under the baseline.
- Agreement with the K-means types falls to ARI 0.42 (77% in the same class), and classification is less certain (mean maximum posterior 0.69 against 0.93).
- On h (Appendix I) the healthiest class collapses: its persistence is 0.25, against 0.96 and 0.78 for the other two, and agreement with the K-means types falls to ARI 0.46, with half of K-means type 2 reassigned to the worst class. This bounded-scale problem is one reason we report θ. The frailty index, bounded at zero, does the same at its floor (the $K = 3$ column of Figure I.6): under the persistent–transitory specification its healthiest class has persistence 0.36 against 0.98 and 0.81, and agreement with the K-means types falls to ARI 0.34; the shifted log lifts the floor only partly, with a small healthiest class (10%) at persistence 0.44 against 0.96 and 0.92 for the other two and ARI 0.29. Under the transitory-only specification all four measures reproduce their K-means types (ARI 0.81–0.89, at least 94% of people in the same type). *[when we go to $K = 4$ then we start getting a convergence again though]*
- The specification is close to that of Hosseini et al. (2022), who model log frailty as a fixed effect plus an AR(1) plus a transitory shock. Our classes play the role of a coarse fixed effect, and their persistence of 0.991 compares with our 0.87–0.96.

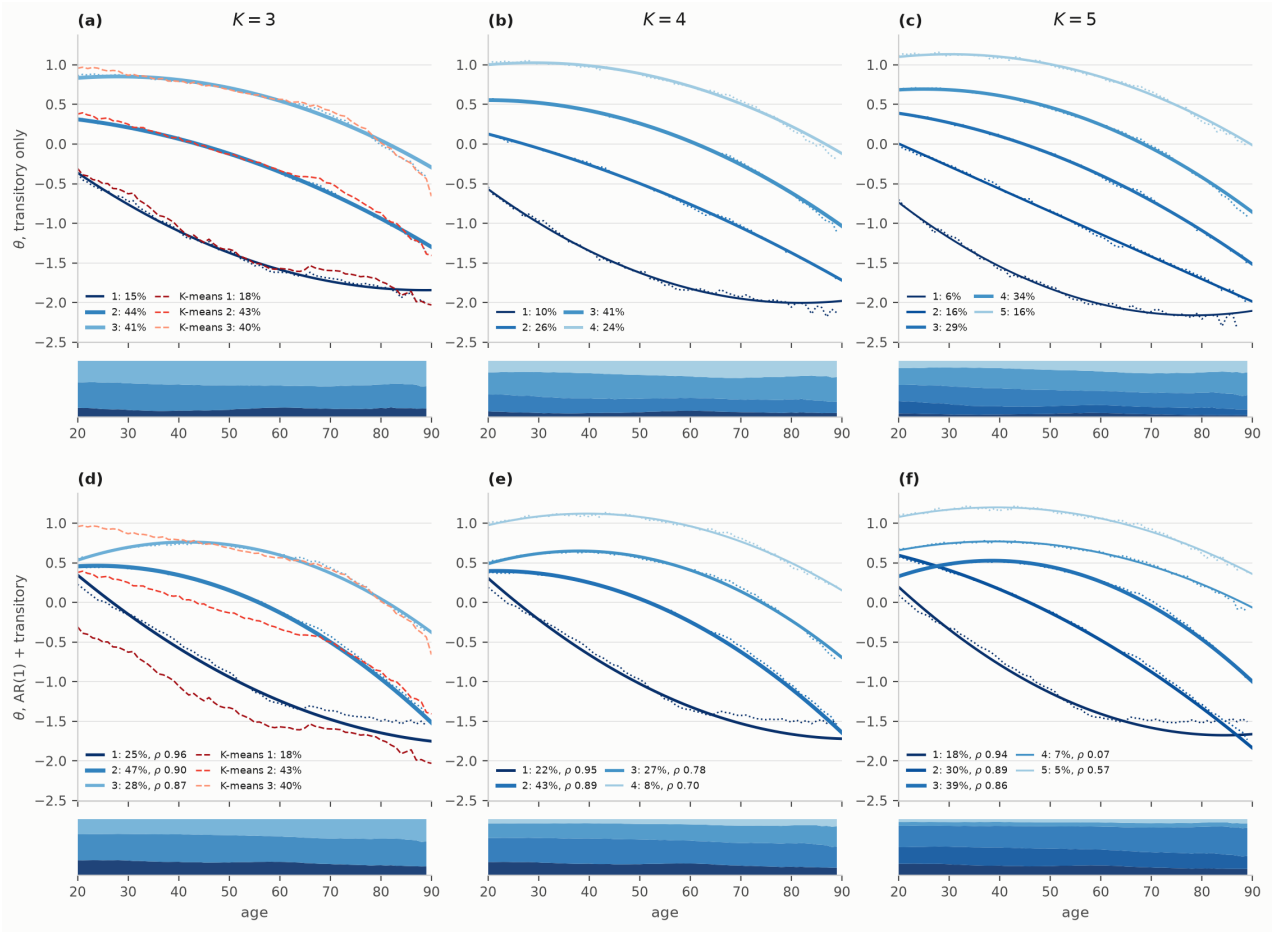


Figure 6: Mixtures on θ by specification and number of classes. Rows: transitory only, (a)–(c), and persistent–transitory, an AR(1) latent state plus a transitory shock, (d)–(f); columns: $K = 3$, 4 and 5. Solid: the class paths at the posterior mean, width proportional to the class share, with the share and, under persistent–transitory, the class persistence in the legend. Dotted: the observed class means, the measure averaged at each age over the people observed there weighted by their posterior class probabilities. Dashed red, in the $K = 3$ panels: the partial K-means type means on the same rows. The strip beneath each panel is the posterior class composition of the person-waves observed at each age. The same figure on h is in Appendix I.

Is there always a bad group?

- **Yes, and it is the same people under every method.** Every typology finds a worst group of 15–18% by modal label: 18% under partial K-means, 15% under the transitory-only specification, 18% under the persistent–transitory specification and 16% under the multidimensional fit. Of the K-means worst type, 84% are worst under the transitory-only specification, 85% under the persistent–transitory specification and 74% under the multidimensional fit; the mixtures give the K-means worst people a posterior probability of the worst class of 0.73–0.84 and everyone else 0.00–0.13. The persistent–transitory fit's larger share, 25%, is probability mass rather than people: a quarter of the population is in the worst class in expectation, 18% by argmax, because the persistent component leaves 38% of people with a maximum posterior below 0.6 (4% under the transitory-only specification).

- **What the stochastic structure changes is the account of how they got there, not the size of the group** (Appendix F.1 for the worst type by method and K).
- **The level-slope reversal of Figures 3 and 5 is this group.** Splitting $\mathrm{Cov}_j(\bar{h}_{j,a}, d_{j,a})$ into each type's term, the worst type's carries nearly all of the negative covariance after 60 under every typology and every K, and it is still negative when the slopes are taken from within-person changes rather than the type paths (Appendix F.2). Refitting the Case 4 moments without the bottom fifth of the age-standardised person mean, or without either method's worst type, leaves no reversal. The accumulation margin of Section 2 turns negative because the unhealthy fifth stops falling away from the rest while the healthy start to fall late, which is also why targeting on the type pays after 60 (Section 5). K-means and the transitory-only mixture, which classify on levels, find a worst type that is already a full unit of θ below the others at 20 and whose cross-sectional path flattens after 60. The persistent–transitory fit finds a worst class that starts near the average and declines fastest, because a low level at 20 can be read as a carried deviation rather than as type; it therefore puts the between-type share of the variance at 19% at 30 where K-means puts it at 48% (Figure 5), and it keeps the class declining through old age. So K-means does not understate the bad group; it overstates its early separation and understates its decline, and it is the decline, not the level, that the prevention returns of Section 5 price. The noisier the process, the more of the early-life gap a level-based method attributes to type.

Extensions: K, dimensions, mortality.

- $K = 4$ and $K = 5$ (Figure 6(b), (c), (e) and (f)) converge cleanly on every specification (max $\hat{R} \leq 1.004$). Under the transitory-only specification each added class is a new sickest group, 10% of people at $K = 4$ and a further 6% at $K = 5$, and the healthy majority is left alone.
- Under the persistent–transitory specification the split goes the other way. At $K = 4$ the three $K = 3$ classes barely move and the new class is a small healthy group, 8% with persistence 0.70. At $K = 5$ the three large classes are unchanged, with persistence 0.86–0.94, and the two new classes are both small and healthy, 7% and 6%, one with persistence 0.07: a collapse like h's reaches θ too at $K = 5$, but in a small class rather than half the sample. Classification certainty falls from a mean maximum posterior of 0.69 at $K = 3$ to 0.66 at $K = 4$.
- On h (Appendix I) the healthiest-class collapse spreads rather than resolves as classes are added: the top two classes have persistence 0.52 and 0.19 at $K = 4$, and 0.39 and 0.18 at $K = 5$, covering about half the sample.
- Multidimensional model (Figure 7): the physical measure (θ; h in Appendix I) and a mental GRM (the three SF-12 mental testlets and two GHQ-12 testlets, no depression diagnosis; Appendix H looks at it on its own) as Gaussian channels, plus mortality, one shared class structure; the health contract's people with a mental score on every row, 38,181 people and 2,045 deaths. Under the persistent–transitory specification the persistence is class and channel specific while the innovation and transitory scales are common to the classes; the hazard is Gompertz–Makeham, $\lambda_k(a) = c + b_k e^{\vartheta_k(a-55)}$, integrated over each row's years at risk, with class-specific level and slope and a common Makeham constant. The figure is the fit with one level shift per birth decade on both channels, common to the classes, drawn at the 1950s: without the shifts (Appendix F.3) the mental class paths rise with age, which the shifts attribute to the older cohorts' higher level, with the caveat on reading them as cohort effects set out below.
- The physical typology survives the extra channels. The classes are .18/.50/.32 with physical persistence .94/.90/.86, the same to two decimals as without the decade shifts (.19/.51/.31 and .94/.91/.87), and 76% of people land in the same class as under the single-channel fit without shifts (73% against the single-channel fit with them, Figure F.7). On h (Appendix I) the healthiest

class keeps its collapse, persistence 0.28, even though its mental persistence is 0.86.

- The decade shifts change nothing else either: the same transitory scales, a class-1 to class-3 hazard ratio of 11.5 at 55 and 3.2 at 85 against 10.6 and 3.3 without them, and a held-out twin that predicts exactly as the fit without shifts does (Figure F.9 and Section 4.5: R^2 0.61 and 0.74 for θ_{t+1} from the classes and the forecast, against 0.61 and 0.74). The shifts themselves fall monotonically from the 1920s to the 2000s, by 0.9 sd on θ and 1.3 sd on the mental score.
- Mental health is faster and noisier than physical health in every class: persistence .93/.89/.82 against .94/.90/.86 on θ, and a transitory standard deviation of 0.54 against 0.42, so the signal share of the mental channel is 0.45–0.66 against 0.55–0.74. A shared persistence across channels, as in earlier versions, would have been wrong. At the 1950s the worst physical class sits a standard deviation below the best on mental health at 55, and every class's mental path declines with age once the decade shifts are taken out.
- Mortality is strongly graded by class and converges with age: the yearly hazard of the worst class is 11.5 times the best class's at 55 and 3.2 times at 85 (10.6 and 3.3 without the shifts; 7.1 and 2.7 under the transitory-only specification), the sickest class having the flattest Gompertz slope (0.09 against 0.14 per year). The Makeham constant is below 10^{-4} in every fit. Mortality describes survival by class; it does not define the classes. This is where the survivor-bias discussion lives.
- Note that in the persistent–transitory case, the class composition by age now looks like it reflects mortality selection. We need to look into this a bit more, but it is certainly promising!
- **Windows** (Figure 8): the persistent–transitory mixture refitted on twenty-year windows and on windows expanding backwards from 90, each on its own contract, compared with the lifecycle classes by the co-assignment correlation (over pairs of people, the correlation between the probability that the window fit puts the pair in the same class and the probability that the lifecycle fit does; it needs no label correspondence and is 1 whenever the two posteriors coincide, however diffuse). The sliding windows agree at 0.68–0.88 up to 60–80 and 0.42 at 70–90; the backward windows at 0.75 from 60, 0.86 from 50, 0.95 from 40 and 0.99 from 30. Short windows blur the posteriors more than they regroup people: the adjusted Rand index of the modal classes is far lower (0.23 for 20–40 against 0.68 on the correlation), which is the modal label becoming a coin toss among the healthy young, not a different typology.
- **A window finds the bad type only if it contains the years in which that type diverges.** The worst class is not the same object in every window. Its share of a window's people runs 15, 22, 33, 8, 16 and 16% across the sliding windows: the 40–60 window finds a worst class a third larger than the lifecycle one (32% of the person-waves at 40–60 against 24%), because that is where the decline happens and a path model separates the decliners there, while every window starting at 50 or later finds a small, flat, very sick class (7–14% of the person-waves at 60–85 against 18–22%) and puts the late decliners into a middle class of 50–80%. The backward windows say the same from the other side: the worst class is 16–17% in the 70–90 and 60–90 windows and 14% in 50–90, then 30% once the window reaches back to 40 and 27% from 30. The bad type is not growing because a later window's people are older; it appears when the window reaches back far enough to contain the divergence, which sits between 40 and 60. A level is visible at any age, a slope needs exposure (Section 2.3), and about fifty years of ages, most of them the 40–60 decade, is what the types need.
- At the oldest ages three classes are not supported: the 70–90 window's third class is 3% of people with persistence 0.09, the mixture wanting two classes and a transitory shock. That is the one window where the correlation is low for a real reason. The partial K-means version (Figure E.2) is more stable across windows (0.58–0.95) and its worst type's share moves with the age composition of the window's people, but that stability is the method's insensitivity to where the decline happens:

K-means classifies on person-by-age profiles, the mixture on paths with persistence, and only the latter can tell a late decliner from a lifelong low type.

- **Cohort effects.** The persistent–transitory fits, single-channel and multidimensional, were repeated with one level shift per birth decade, common to the classes (Appendix F.3). With fifteen waves the panel observes each person over at most a fifteen-year span, so a cohort's level is identified from the overlap of neighbouring decades at the ages they share.
- **The typology is unaffected.** Shares, persistence and the transitory scale agree with the fits without cohort effects to two or three decimals on every variant, and the class paths keep their shape; the worst class is a little steeper once the decade shifts carry part of the level. Whatever cohort variation there is, it is not what separates the classes.
- **The cohort effects themselves should not be read strongly.** The fit reports a monotone decline between the 1950s and the 2000s of about two thirds of a standard deviation on θ (a third on h). But the raw decade profiles line up almost exactly at the ages they share, and subtracting the estimated shifts opens gaps that were not there. Over a fifteen-year window age and birth year are nearly collinear, and a decade dummy is close to a linear function of age within it, so with a single quadratic age path per class the dummies absorb the curvature the quadratic cannot follow. With single-year age dummies in place of the quadratic, the pre-1960 shifts vanish and only the post-1970 cohorts decline (Appendix F.3), so the mixture's shifts should not be read as cohort effects.
- **Specification.** The same $K = 3$ fits with an AR(1) in the class residual but no transitory shock give persistence of 0.79/0.28/0.40 on θ, against 0.87–0.96 once the one-period term is separated: without it ρ has to absorb the one-period noise and is pulled down.
- [**TODO:** still to do for the bounded measures in the appendix: a bounded observation model (scores at the ceiling of h or the floor of frailty treated as censored).]

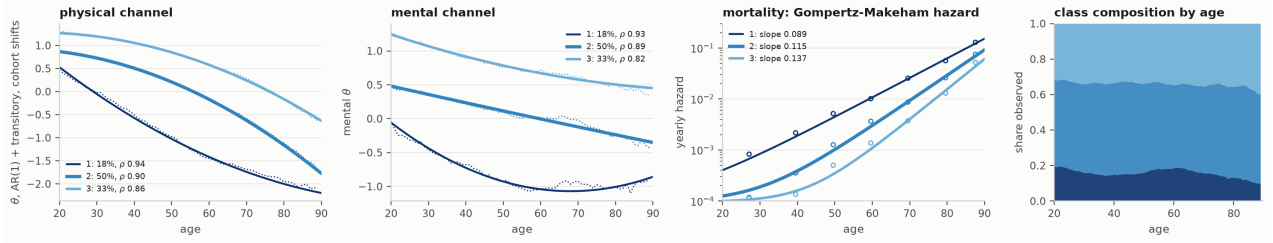


Figure 7: The multidimensional $K = 3$ mixture on θ, persistent–transitory, with one level shift per birth decade on the physical and the mental channel, common to the classes. Columns: the physical and mental class paths at the posterior mean in each channel's units, drawn at the 1950s birth decade with every score net of its estimated decade shift (dotted: the observed class means, posterior-weighted); the class-specific Gompertz–Makeham yearly hazard on a log scale (circles: the posterior-weighted crude death rate by five-year band); and the posterior class composition of the person-waves observed at each age. The legend gives each class's persistence on that channel. The fits without the decade shifts, persistent–transitory and transitory only, are in Appendix F.3; the same on h in Appendix I.

In the multidimensional model the physical channel dominates the mental one: its classes are the physical classes with a mental path attached, 73% of people taking the same class as in the fit on θ alone against 49% for the fit on the mental score alone (Appendix F.4).

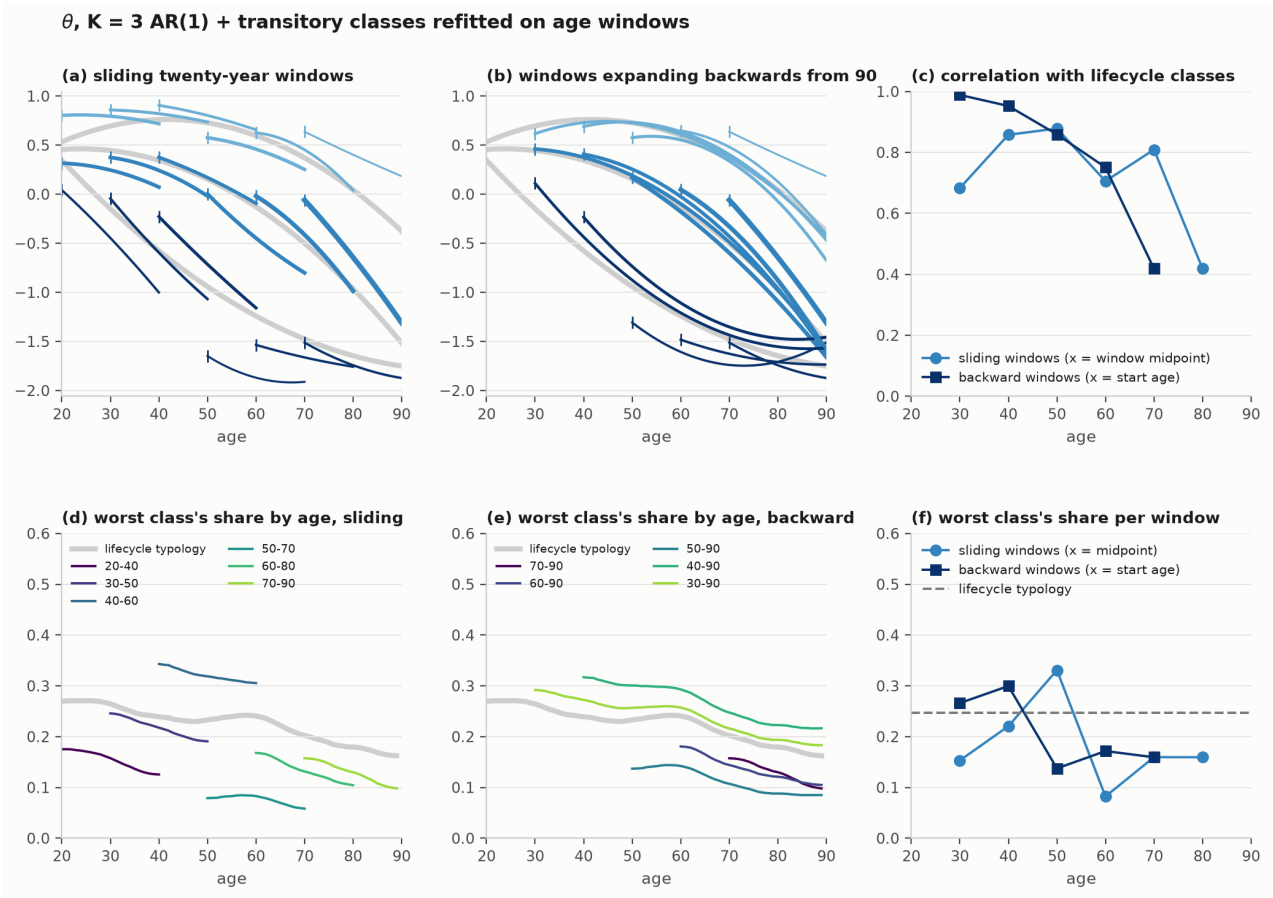


Figure 8: The $K = 3$ persistent–transitory classes on θ refitted on sliding twenty-year windows (a) and on windows expanding backwards from 90 (b), each on the people observed at least three times inside it, against the lifecycle classes in grey; (c) the co-assignment correlation with the lifecycle classes: over pairs of people in both fits, the correlation between the probability that the window fit puts the pair in the same class and the probability that the lifecycle fit does, which needs no correspondence between the two sets of labels and is 1 whenever the two posteriors coincide. Lower row: the worst class's posterior share of the person-waves observed at each age, for the sliding (d) and backward (e) windows, against the lifecycle fit in grey; (f) the worst class's share of each window's people. The partial K-means version is Figure E.2 in Appendix E, K-means on h is in Appendix I, and the $K = 4$ twin is Figure F.8.

- **Life expectancy by class** (the solid lines of Figure G.11(a), (b) in Appendix G.6). The panel records a death only when the survey learns of it, so the fitted hazards understate mortality: left alone they give a population life expectancy of 23.5 years at 65 against the ONS figure of 19.7 (both sexes, at the sample's 56% female). One common factor on every class hazard, rising from 1.17 at 20 to 1.54 at 65 and 1.81 at 90, matches the ONS values at 65 and at 90 exactly, and then gives 61.1 years at 20 against the 60.8 implied by the ONS figure at birth, which it was not fitted to. The class *differences* are the fit's; only the level is calibrated.
- From age 25 the three classes expect 47.7, 57.2 and 62.2 more years: a gap of 14.5 years between the worst and the best class, which narrows to 9.9 years at 65 (13.2 against 23.1) and 5.2 at 80. On the univariate classes used for the prevention exercises of Section 5 (Figure 12), whose hazards are estimated after the fit and calibrated in the same way, the gap from 25 is 10.6 years (51.4,

57.6 and 61.9).

- Healthy life expectancy separates the classes far more. From 25 the worst class expects 17.5 healthy years of its 47.7, the best 59.2 of 62.2: the worst class spends 63% of its remaining life in less than good health, the best 5%. From 65 the worst class has 2.2 healthy years left of 13.2, the middle class 11.4 of 19.1, the best 21.0 of 23.1. Most of what separates the types is years of ill health lived, not years of life.
- The population life expectancy weights the classes by their posterior composition among the people observed at each age, so it carries the selection of Figure 7. The population's healthy life expectancy at 65 (13.5 years) is above the ONS healthy life expectancy, which uses a similar self-rated measure (general health very good or good, where ours is good, very good or excellent), because the panel's survivors at each age are healthier than the population's; the mortality calibration corrects the length of life but not the health of those observed. [**TODO:** Decide whether to calibrate the healthy share as well, against the ONS health state life expectancies.]

Predicting death. Predicting mortality is hard: death is rare (0.4% of person-waves end in death before the next wave) and age alone already gives an AUC of 0.85. The class improves on age a little (0.88 for the persistent–transitory class), but does slightly worse than the current health reading on its own (0.90; Figure F.9 in Appendix F.6).

4.4 (C) Types against observables

- Figure 9: (a) the three types, the persistent–transitory class paths solid and the K-means type means dashed, which agree from 40 to 70 and part at the ends (the mixture's classes coincide near 20, the K-means worst type's mean plateaus after 60 as its membership turns over); (b) every cell of highest qualification × sex × white/other ethnic group × income tercile, 40 groups with at least 30 people at an age; (c) education; (d) deciles of the person's mean within-wave rank of equivalised household net income (both built from the raw files). Region is left out of (b) because twelve regions empty most cells; add it if the point needs making with more lines.
- Gradients exist and are in the expected order: at 50, θ is 0.21 for degree holders against -0.20 for no qualification, and 0.44 for the top income decile against -0.39 for the bottom, with the income gap widest at 55 (0.87 standard deviations).
- But even the 40-way partition spans a band of 0.9 to 1.1 standard deviations across ages 30 to 70, against 1.5 to 2.0 between the types.
- The between-group share of the cross-sectional variance is 3% for education, 5–8% for income deciles and 6–9% for the 40 cells, against 48–65% for the types; the observable partitions are essentially independent of the types (ARI 0.01–0.03).
- *[The between-share comparison is partly mechanical (the types maximise it); the fairer statements are the ARI, the type composition of each observable group, and the predictive comparison below.]*

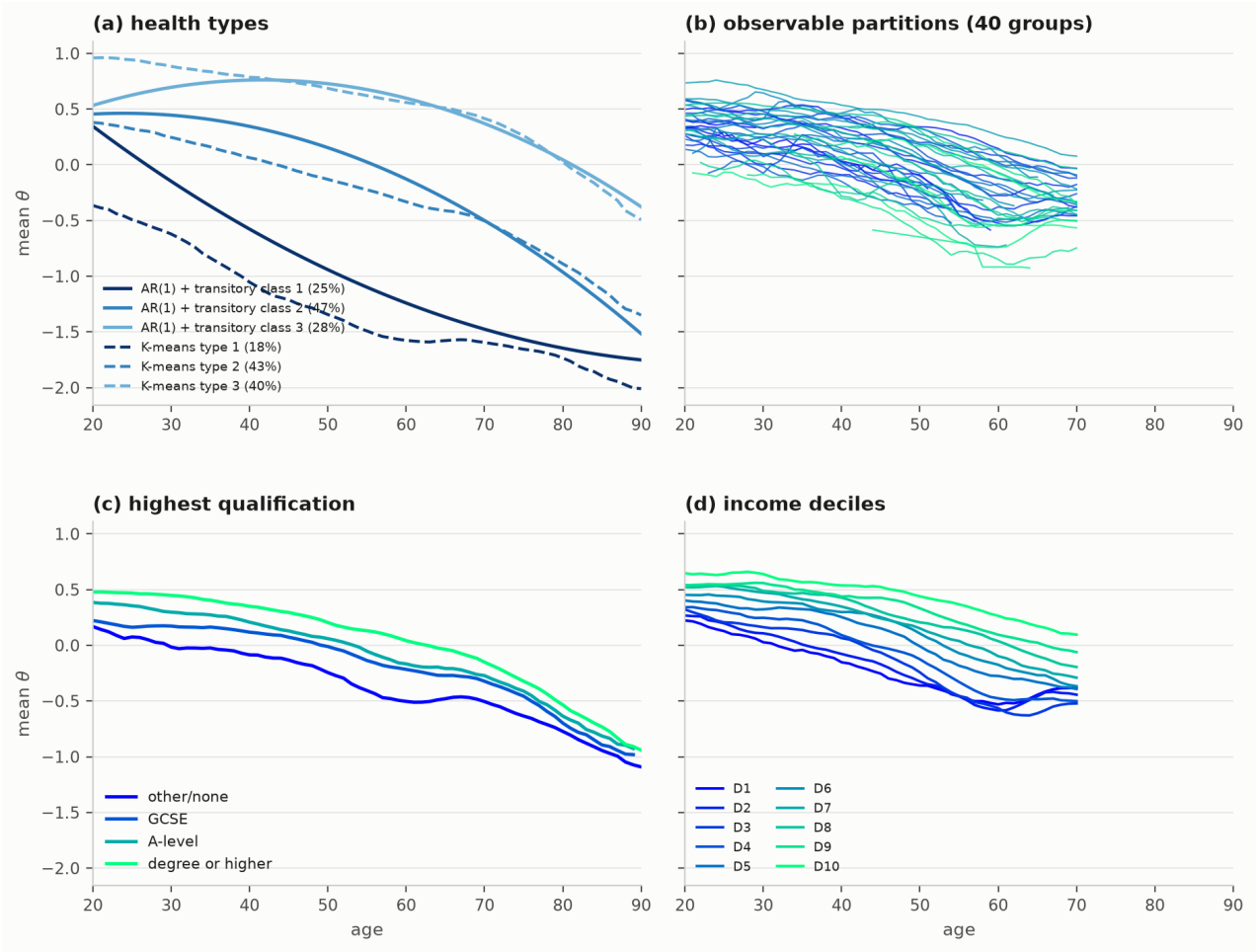


Figure 9: Lifecycle trajectories of θ by (a) health type, (b) 40 observable partitions, (c) education and (d) income decile, on the clustered people. Every series is a five-year centred rolling mean of the age-specific cell means, cells with fewer than 30 people suppressed; in (a) the persistent–transitory class paths are solid and the K-means type means dashed. Panels (b) and (d) involve income and stop at 70. The same figure on h is in Appendix I.

4.5 Predictive value

Broadly two types of exercise here - hold out the last two observed rows, or hold out the first two observed rows. The first tells us how valuable/accurate these clusters are at predicting future developments, and the second tells us how easy cluster membership is to predict based on health and other observables. Headline result is that health is what matters, the model helps a bit, and observables are almost irrelevant.

In order to run these exercises, we use people with at least five rows, so that every fit keeps at least three after two are held out: 26,664 people with at least five rows in both the single-channel and the multidimensional samples for predicting ahead, and 29,899 with at least five rows of health for predicting the type (29,278 for the multidimensional types).

First exercise - predicting the future:

- Fit model on the smaller sample and held out the final two observations for each person, so t is a

person's final in-sample observation and out-of-sample predictions can be made for $t+1$ and $t+2$.

- Predict two things: health and the cost proxy, using only information up to t.
- In each case, baseline is a regression of quadratic in age on the outcome, estimated on 70% on individuals and predicted on 30%. The extra information is then added as a predictor to this regression.
- Key results are:
  - Observables predict basically nothing.
  - Current health and the class contain about the same amount, but the forecast of the model does slightly better (but still only sligthly better than having two past health observations).
  - The multidimensional classes and forecast match the single-channel ones, because the multidimensional model is dominated by the phsical health channel.
  - Ranking is about the same for $t+1$ and $t+2$,

Second exercise - predicting types:

- Fit model on the smaller sample but held out the first two observations for each person, so t is a person's first in-sample observation and so their type can be predicted using information from $t-1$ and $t-2$.
- Observables basically don't predict type (panel b), but there are clear and intuitive differences between the types (panel d).
- One health reading is by far the most valuable, and a second one helps a bit as well, but really not much.
- Health is the weakest guide to type when young: the balanced accuracy of the persistent–transitory type from θ_{t-1} rises from 0.43 at ages 20–34 to 0.60 at 35–49 and 0.72 at 50–64, then eases to 0.67 at 65–90 (0.58, 0.65, 0.70, 0.66 for K-means; by the age the reading was taken). Predicting ahead has the same shape (panel c), peaking at 50–64; trajectories have not yet fanned out when people are young.
- Footnote comparing to h - θ does much better because h ends up really bunched at the ceiling while θ has a more balanced distribution (Figure I.11).

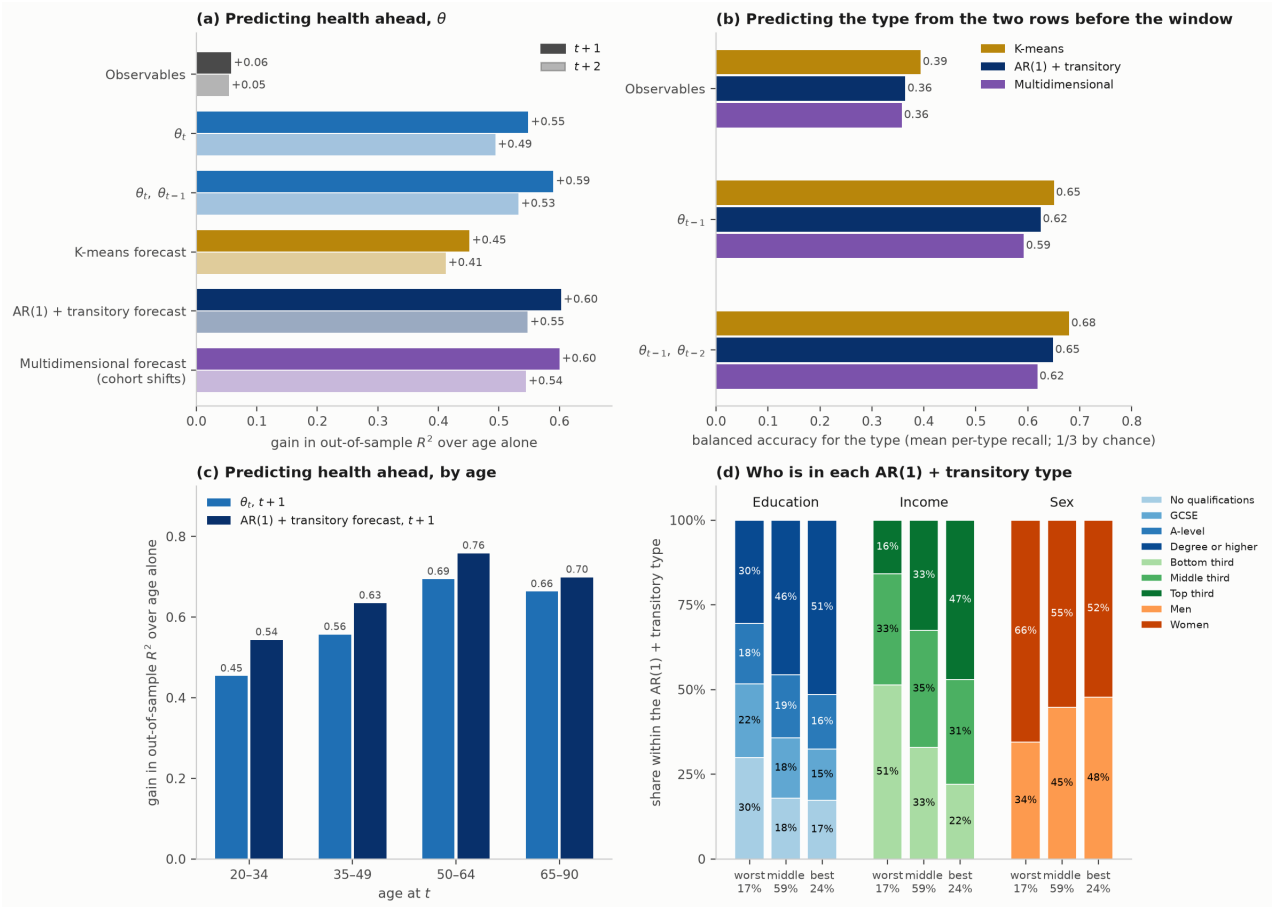


Figure 10: Predicting ahead and predicting the type, on θ. Left: the gain in out-of-sample R^2 over a quadratic in age in predicting θ one (full bars) and two (light bars) waves ahead, from what is known at t, a person's last fitted row, (a) overall and (c) one wave ahead within age bands at t for θ_t and the persistent–transitory forecast (models fitted on everyone; people with θ_{t-1} observed; within a band age alone explains almost nothing, so the gain is close to the R^2 itself); every model was fitted with each person's last two rows held out (26,664 people with at least five rows in both the single-channel and the multidimensional samples). "Observables" are education, income rank, sex, region and ethnic group; the K-means forecast is the type's mean at the outcome's age, the persistent–transitory and multidimensional forecasts the class-weighted path plus the filtered state carried forward; the multidimensional fits, here and in (b), have birth-decade shifts. Right: every typology refitted with each person's first two rows held out, so the type is defined from t, a person's first fitted row. (b) Balanced accuracy (the mean recall over the three types; 1/3 by chance) of a multinomial logit for the modal type, from the observables, the closest held reading θ_{t-1}, and both held readings, each on a quadratic in age (29,899 people; 29,278 for the multidimensional fit). (d) Education, income third and sex within each persistent–transitory type of (b), with the type's share. People split 70/30, fitted on the 70, scored on the 30. Panels (a) and (c) for the cost proxy are Figure I.12 in Appendix I.

5 Prevention exercises

We price the returns of Section 2 on the $K = 3$ persistent–transitory classes on θ (Figure 6(d)). The cost function is the curve estimated in pounds on θ (Figure 1), a cubic fitted to the cost index over waves 7–15, and the returns take the general-cost form of Section 2.2. Class shares and paths are the fitted mixture at its posterior mean; $\mathbb{E}[-c'(\theta) \mid j]$ at each age is the posterior-weighted mean over the person-waves observed there, and the bill $C(r)$ and its three parts are the observed cross-section priced by the same curve. Returns are pounds of treatment cost to 90 avoided, per unit of μ to first order and exactly at $\mu = 1$ (8), with no survival weighting and no discounting. The K-means version and the capped cost curve are in Appendix G; the same exercise on h is in Appendix I.

- **The bill and what prevention avoids** (Figure 11(a), (b)). The treatment bill from 25 is £67,500 per person to 90, £53,000 from 50 and £33,000 from 70. On the persistent–transitory classes about 63% of it is the cost along the mean path, 16% is what the type differences add and 21% is realised deviations, the part no prevention reaches (19% from 50, 17% from 70). Uniform prevention from 25 avoids £79,200 per unit of μ to first order, so $P(r)/C(r)$ is 1.17 at 25, falling to 0.78 at 50 and 0.45 at 70. $P(r)$ is the saving per unit of μ as μ starts to rise from zero, so on a convex cost curve it is not bounded by the bill: each unit of health restored is worth less once the person is healthier. Because the curve is a cubic, (8) gives the saving at any μ exactly: from 25 it is $\mu\,(79{,}200 - 46{,}200\,\mu + 10{,}700\,\mu^2)$ pounds, so the first-order return overstates the saving per unit of μ by 6% at $\mu = 0.1$, 16% at 0.25, 35% at 0.5 and 81% at $\mu = 1$. Holding back the whole decline ($\mu = 1$) saves £43,700 from 25, 65% of the bill, and 52% from 50 and 35% from 70. Either way, the later the start, the larger the share of the remaining bill that is already determined. On the transitory-only classes $P(r)/C(r)$ is 0.88, 0.54 and 0.29, and $S(r)/C(r)$ is 0.56, 0.39 and 0.24, since more of the decline is read as noise. [**TODO:** Note here the difference between the $\mu = 0.1$ case which is basically the first order approx, and teh full bill in panel (b), which tails off because of the convexity in health costs.]
- **Gap times preventable decline** (Result 2). Panels (a) and (c) of Figure 11 are the two factors of (5) multiplied age by age, at $\mu = 0.1$: (c) is the saving in each year, to first order $0.1\,s_j(a \mid r)$, the gap at a times the decline opened by a, and (a) is its sum over ages. From 25 the worst class returns more than twice the uniform return (£184,400 against £79,200 per unit of μ to first order; £17,400 against £7,500 at $\mu = 0.1$) because its gap and its decline coincide, already below the others at 25 and falling fastest; the middle class returns a little less than the uniform amount (£66,900); the best class, healthy and nearly flat, a tenth of it (£8,400). The mean channel of Corollary 1 is 60% of the uniform return from 25, 75% from 50 and 93% from 70, the between-class covariance the rest; of the exact saving at $\mu = 1$ (panel (b)) it is 72%, 81% and 97%.
- **Timing.** On the persistent–transitory classes the between-class channel of $P(r)$ is positive at every start age: +40% of $P(25)$, +25% of $P(50)$ and +7% of $P(70)$. On the transitory-only classes it is +20% at 25 and turns negative by 50 (−8%), reaching −44% at 70: the sign change of Figure 2 priced. Once the one-period noise is absorbed, the classes fan out for longer, and divergence keeps working for uniform prevention into old age.
- **Targeting on the class.** From 25 the worst class's return is £184,400, against £79,200 uniform and £8,400 for the best class, so class targeting collects 2.3 times the uniform return (a gain of 133%). From 50 the worst class still has the highest return and the gain is 38%; from 70 the middle class does, and the gain is 51%. The exact saving at $\mu = 1$ gives the same ranking, with gains of 135%, 64% and 45%. On the transitory-only classes the gains are 193%, 61% and 53% (210%, 93% and 51% at $\mu = 1$).

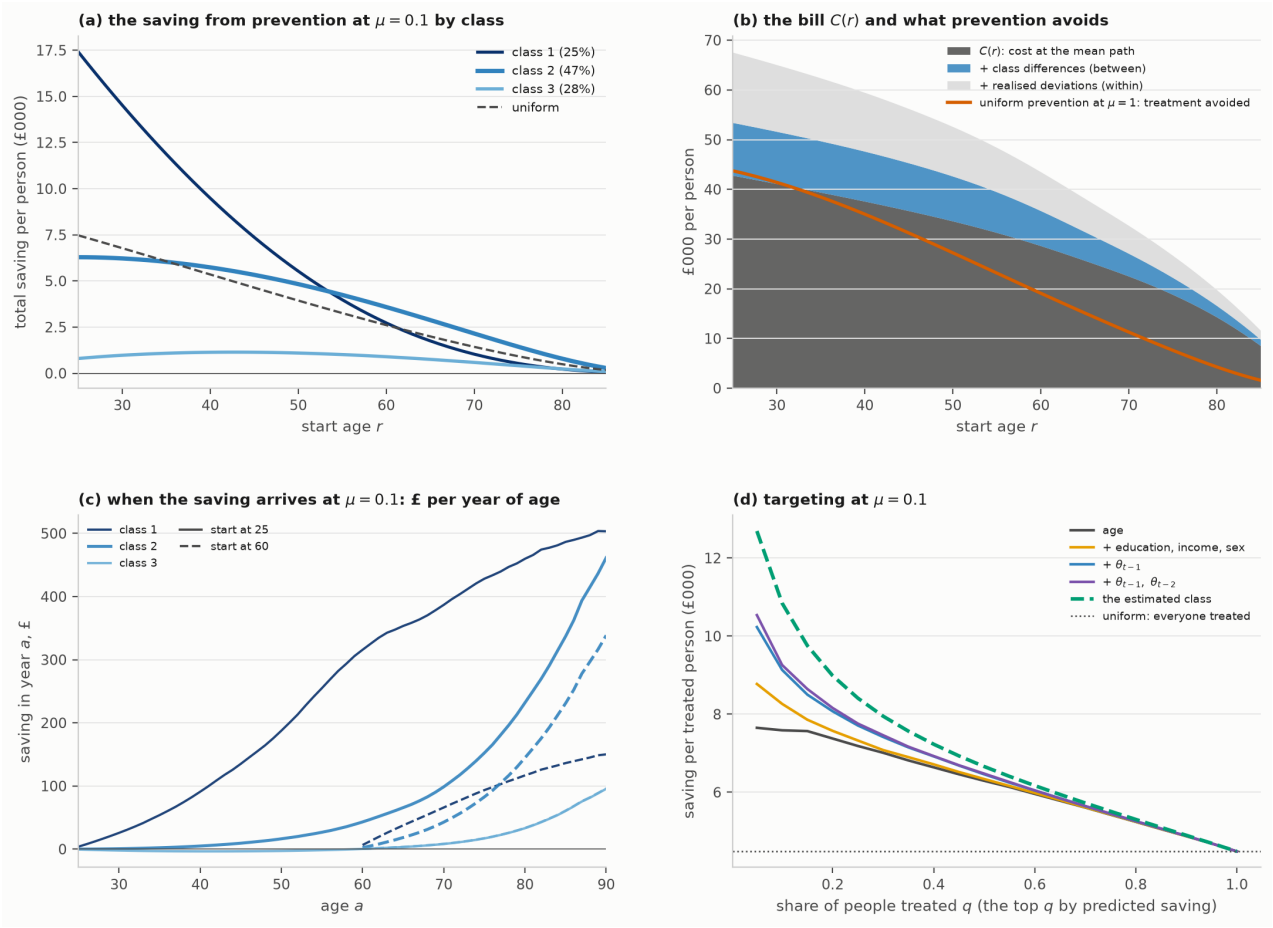


Figure 11: Prevention on the $K = 3$ persistent–transitory classes on θ, in pounds under the estimated cost curve. The savings in (a)–(c) are exact for the cubic cost curve (8). (a) The treatment cost avoided by prevention at $\mu = 0.1$, a tenth of each class's future decline held back, by class and start age r, with the uniform policy dashed. (b) The treatment bill still to come $C(r)$, stacked into the cost at the mean path, what the class differences add and what realised deviations add, with the saving from uniform prevention at $\mu = 1$, the whole decline held back. (c) When the saving arrives: the saving at $\mu = 0.1$ in each year of age a from prevention started at 25 (solid) and 60 (dashed), by class, the per-age terms whose sum is (a). (d) Targeting at $\mu = 0.1$, on the same classes refitted with each person's first two rows held out: the saving per treated person when the top share q by predicted saving is treated, by what the prediction uses (age alone; adding education, income and sex; adding θ_{t-1}; adding θ_{t-1} and θ_{t-2}, where t is the person's first fitted row as in Figure 10), with targeting on the estimated class, the posterior from the person's whole fitted window, dashed and everyone treated dotted. Each person is treated from their age at $t - 1$. The first-order returns $P_j(r)$, the cumulative view of (c) with the present values, and the other targeting rules with the share-of-return view of (d) are in Appendix G.2.

When do we get the savings?

- Figure 11(c) shows that the actual savings of prevention are generally backloaded, but less so for the worst class. The worst class pays back soonest (half by 72 from a start at 25), the middle class later (82), because its decline comes late, and the best class's small return almost entirely in old age (86). The saving per year is still rising at 90, so the horizon matters too.

- Given this mismatch of the cost and benefit of prevention, discounting is likely to bite hard. We haven't looked into this in detail yet. [**TODO:** Could also add the cost of a unit of μ and the survival weighting; both change the timing arithmetic.]

What should we target on?

- The class returns in panels (a)-(c) assume that the class is known, but of course in practice it wouldn't be.
- Figure 11(d) uses the version with the first two observations held out to show the value of targetting based only on known information. The rules are fitted on 70% of them and scored on the other 30%, so the two readings they predict from never shaped those classes.
- In each case it shows the return to prevention at $\mu = 0.1$ when targetting based on information known about that person at time $t - 1$, just before their fitted window starts at t (θ_{t-1}, then θ_{t-1} and θ_{t-2}, as in Figure 10). The estimated class is the posterior from the person's whole fitted window (so excluding the first two observations).
- People are treated from their own age at $t - 1$, not from 25, so the first result is that targetting by age is obviously much more effective than uniform prevention (at 10% coverage, £7,600 per treated person against £4,500 for everyone).
- After age, the health reading is the information that matters: at 10% coverage, adding education, income and sex raises the saving to £8,300 per treated person, θ_{t-1} to £9,100, and θ_{t-1} and θ_{t-2} to £9,300, against £10,800 for the estimated class.
- The remaining gap to the estimated class is what a longer health history buys: the class is estimated from a person's whole fitted window, and two readings close about half of the gap between age alone and it (51%, against 47% for one reading and 20% for the observables).

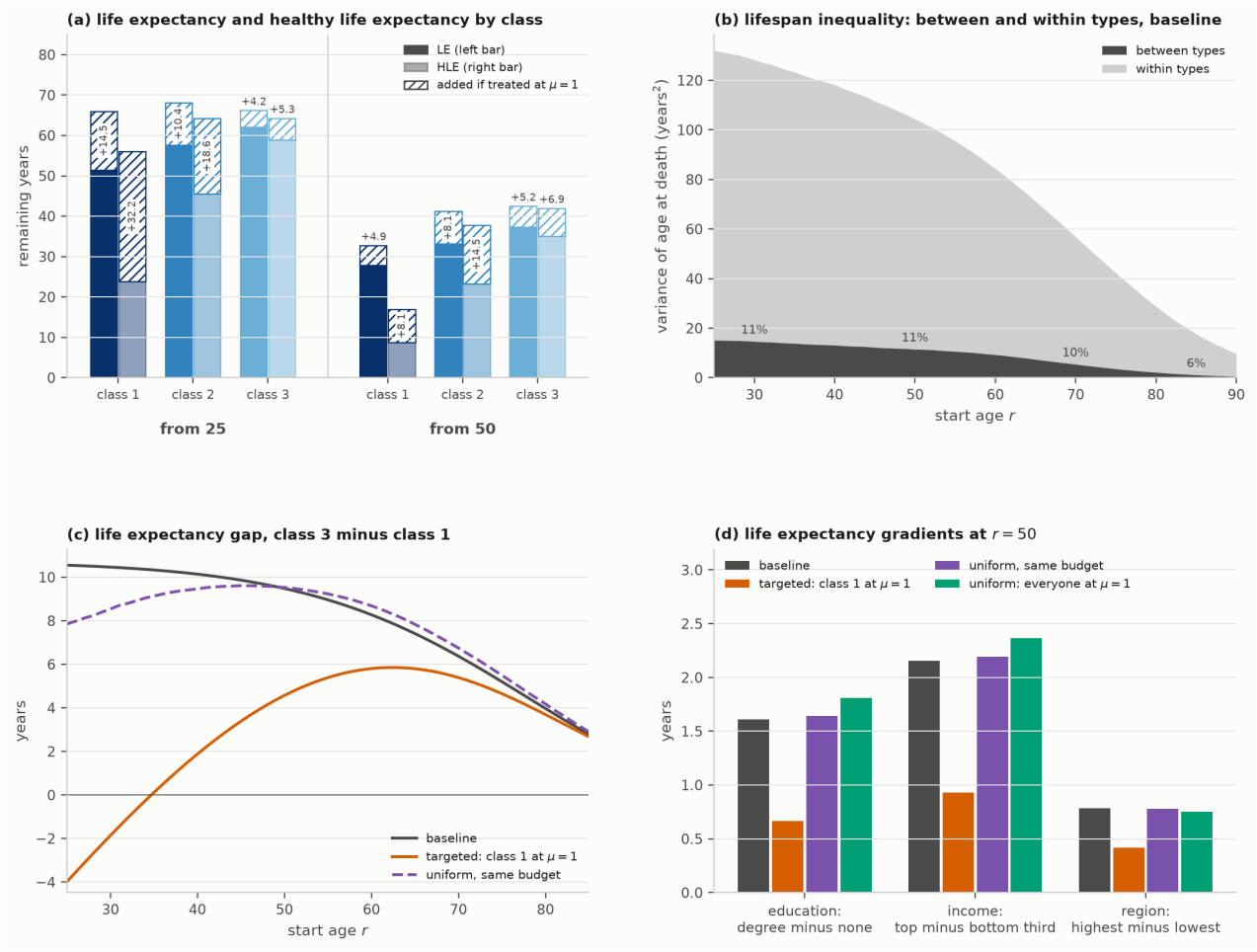


Figure 12: What targeted prevention does to life expectancy and to inequality, on the univariate $K = 3$ persistent–transitory classes on θ, with class hazards estimated for them and calibrated to the ONS life tables. Prevention from r holds back a share μ of the class's mean decline after $r - 1$; mortality and the share in good health respond through their within-age gradients in θ. Policies: targeted, class 1 at $\mu = 1$; uniform with the same budget, everyone at $\mu = \pi_1(r)$. (a) LE and HLE by class from 25 and 50, hatched the gain if the class is treated at $\mu = 1$. (b) The variance of age at death at baseline, between and within types. (c) The LE gap between the best and the worst class. (d) The LE gradients from 50 that the types carry, with everyone treated at $\mu = 1$ added. The detailed version, the version at $\mu = 0.1$ and the multidimensional fit's version are Figures G.9–G.11.

Figure 12 prices the same instrument in (healthy) life years rather than pounds and shows the consequences for lifespan inequality. It is based on the univariate classes of Figure 11, so mortality rates are estimated afterwards. Health moves mortality through the within-age gradient of death in θ (-1.0 log-odds per unit at 55, from the logit of Section 3). The multidimensional fit (Figure G.11) gives wider gaps and the same ranking of policies.

- **(a) Prevention adds more healthy years than years of life.** From 25 the classes expect 51.4, 57.6 and 61.9 more years, 23.8, 45.5 and 58.9 of them in good health. Holding a class's mean health at its level at $r - 1$ adds more HLE than LE in every class: from 50, 8.1, 14.5 and 6.9 healthy years against 4.9, 8.1 and 5.2 years of life; from 25 the worst class gains 32.2 healthy years and 14.5 years of life. Held-back decline keeps people on the "good" side of the self-rated line for

longer than it keeps them alive.

- **(b) Most lifespan inequality is within type.** At baseline the types account for 11% of the variance of age at death from 25 and from 50, 10% from 70 and 6% from 85; the rest is the randomness of when people die within a type. Targeting the worst class from 50 cuts the between-type share to 4%, but the total barely moves (a standard deviation of 10.3 years against 10.2, Figure G.9(d)), because a policy that lengthens lives also spreads them out.
- **(c) Targeting the worst type narrows the gap between types; the same budget spread evenly does not.** From 50, treating the worst class at $\mu = 1$ takes the LE gap from 9.5 years to 4.6 (the HLE gap from 26.2 to 18.1), while the same budget spread evenly ($\mu = 0.23$ for everyone) leaves both where they are, even though it adds slightly more years in total (1.4 against 1.1). From 25 the targeted policy overshoots, holding the worst class flat for 65 years and taking it past the best (-4.0 years), so $\mu = 1$ from 25 is an upper bound rather than a policy; from 70 neither policy does much.
- (d) Targetting prevention on the worst type can more than halves socioeconomic gradients. Around 50, worst-class membership runs from 37% with no qualifications to 18% among graduates and from 38% in the bottom income third to 13% in the top, so the types carry LE gradients of 1.6 years by education, 2.2 by income and 0.8 across regions, a fraction of the published UK gradients. Treating the worst class cuts these to 0.7, 0.9 and 0.4; the even spread leaves them unchanged, and treating everyone at $\mu = 1$ widens the first two slightly. Ranking income within five-year age bands, so that the income thirds cannot pick up the age profile of income, gives 2.1 years against 2.2. [**TODO:** The gradients here are the composition channel only; add the within-class gradients (class-specific LE by education and income, or a Gompertz fit by group) to say what share of the published gradients runs through the types. Also the cost side: a unit of μ for the worst class may cost more than for the others.]

6 Conclusions

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Appendices

A Proofs and Additional Material for the Conceptual Framework

[Include proofs for any propositions and perhaps some additional illustrative figures here.]

B The Health Measure

B.1 Health Variables

Table 1 lists the eight items. The six SF-12 physical items enter as four testlets: the two physical-function items sum into PF, the two role-physical items sum into RP, and general health and pain enter alone. *[add explanation of relationship to PCS here.]* Every item is coded so that category 1 is worst health. The impairment counts come from the UKHLS question on which areas a person's health limits them in. Eight of the areas are physical, in three groups:

1. LIM_FL (0–4): mobility (`disdif1`), lifting (`disdif2`), dexterity (`disdif3`), co-ordination (`disdif10`);
2. LIM_SELF (0–2): continence (`disdif4`), personal care (`disdif11`);
3. LIM_SENS (0–2): hearing (`disdif5`), sight (`disdif6`).

In the early waves the battery was asked only of people reporting a long-standing illness, so someone who reports none has no answer rather than a zero. We count them as having no limitation in every wave, so that the variable means the same thing across the panel; someone who reports a long-standing illness but gives no usable answer is missing. Two areas are left out: "recognising when in physical danger" belongs with the mental items, and "other health problem or disability" changes nothing when it is added. COND counts the sixteen ever-diagnosed physical conditions of the UKHLS inventory. A count's top category is merged into the one below while it holds less than 0.5% of person-waves, a rule applied to the sample before any fitting, so COND runs from 0 to 5 and 6 or more (6 or more holds 0.86%).

Table B.1 shows the distribution of each item. In total, 7.1% of person-waves give the best answer to all eight items, while only 12 person-waves sit at the worst answer on every item.

Table B.1: Unconditional distribution by health variable

	k	share of person-waves, best category first (%)								
GH	5	13.29	33.33	31.71	15.58	6.10				
PF	5	65.13	11.66	11.82	4.60	6.79				
RP	9	52.18	6.85	12.77	4.55	11.57	2.68	4.76	1.20	3.44
BP	5	53.72	25.24	9.14	8.26	3.64				
LIM_FL	5	83.05	6.66	5.30	3.24	1.76				
LIM_SELF	3	94.15	4.74	1.12						
LIM_SENS	3	95.62	3.82	0.56						
COND	7	48.28	28.88	12.90	5.59	2.44	1.05	0.86		

B.2 The Graded Response Model

[Fill in more details on the GRM here] Section 3 sets out the model and how it is estimated and scored. Table B.2 reports the estimated item parameters, Figure B.1 maps θ to h, and Table B.3 correlates the four measures with the published alternatives.

Table B.2: Graded response model item parameters: the number of categories k, the discrimination a and the category thresholds, on the pooled sample.

Item	k	a	thresholds							
GH	5	2.03	−2.10	−0.98	0.16	1.45				
PF	5	3.52	−1.73	−1.38	−0.80	−0.40				
RP	9	3.09	−2.20	−2.03	−1.56	−1.37	−0.81	−0.64	−0.23	−0.03
BP	5	2.31	−2.35	−1.50	−0.99	−0.09				
LIM_FL	5	3.38	−2.49	−1.94	−1.46	−1.07				
LIM_SELF	3	2.43	−2.95	−1.99						
LIM_SENS	3	1.36	−4.49	−2.85						
COND	7	1.18	−4.61	−3.90	−3.12	−2.26	−1.25	0.11		

[Explain h vs θ figure here]

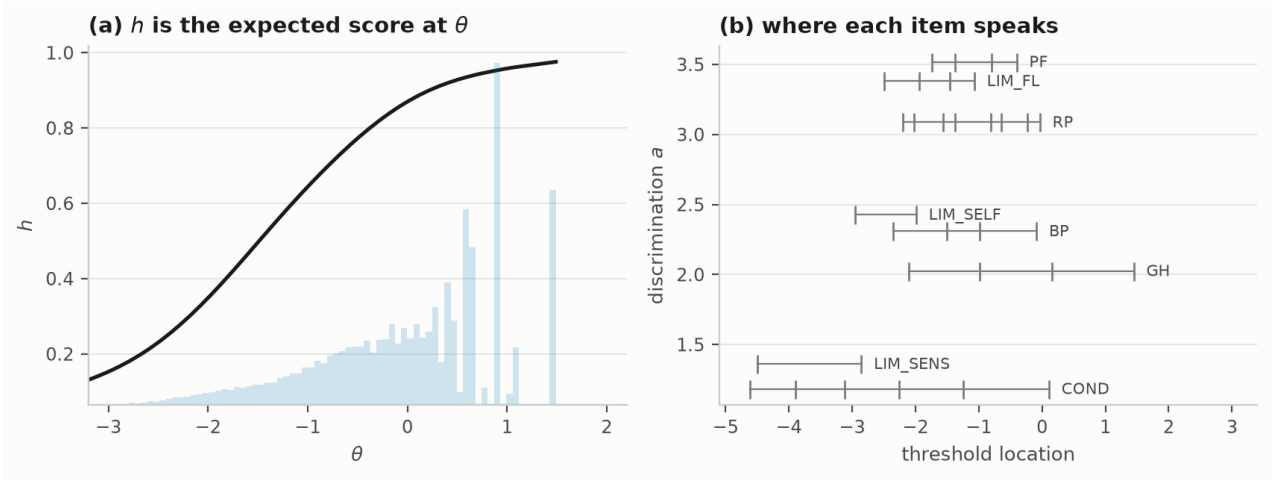


Figure B.1: The measure. (a) The expected-score curve mapping θ to h, over the pooled distribution of θ; (b) each item's discrimination against its thresholds.

	θ	h	frailty index (negated)	log frailty (negated)
UKHLS PCS	0.903	0.935	0.859	0.837
UKHLS MCS	0.274	0.266	0.232	0.228
chronic count (negated)	0.576	0.587	0.785	0.763
Spearman, vs θ	1.000	1.000	0.920	0.920

Table B.3: Correlations of θ, h, the frailty index and log frailty with the comparison metrics, 385,427 person-waves. The two frailty indices are negated, so higher is healthier on every column.

B.3 The Frailty Indices

The frailty index counts 31 deficits, each scored 0 to 1 and weighted equally: the six SF-12 physical items (general health, moderate activities, climbing stairs, accomplished less, limited in kind, pain), graded by response step; the eight physical impairment areas of Appendix B.1 (mobility, lifting, dexterity, co-ordination, continence, personal care, hearing, sight), each 0 or 1; and the 17 ever-diagnosed conditions of the UKHLS inventory (asthma, arthritis, congestive heart failure, coronary heart disease, angina, heart attack, stroke, emphysema, hyper- and hypothyroidism, chronic bronchitis, liver condition, cancer, diabetes, epilepsy, high blood pressure, clinical depression), each 0 or 1. A person-wave is scored only when every deficit is observed. The index is the share of deficits present, so higher is frailer. 7% of person-waves have none: 12% in the twenties, 6% in the fifties and under 1% past 80. Log frailty, $\log(\text{frailty} + 1/31)$, keeps these zeros at the floor (Section 3).

The index differs from that of Hosseini et al. (2022) in three ways. They score 28 deficits, all 0/1, including high BMI, smoking and mental and cognitive impairments; they exclude self-rated health; and they model the zeros with a separate probit rather than shifting them.

C The Cost Proxy

Section 3 sets out the index and its unit costs. This appendix lists its inputs, shows that the convexity the returns rest on survives every costing choice, and checks the index against what is known about NHS spending.

C.1 Inputs

Table C.1 lists the UKHLS variables. The utilisation sample is every person-wave that answered the utilisation block, with or without a health score: 301,390 person-waves on 59,750 people. Every input is self-reported over a twelve-month recall window. Two choices are not spelled out in Section 3. First, UKHLS records the nights spent in hospital but not the number of stays, so 20 nights might be one long admission or four short ones. With every night priced the same this matters only for the two-part price below, which charges a fixed cost per stay. Second, the top band of GP and out-patient contacts, "more than ten", is open-ended and scored at 15. Both are varied in Figure C.1.

Table C.1: UKHLS variables behind the cost proxy, waves 7–15.

Quantity	Source	Coverage
Any admission	`hosp`	complete for respondents
Nights in hospital	`hospd`	all admitted; median 3, mean 7.5, max 300
Maternity flag	`hospch`	12.7% of admissions, mean age 31.7; left out
GP visits, banded	`hl2gp`	0 / 1–2 / 3–5 / 6–10 / more than 10
Out-patient attendances, banded	`hl2hop`	same bands

C.2 Sensitivity of the Convexity

Figure C.1 varies every costing assumption the index rests on. All twelve variants are convex in pounds on θ, with person-clustered t between 17 and 36. The grid crosses six pricings with winsorising at the 99th percentile. The pricings are every night at the bed-day cost (the index); a fixed cost per

stay plus £350 a night, with one stay a year or one per five nights; maternity stays included; the top band at 30 contacts; and condition-weighted nights. The condition-weighted variant prices the nights UKHLS attributes to a condition (18% of admissions over waves 7–15) by the condition's cost group, and gives the unattributed nights the average weight of the attributed ones within the same decile of θ, never a global average, which would flatten the gradient the index is for.

Winsorising roughly halves the magnitude (the quadratic term falls from £443 to £207) and changes nothing else. The sickest-to-healthiest decile ratio ranges from 10.5× to 20.8×, and the pricing of long stays moves it most: 18.6× when every night costs £791, 13.9× under the two-part price, and 20.0× when a year's nights are read as one stay per five. Across the health measures the bend per standard deviation differs but its sign does not: the quadratic term is £443 on θ, £282 on h, £517 on log frailty and £182 on the frailty index (the t-statistics are in Section 3).

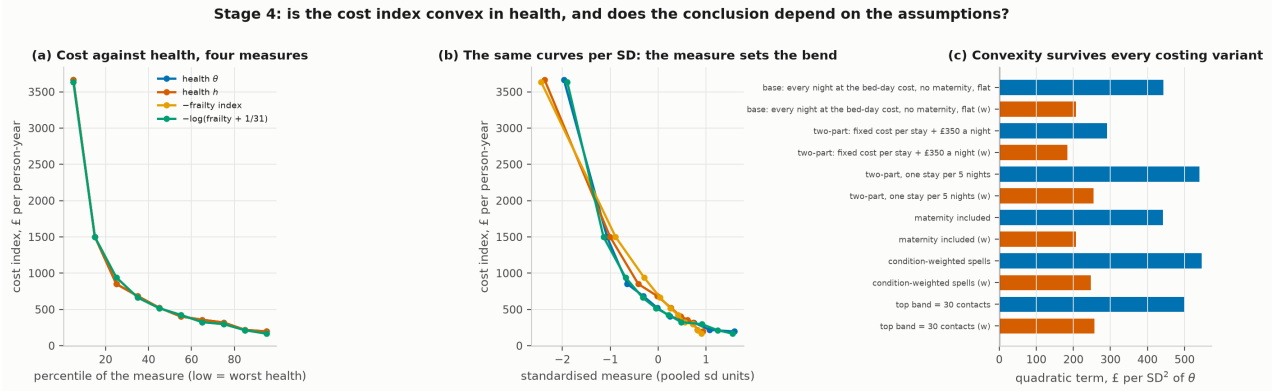


Figure C.1: Cost index in pounds against the four health measures (θ, h, and the frailty index and log frailty negated): by decile, per standard deviation of each measure, and the sensitivity of the convexity to every costing assumption the index rests on. Measures oriented so higher means better health. Panel (c) varies the pricing of stays (every night at the bed-day cost, or a fixed cost per stay plus £350 a night), the spell rule, maternity treatment, condition weighting and top-band value, each with a winsorised-at-the-99th-percentile twin.

C.3 Validation against Published Profiles

Figure C.2 makes three checks against what is independently known about NHS spending, none of which the index was fitted to, on ages 20–90 (283,852 person-waves).

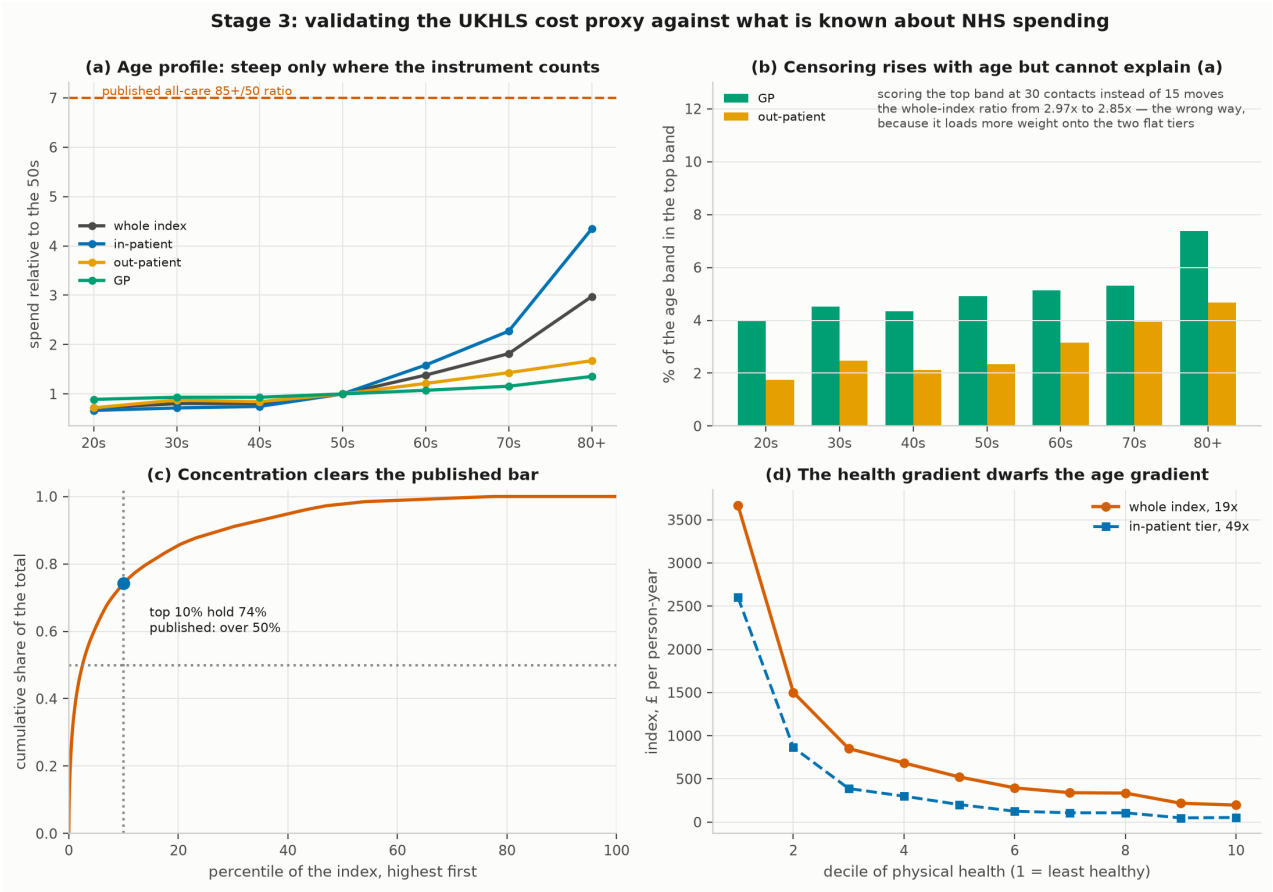


Figure C.2: Validation of the flat-rate index. Panel (a) the age profile of the whole index and of each tier, relative to the 50s, against the published all-care ratio; (b) the share of each age band top-coded at "more than ten" contacts, with the top-band sensitivity; (c) the concentration curve against the published top-decile share; (d) the gradient in physical health, the quantity the index is for.

- **Concentration passes.** The top 1% of person-waves hold 36.2% of the total, the top 5% 61.5% and the top decile 74.2%, against a published benchmark of more than half. If anything the index is over-concentrated, which is expected: 22.7% of person-waves are exactly zero on an index that carries no prescribing, A&E or community contact, the components that would give almost everyone a small positive amount.
- **The level undershoots, as it should.** The index averages £908 per person-year (£510 in-patient, £290 out-patient, £109 GP) against NHS spending near £3,000 per head. Three tiers of contact priced at national averages cannot add up to the national total.
- **The age profile is too flat.** The index puts the 80+ band at 3.0× the 50s, against a published all-care ratio near 7×; by tier, in-patient 4.3×, out-patient 1.7×, GP 1.4×. The one tier counted rather than banded is the one with a credible gradient. Top-coding is not the reason: it does rise with age (7.4% of the 80+ band report more than ten GP visits against 4.9% of the 50s), but scoring the top band at 30 rather than 15 moves the ratio from 2.97× to 2.85×, the wrong way. The binding limitation is that a contact costs the same at every age: an out-patient attendance enters at £191 for an 85-year-old and a 30-year-old alike.

The index is built for the gradient in health, and there it is strong: 18.6× between the least and the most healthy decile on the whole index and 49.0× on the in-patient tier, against an age gradient of

3.0×. Because the age profile is understated by more than half, the index should not be used on its own for anything that turns on cost by age, such as a lifetime cost projection. The returns of Section 5 price health through one curve pooled across ages, so they inherit the understatement only where cost at a given level of health rises with age, mainly in the 75–90 band, which sits about a third above the pooled curve (Appendix G.1).

D Additional Descriptive Results

D.1 Robustness of the Moments

Figure D.1 redraws the covariance rows of Figure 2 on balanced samples, and Figure D.2 passes the cost index through the same moments.

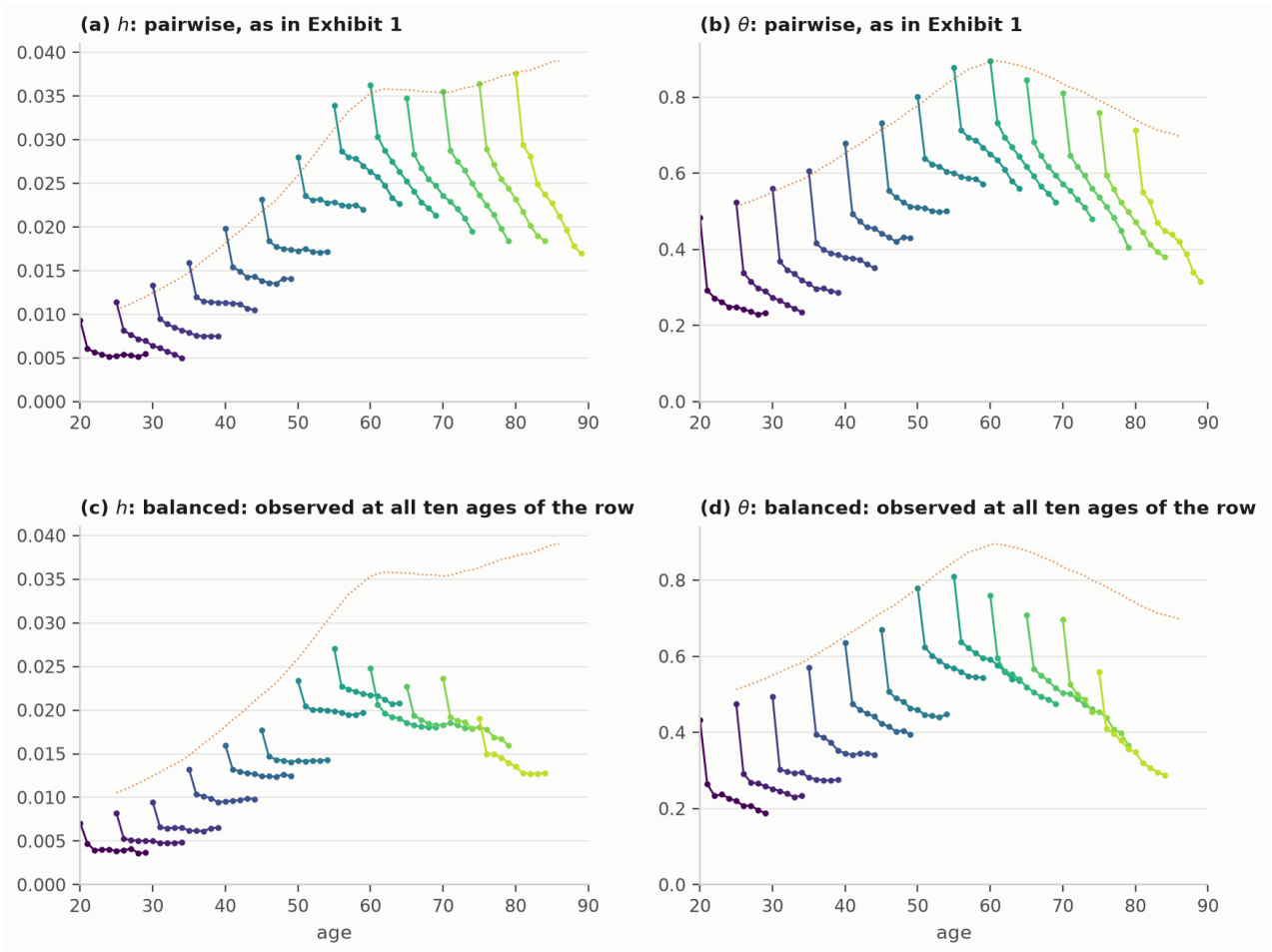


Figure D.1: The covariance rows of Figure 2 on pairwise and on balanced samples. Top: each cell over the people observed at both of its ages, as in Figure 2. Bottom: each row over the people observed at every one of its ten ages, which keeps between 719 and 4,757 people per row and nothing from age 80. Dotted red is the smoothed variance profile. The balanced rows sit below it throughout, and flatten after 60 on h: people observed ten years running are healthier and more stable than the cross-section at the same age. The first-year drop and the sign of the rows' slopes are the same on both.

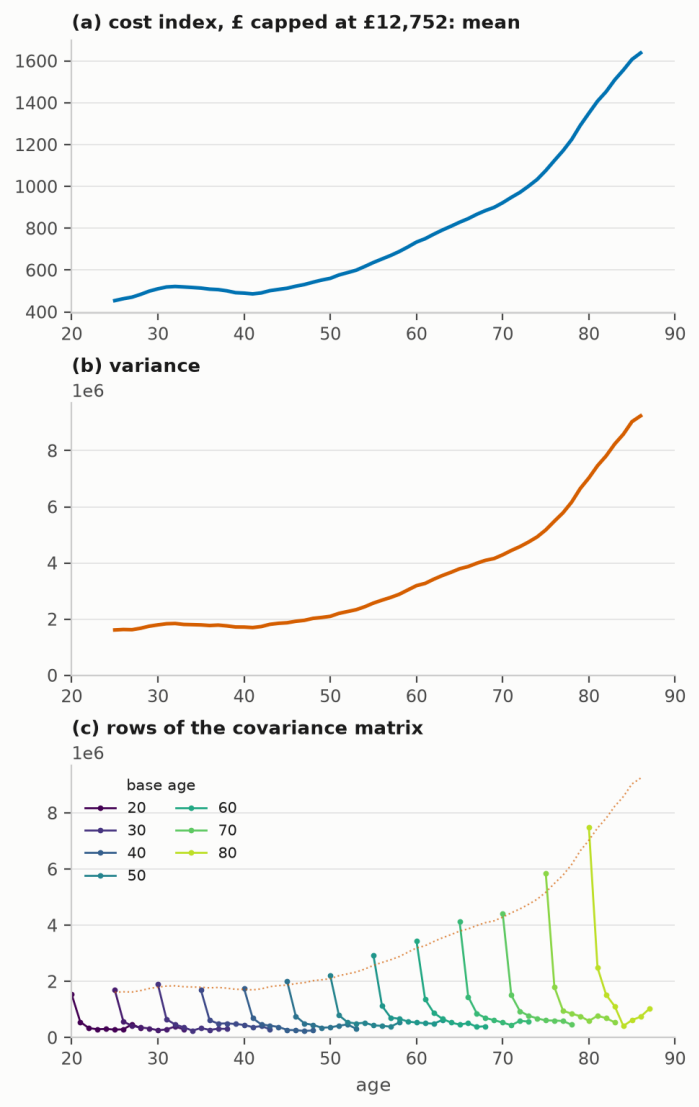


Figure D.2: The cost index through the moments of Figure 2, in pounds capped at the 99th percentile; waves 7–15, rows run eight years.

- Cost is what $c(h)$ stands for, so its mean by age is the bill $C(r)$ sums directly: the mean is flat to 40 and then rises steeply, from about £450 to £1,640 by 86; its variance rises nearly sixfold.
- The rows say what kind of variance it is: every row drops to about a third of the diagonal within a year and declines slowly after. Realised cost is almost entirely a one-period draw around the health state, which is exactly the within-type, in-the-moment part of the bill (Result 1) that is treated and that prevention cannot reach (Result 2).

D.2 The Linear Approximation

Figures D.3 to D.5 vary the exercise of Figure 3: Case 1 on all four measures, the implied $\mathrm{Corr}(H, d)$ under every case and design, and the two-year design.



Figure D.3: Figure 3 under Case 1, transitory only, on θ, h, the frailty index and log frailty (rows), in the design of Figure 3.

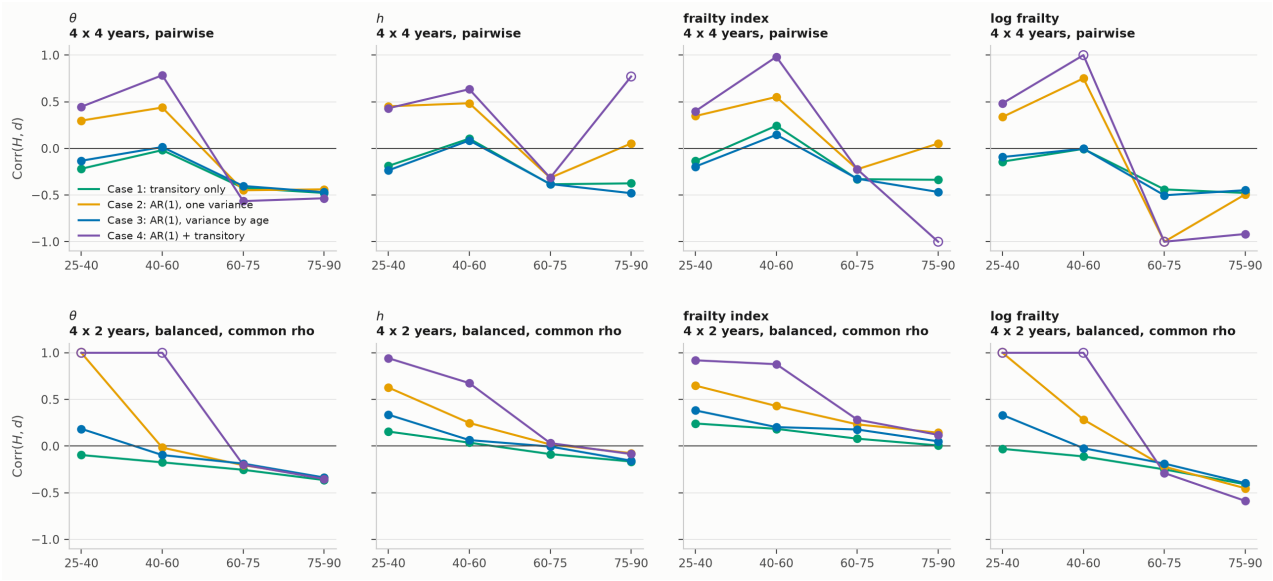


Figure D.4: Summary of Figure 3: the implied $\text{Corr}(H, d)$ by age band under Cases 1 to 4, for θ, h, the frailty index and log frailty. Top: the main design, four ages four years apart on pairwise moments. Bottom: four ages two years apart on balanced moments, with one ρ shared by the four bands. Open circles mark bands where the admissibility constraint binds, or where V_H is below 5% of the first variance.

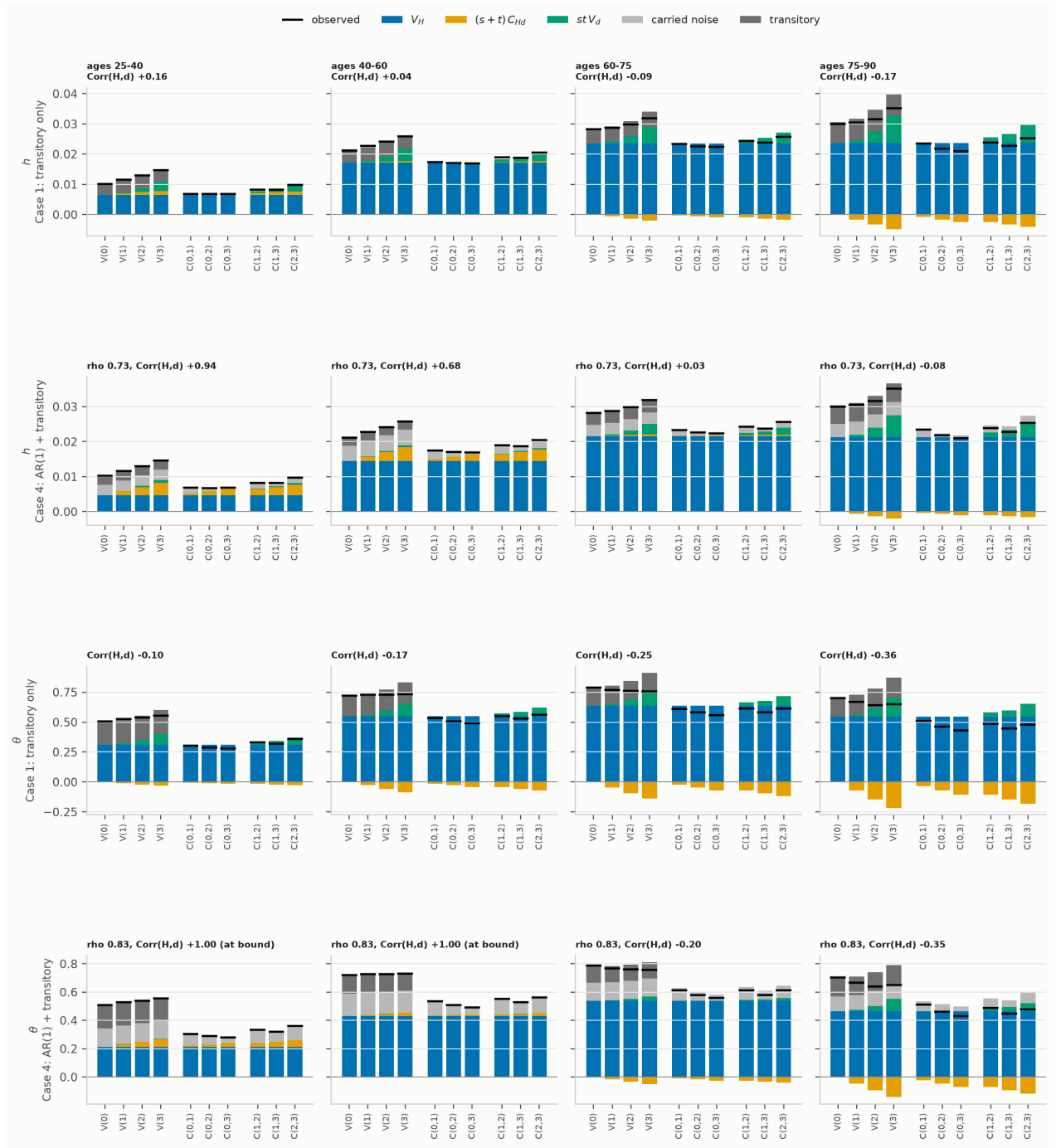


Figure D.5: Figure 3 on the two-year design: four ages two years apart, balanced moments, Cases 1 and 4 on h and θ, with one ρ shared by the four bands in Case 4. On h this is admissible in every band with nothing imposed.

E Additional K-Means Results

E.1 The Types on Predicted Cost

Figure E.1 clusters on each person's predicted cost rather than on health.

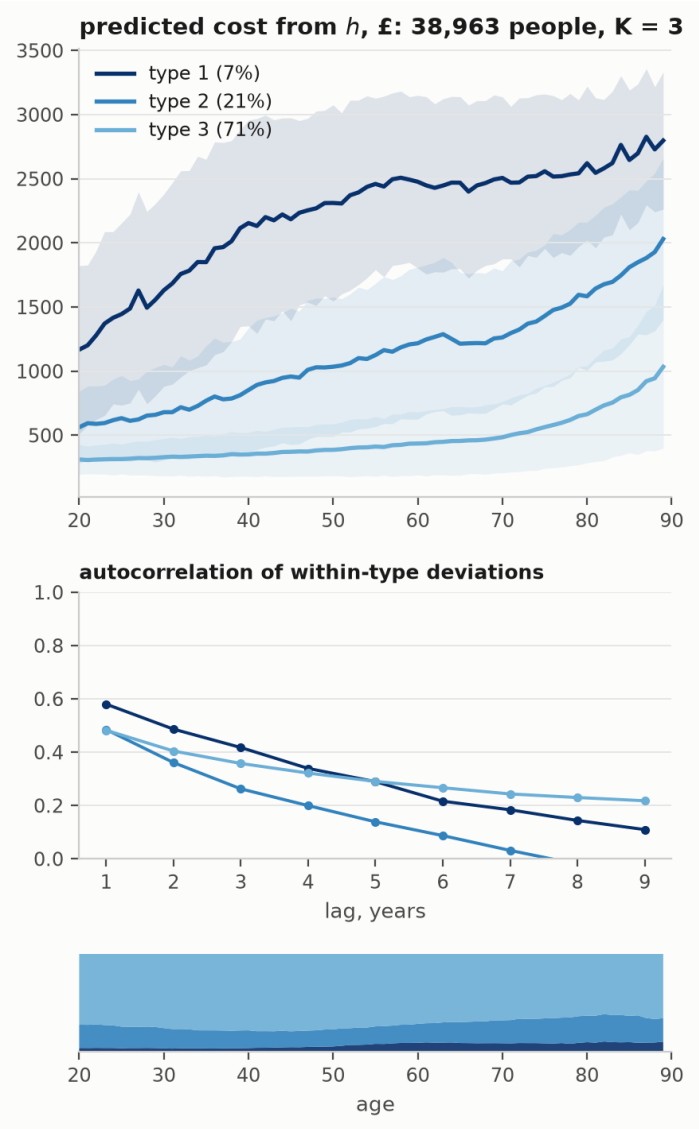


Figure E.1: As Figure 4, for partial K-means on each person's predicted cost: type means with the within-type ±1 sd band, the autocorrelation of within-type deviations by lag, and the type composition by age. Predicted cost is the pooled cost curve on h, a cubic in the standardised score fitted to costs capped at the 99th percentile over waves 7–15 and made monotone, evaluated at every contract row: a cost-anchored cardinalisation of the measure. It lands on the bounded side of the h–θ contrast: ARI 0.65 with the h types and 0.31 with the θ types.

E.2 Types over Age Windows

Figure E.2 refits partial K-means on the age windows of Figure 8.

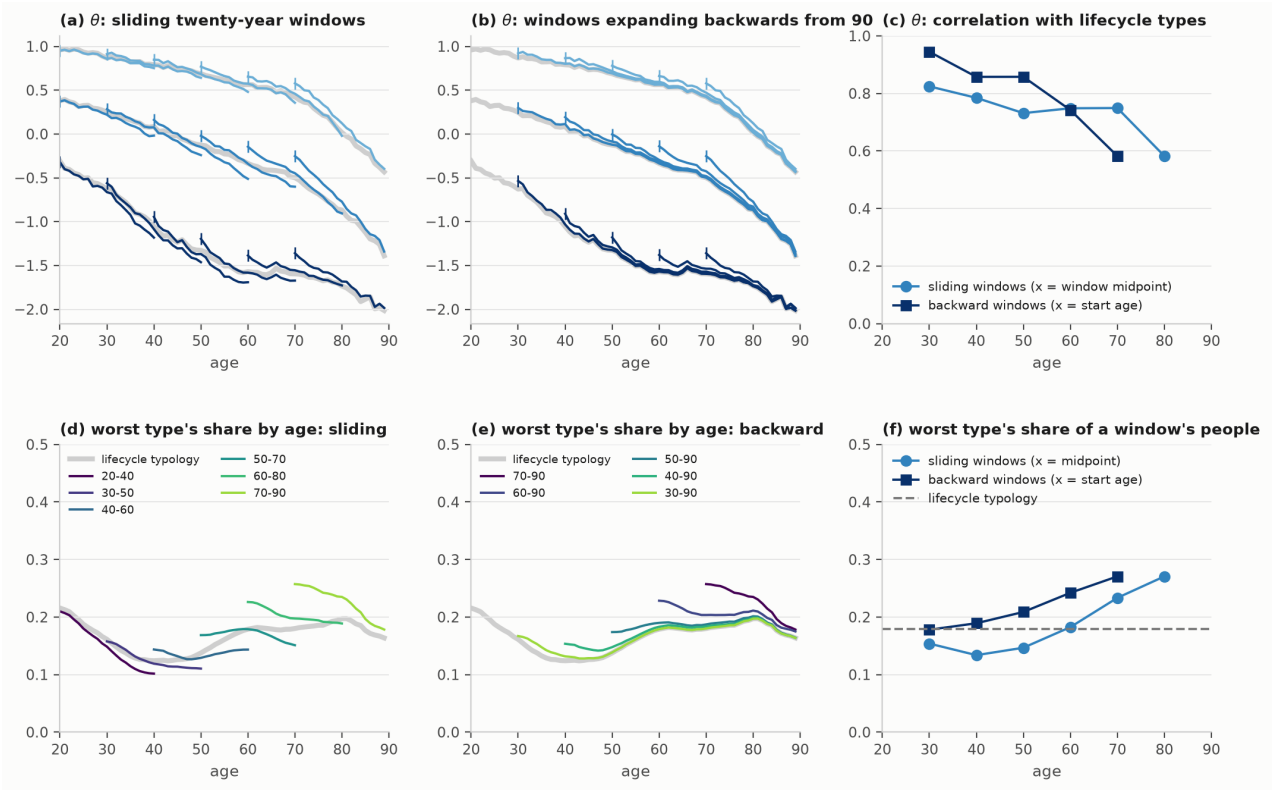


Figure E.2: As Figure 8, for partial K-means: types refitted on sliding twenty-year windows (a) and windows expanding backwards from 90 (b), against the lifecycle typology in grey; (c) the co-assignment correlation with the lifecycle assignment, here the correlation of the two same-type indicators over pairs of people. Lower row: the share of the person-waves observed at each age that each window assigns to its worst type (d, e), and the worst type's share of each window's people (f). The same figure on h is in Appendix I.

- Partial K-means refitted on the same windows is more stable than the mixture: the sliding windows agree with the lifecycle typology at 0.73–0.83 up to 60–80 and 0.58 at 70–90, the backward windows at 0.74 from 60 and 0.95 from 30. What a late window sees are the types after the divergence phase: the worst type's share of a window's people rises from 15% (20–40) to 27% (70–90) as the window moves, and from 18% to 27% as the backward window shrinks from 30–90 to 70–90.
- The lower row says that this rise is composition, not a growing bad type. Within the ages two windows share, the worst type's share of the person-waves observed at each age is nearly the same whichever window assigned it: the backward windows starting at 30, 40 or 50 sit on the lifecycle typology's line (about 13% at 45, 18% at 65, 19% at 75), so the rise is the window's people getting older. Only the windows confined to late life relabel: a window starting at 60 or 70 puts 21–25% of the person-waves at 65–75 in its worst type against the lifecycle typology's 18–19%, picking up people who were middle type for most of their lives and decline late.
- Set against Figure 8, the contrast is the point. K-means classifies on person-by-age profiles, so a window's worst type is always the lowest profiles in that window and its share follows the age mix; the mixture classifies on paths with persistence, so its worst class needs the divergence inside the window and can shrink to a small very sick group or grow to a third of people as the window moves. The stability of K-means across windows is its insensitivity to where the decline happens.

F Additional Bayesian Results

F.1 The Worst Type by Method and Number of Classes

Figure F.1 and Table F.1 compare the worst type across the two methods and across K.

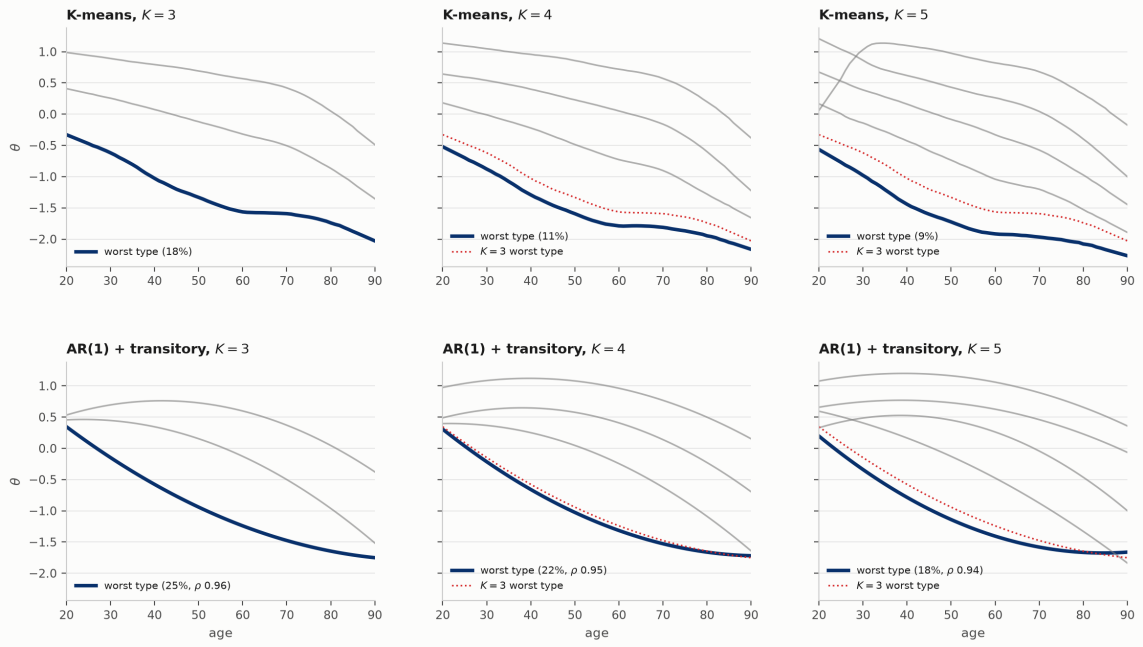


Figure F.1: The worst type on θ under partial K-means (top) and the persistent–transitory mixture (bottom) at $K = 3$, 4 and 5, drawn in colour with the other types in grey and the $K = 3$ worst type dotted in red. K-means paths are ten-year rolling means of the type means, mixture paths the quadratic class paths; the mixture's share in the legend is probability mass, Table F.1 gives the modal share too.

method	K	share	θ_{20}	θ_{50}	θ_{80}	slope to 50	sd at 50	ρ	of K=3 worst	worst under other
K-means	3	18%	-0.33	-1.33	-1.74	-0.033	0.66		100%	85%
	4	11%	-0.52	-1.59	-1.95	-0.036	0.59		63%	92%
	5	9%	-0.57	-1.72	-2.08	-0.039	0.55		48%	94%
AR(1) + transitory	3	18% (25%)	0.35	-0.94	-1.65	-0.043	0.81	0.96	100%	83%
	4	15% (22%)	0.31	-1.03	-1.66	-0.044	0.80	0.95	84%	67%
	5	14% (18%)	0.20	-1.14	-1.67	-0.045	0.79	0.94	76%	57%

Table F.1: The worst type by method and K: its share (mixture: modal, with the probability mass in brackets), its path, its slope to 50 in θ per year, the within-type sd at 50 (K-means: among members at 48–52; mixture: the stationary within-class sd), the mixture's persistence, the share of the $K = 3$ worst type still worst at this K, and the share of this method's worst type that is worst under the other method at the same K.

- **The error structure changes the shape of the worst type's path, not its membership.** Of the K-means worst type, 85% are in the mixture's worst class at $K = 3$, 92% at $K = 4$ and 94% at $K = 5$. Yet the two paths differ: the mixture's starts near the average (+0.35 at 20) and declines at 0.043 a year to 50, the K-means type starts a unit lower (−0.33) and declines at 0.033, because the persistent component lets the mixture read a low level at 20 as a carried deviation.

The mixture's worst class is also the wider one within (sd 0.81 against 0.66 at 50) and its members are held with uncertainty: 45% of its modal members have a posterior above 0.9, at every K.

- **The number of classes refines who is in the worst type and tightens it, but leaves its mean shape alone.** Under K-means the worst type halves from 18% to 9% by $K = 5$, keeping 63% and then 48% of the $K = 3$ members, its within-type sd falls from 0.66 to 0.55, and its path keeps the same shape shifted down by a quarter to a third of a unit. Under the mixture the worst class moves from 18% to 14% by modal label, keeps 84% and 76% of its $K = 3$ members, and its path, persistence and within-class spread barely move; the added classes split the middle and the healthy. The two methods converge on the same core: the $K = 5$ K-means worst type is almost entirely inside the mixture's worst class.

F.2 Which Type Drives the Level–Slope Reversal?

Figure F.2 splits the level–slope covariance across types into each type's contribution.

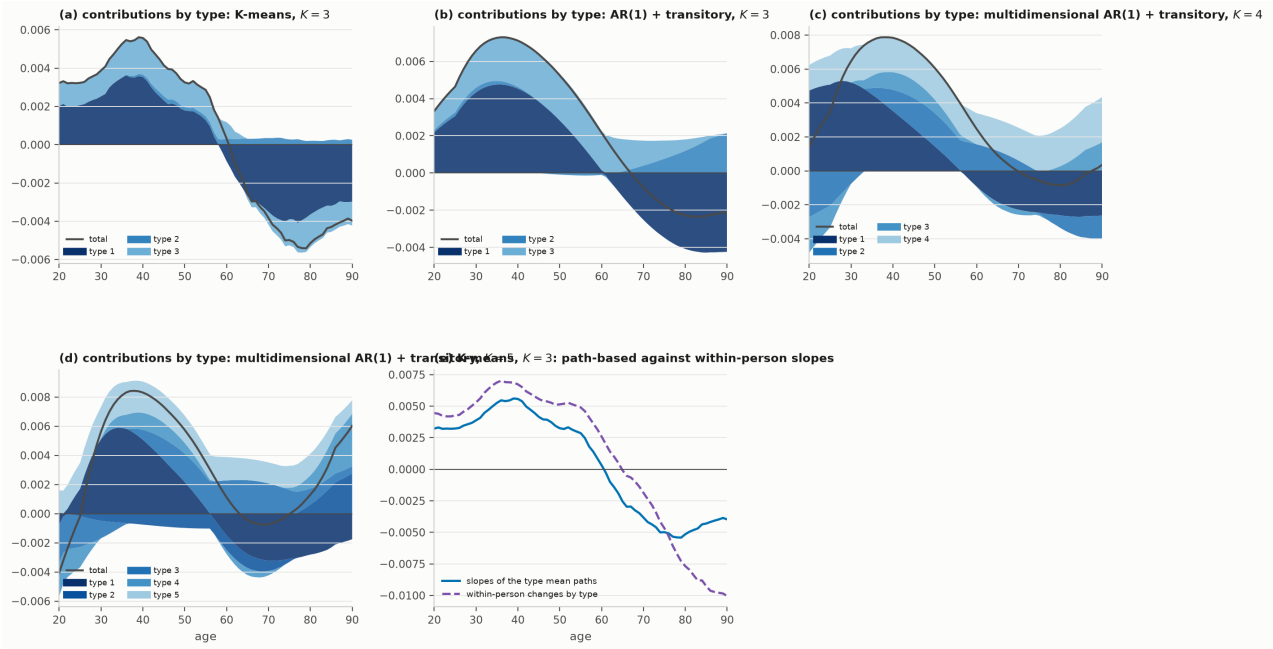


Figure F.2: The level-slope covariance across types split into each type's term $\pi_j(a)(\bar{h}_{j,a}-\bar{h}_a)(d_{j,a}-\bar{d}_a)$, age-specific composition shares, stacked: (a) the K-means types and (b) the persistent–transitory classes at $K = 3$; (c), (d) the multidimensional mixture's physical channel at $K = 4$ and 5. A type's term is negative when it sits below the mean and declines more slowly than the mean, or above and faster. (e) The K-means covariance with slopes taken from within-person changes rather than the type mean paths, which removes composition and selection from the slopes. Ten-year rolling means.

- **The reversal is the worst type's own convergence.** Its term carries nearly all of the negative covariance after 60 under every typology and every K (K-means -3.5 of -3.8 at 70, in thousandths; the univariate mixture -2.5 of -0.8, with the healthy class still diverging). With slopes from within-person changes the K-means covariance still turns negative, at 65 rather than 61, and more steeply after 75: the worst type's members do slow their decline relative to the others as they age, over and above the composition and mortality selection that flatten its cross-sectional path.
- The same holds in the moments of Figure 3: refitting Case 4 without the bottom quintile of the

person's mean age-standardised θ, or without either method's worst type, leaves $\mathrm{Corr}(H, d)$ at or near zero in the two bands after 60; the decile is not enough. The same fifth holds most of the level variance, so "no reversal without them" and "little between-person variance without them" are partly one statement.

F.3 Birth-Decade Cohort Effects

Section 4 checks the typology for cohort effects in two ways: within partial K-means with a fully flexible age profile, and within the mixtures with one level shift per birth decade, common to the classes. Neither changes the typology. They disagree about the shifts themselves, and the flexible version is the one to believe.

Within partial K-means. We regress the score on single-year age dummies and birth-decade dummies, with the 1950s as reference and standard errors clustered on the person. With age fully flexible a decade shift is identified only from the overlap of neighbouring decades at the ages they share. We then subtract each person's decade shift from every one of their scores, rerun partial K-means with $K = 3$ and 50 seeded starts on the adjusted profiles, and compare the two typologies person by person (Figure F.3). The shifts are small for every cohort born before 1970, at most 0.04 standard deviations of θ, and negative from the 1970s on: -0.11, -0.21, -0.28 and -0.35 for the 1970s to the 2000s. Those cohorts are observed only when young, so a cohort effect and a period effect cannot be told apart there. The typology is unaffected: the adjusted Rand index between the adjusted and unadjusted types is 0.903 on θ and 0.956 on h, at least 97% of people keep their type, and no type's share moves by more than half a percentage point.

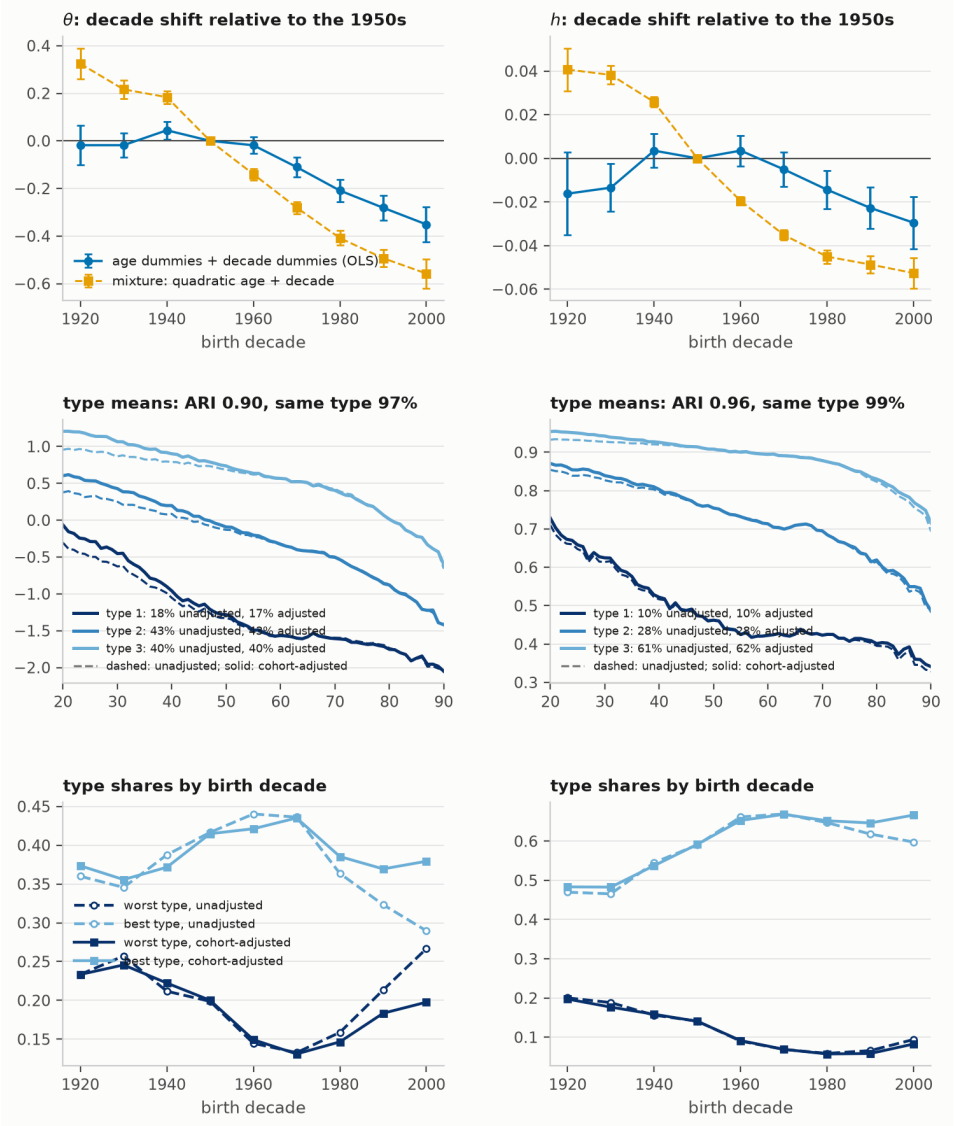


Figure F.3: Cohort adjustment within partial K-means, one column per variant. Top: birth-decade level shifts relative to the 1950s from OLS on single-year age dummies plus decade dummies, with 95% person-clustered intervals, against the mixture's quadratic-plus-decade estimates. Middle: K-means type means by age, on the unadjusted scores (dashed) and on the scores net of the OLS shifts, re-clustered (solid). Bottom: the share of people in the worst and the best type by birth decade, unadjusted (dashed, open circles) and cohort-adjusted (solid, filled squares).

Within the mixtures. Figure F.4 adds the shared decade shift to the single-channel persistent–transitory fits. Their quadratic age path attributes shifts to the earlier decades as well, which is what a quadratic does when the age profile is not quadratic (Section 4). Figure F.5 shows the multidimensional fit without the shifts, and Figure F.6 compares the multidimensional fits with and without them at $K = 4$ and 5.

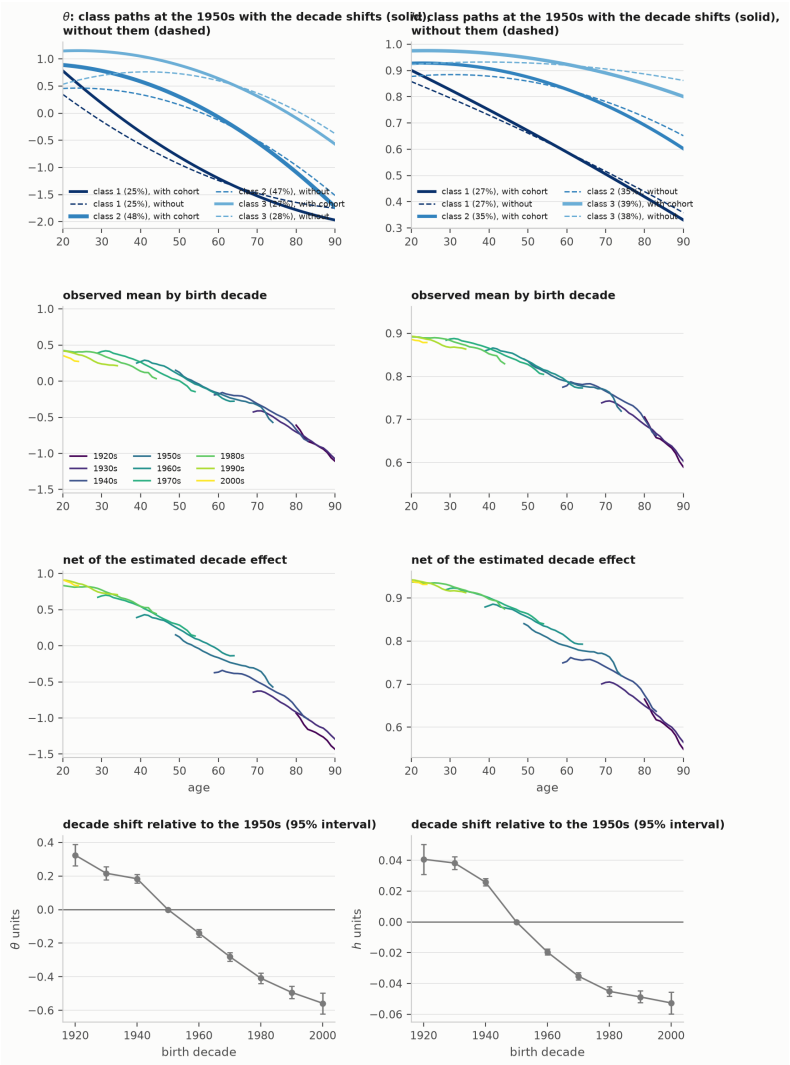


Figure F.4: The persistent–transitory fits with a shared birth-decade level shift, one column per variant. Row 1: class paths with the shifts, evaluated at the 1950s (solid), against the fits without them (dashed). Row 2: the observed mean of the variant by age within each birth decade. Row 3: the same profiles net of the estimated decade shift. Row 4: the shifts relative to the 1950s with 95% posterior intervals, in the variant's units. Contract people, cells of at least 50 person-waves.

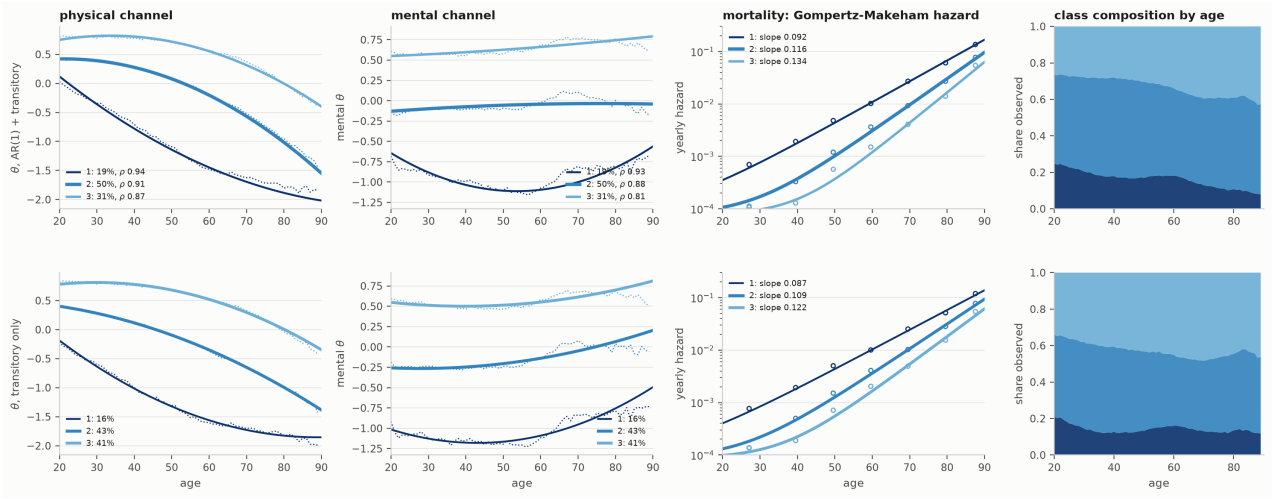


Figure F.5: As Figure 7, the multidimensional $K = 3$ mixtures on θ without the birth-decade shifts: rows are persistent–transitory and transitory only. Without the shifts the mental class paths rise with age, which the decade shifts of the main-text figure attribute to the older cohorts' higher level.

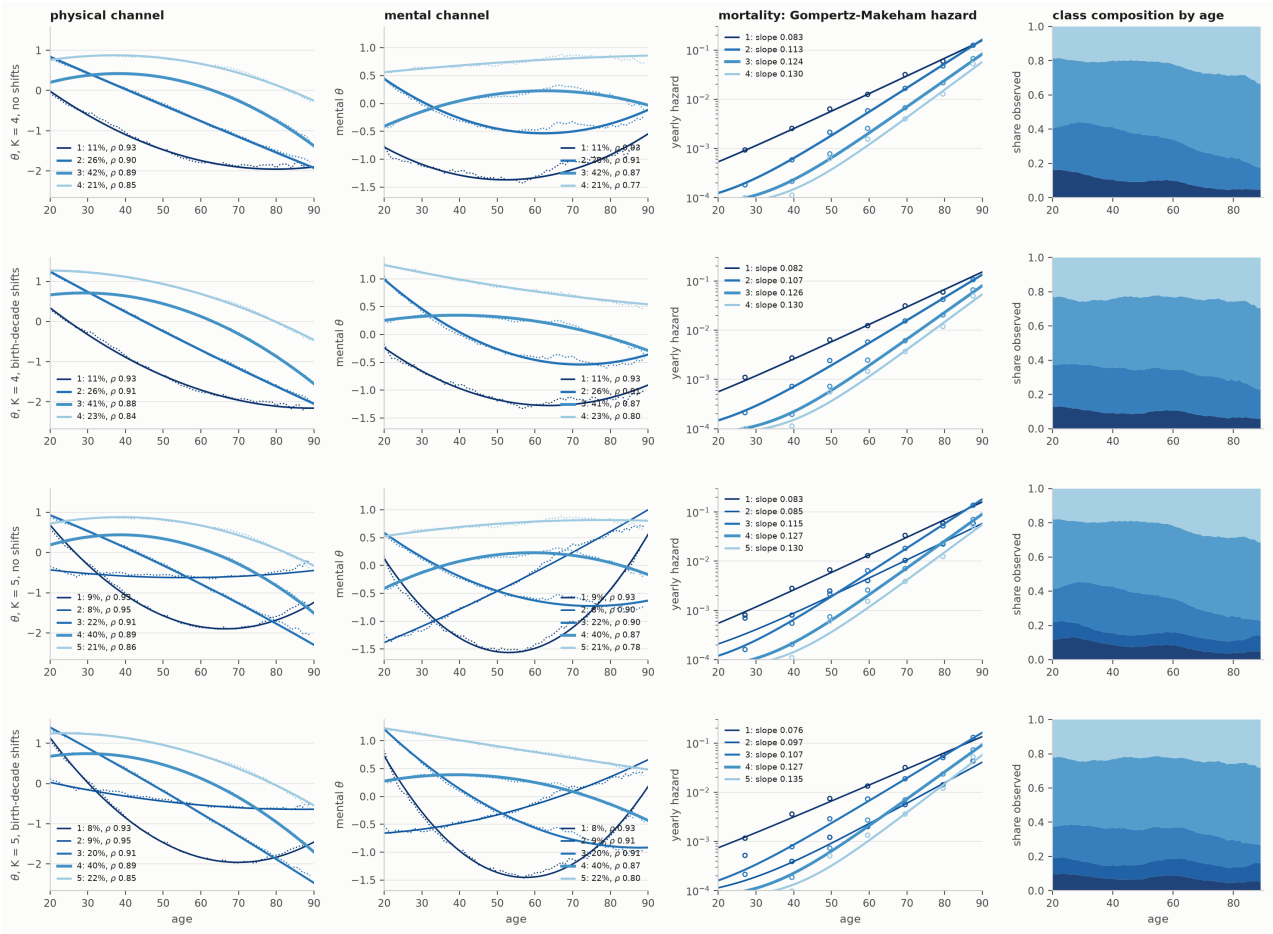


Figure F.6: The multidimensional $K = 4$ and $K = 5$ fits without and with the birth-decade shifts, columns as in Figure 7. With the shifts the four classes are the same as without (11, 26, 41 and 23% against 11, 26, 42 and 21%), with the same shapes on both channels and the same hazard ordering; the shifts on the physical channel run from $+0.49$ for the 1920s cohort to -0.45 for the 2000s relative to the 1950s, so the paths drawn at the 1950s sit higher at young ages. The shifted fit has two modes of identical log posterior; the one drawn reproduces the no-shift classes, the other puts 36% in the steep decliner and 14% in the healthy class. At $K = 5$ the shifted fit's two best chains agree (chains 2 and 3, about 170 log-posterior units above the next) and reproduce the no-shift classes read from its single best chain: 8, 9, 20, 40 and 22% against 9, 8, 22, 40 and 21%, with the same shapes and hazard ordering (worst-to-best hazard ratio at 55 of 16.9 against 13.8). Each fit is read over the chains that share its mode.

F.4 Multidimensional against Univariate Classes

Figure F.7 compares the classes of the multidimensional fit (Figure 7) with univariate fits on each of its two channels.

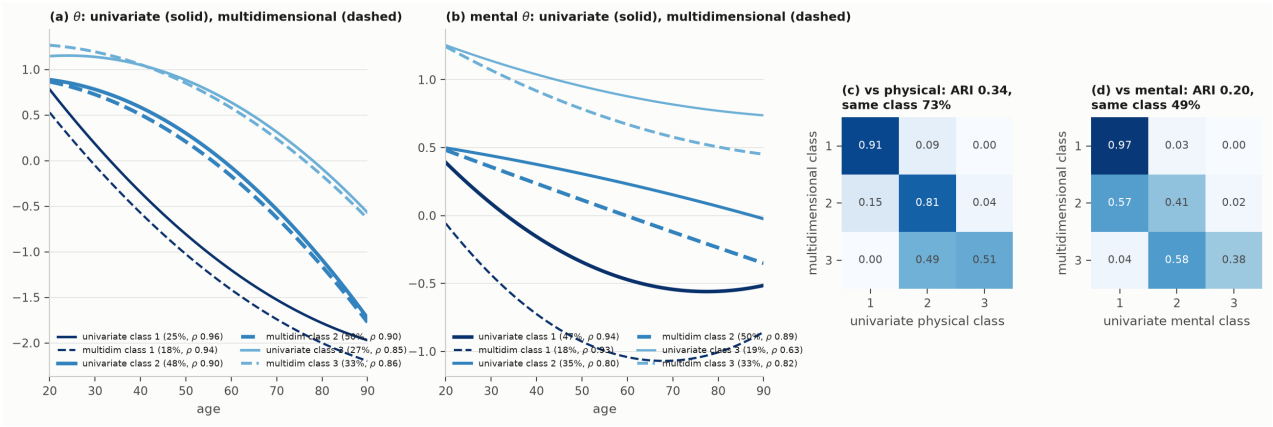


Figure F.7: Do the multidimensional classes differ from the univariate ones? Three $K = 3$ persistent–transitory fits with birth-decade shifts, all at the 1950s: θ alone, the mental GRM alone, and the multidimensional fit. (a) The physical class paths from the univariate fit (solid) and the multidimensional fit (dashed); (b) the same for the mental channel; (c), (d) the multidimensional modal class against the univariate physical and mental modal classes on the people in both fits, rows normalised, with the ARI and the share in the same class. Classes are ordered worst to best on each fit's anchor, the physical channel for the multidimensional fit.

- **The multidimensional classes are the physical classes with a mental path attached** (Figure F.7). The physical paths of the multidimensional fit lie on top of the univariate θ paths, with the worst class a little lower (-1.23 against -1.01 at 55) and a little smaller (18% against 25%); 73% of people take the same class in both fits and the disagreement is almost all between the best and the middle class (half of the univariate best class is the multidimensional middle class), the boundary the univariate fit draws with less to go on.
- **The mental classes are a different partition**, as Appendix H found: on its own the mental GRM separates people by how persistent their score is (shares .47/.35/.19, persistence .94/.80/.63) rather than by its level, so its largest class is its lowest. Against the multidimensional classes the agreement is ARI 0.20 with 49% in the same class: the multidimensional worst class is the univariate mental worst class (97%), but the multidimensional middle class splits between the two lower mental classes and the best between the two upper ones.
- **The mental paths the multidimensional fit learns are steeper and more spread than the univariate ones**: at 55 its classes sit at -1.00, 0.06 and 0.73 on the mental score against -0.41, 0.27 and 0.91 for the univariate classes, and the worst class's mental path falls by a standard deviation from 20 to 55 where the univariate worst class falls by 0.8. With the classes pinned by physical health and mortality, the mental channel reads as the mental-health profile of the physical types, which is what the joint model is for; on its own, the mental score is too noisy to carry a level typology.

F.5 The Windows at Four Classes

Figure F.8 repeats the window exercise of Figure 8 with four classes.

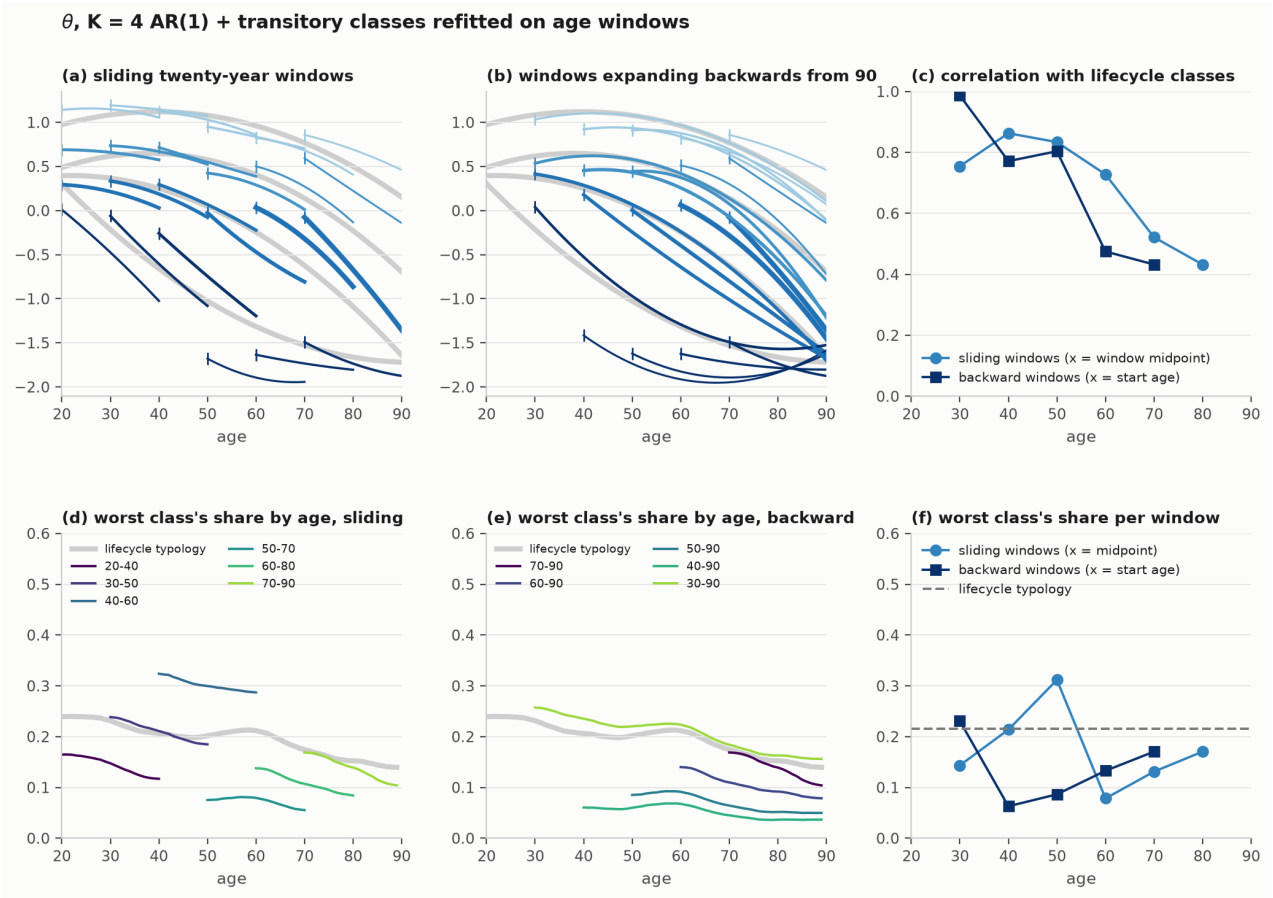


Figure F.8: As Figure 8, at $K = 4$: the persistent–transitory mixture with four classes refitted on each window, against the $K = 4$ lifecycle fit in grey (worst class 22%). Agreement with the lifecycle classes is lower than at $K = 3$ once the window starts late: the backward windows' co-assignment correlation is 0.43 from 70, 0.48 from 60, 0.80 from 50, 0.77 from 40 and 0.99 from 30, the sliding windows 0.43–0.86. The windows starting at 50 or later find a small, very low class (8–17%, near -1.5 to -2 in θ) that sits below the lifecycle worst class, and the 40–60 window again has the largest worst class (31%). Two of the ten fits are multimodal, both in the oldest windows, where two of the four classes are small (4–6%): the 70–90 chains sit in three modes and the fit is read over the two that agree with the highest log posterior (8–11 units above the others); in the 60–90 fit one chain sits in a nearby mode 12 units lower, with the two small classes' shares and persistences exchanged, and is left out. The other eight have all four chains at one mode.

F.6 Predicting Death

Figure F.9 reports how well each typology predicts death before the next wave.

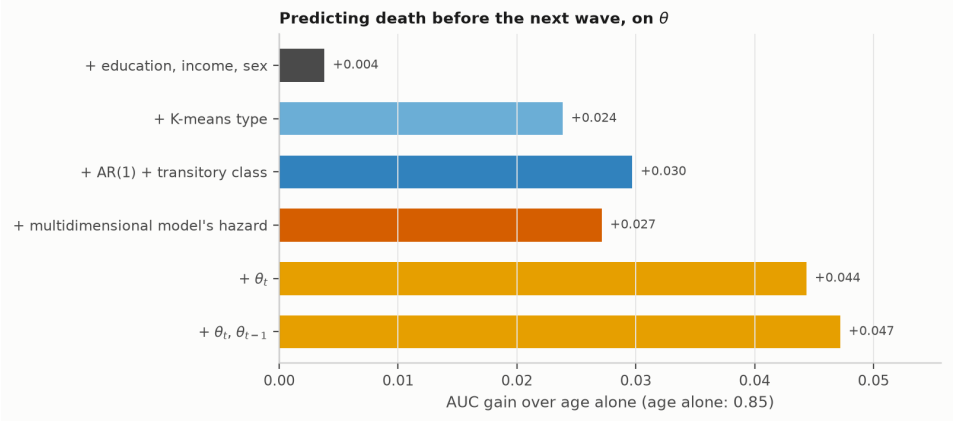


Figure F.9: Predicting death before the next wave on θ: the gain in AUC over a quadratic in age (AUC 0.85) from adding each set of predictors, at every person-wave at risk on the full-sample fits (308,949 person-waves, 1,267 deaths), fitted on 70% of people and scored on the other 30%. The K-means type and the persistent–transitory class never see the death record; the multidimensional model's hazard is its class-weighted Gompertz–Makeham hazard at the person's age, with the class taken from the physical and mental channels only. Each gain is over age alone on the same rows; θ_{t-1} needs the previous wave, so that row uses the 248,802 person-waves with one. The same exercise on h, with the held-out comparison, is Figure I.13.

G Additional Results on the Returns to Prevention

G.1 The Estimated Cost Curve

Figure G.1 shows the cost curve the returns are priced with.

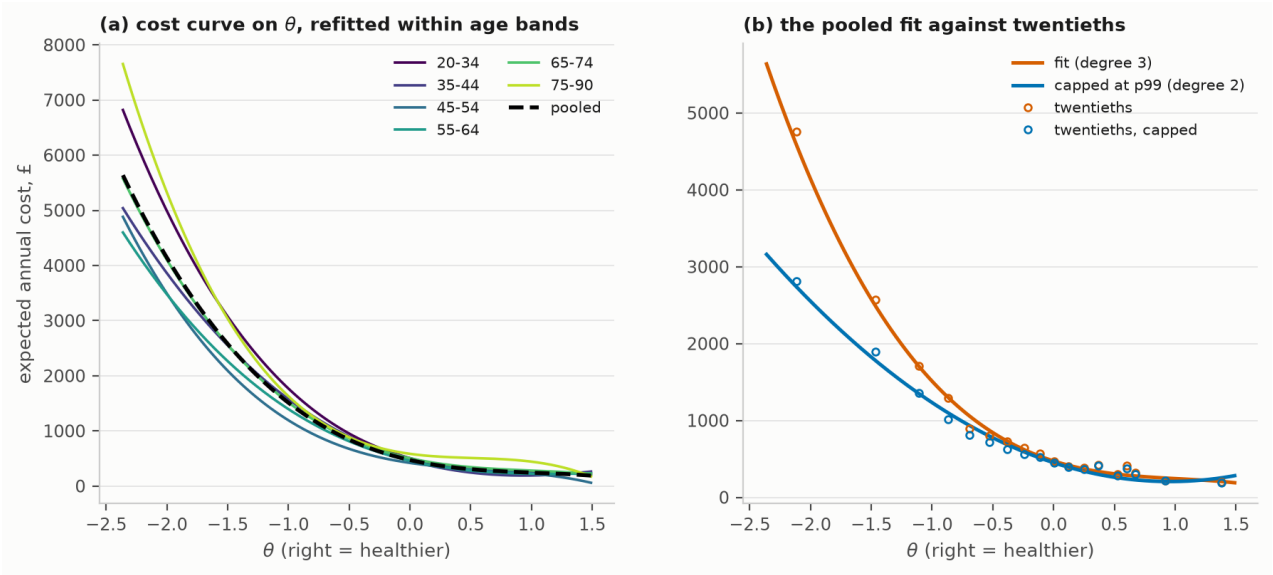


Figure G.1: The cost curve Section 5 prices with: the cost index in pounds against θ, with a cubic fitted to it. (a) The same polynomial refitted within six age bands against the pooled fit; (b) the pooled fit, and the same fit on costs capped at the 99th percentile, against the mean cost in twentieths of θ.

- Below 75 the estimated curve is close to age-invariant in shape. Refitted within six age bands it stays within about 16% of the pooled curve on average from 20 to 74, with a shallow U in age (16% below the pooled curve at 45–54, 12% above it at 20–34), and the 75–90 band sits clearly above it, by 32%. Costing health rather than age is therefore defensible for the returns, with an understatement of late-life cost.
- Capping at the 99th percentile flattens the curve at the sick end, where the capped fit is a quadratic, and lowers every first-order return by a little under half and the exact saving at $\mu = 1$ by about a third (Appendix G.4).

G.2 The Detailed Returns and the Targeting Rules

Figure G.2 breaks down the treatment bill and the return by start age, Figure G.3 compares the targeting rules, and Figure G.4 shows when the saving arrives.

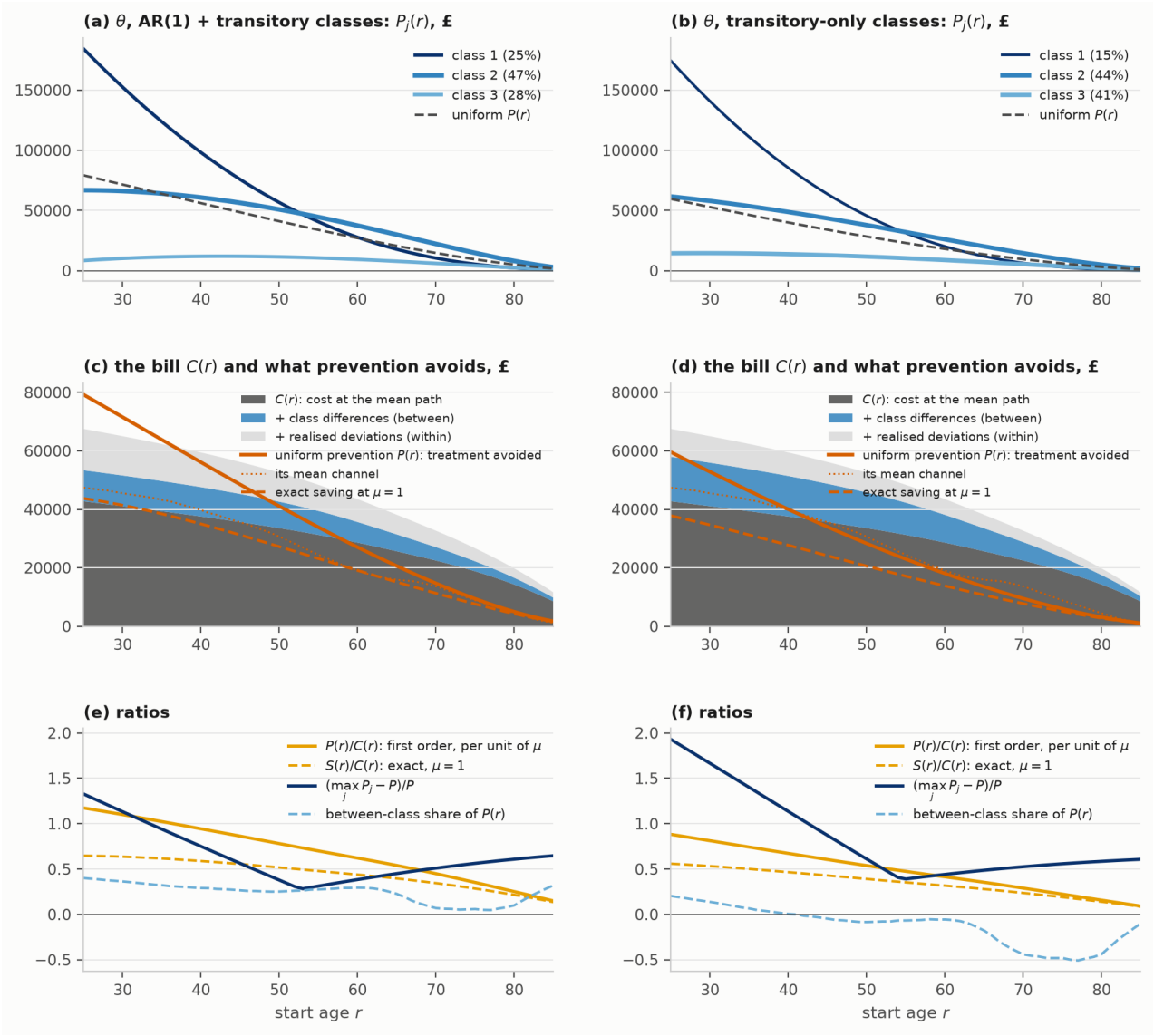


Figure G.2: The treatment bill and the return to prevention by start age r on θ, in pounds under the estimated cost curve: (left) the persistent–transitory classes, (right) the transitory-only classes. Top: $P_j(r)$ by class and the uniform $P(r)$, the treatment cost avoided per unit of μ. Middle: the bill $C(r)$ stacked into the cost at the mean path, what the type differences add and what realised deviations add, with $P(r)$, its mean channel and the exact saving at $\mu = 1$, $S(r)$ (dashed). Bottom: P/C, the first-order return relative to the bill, and S/C, the exact saving at $\mu = 1$ relative to it; the gain from class targeting relative to the uniform return; and the between-class share of $P(r)$.

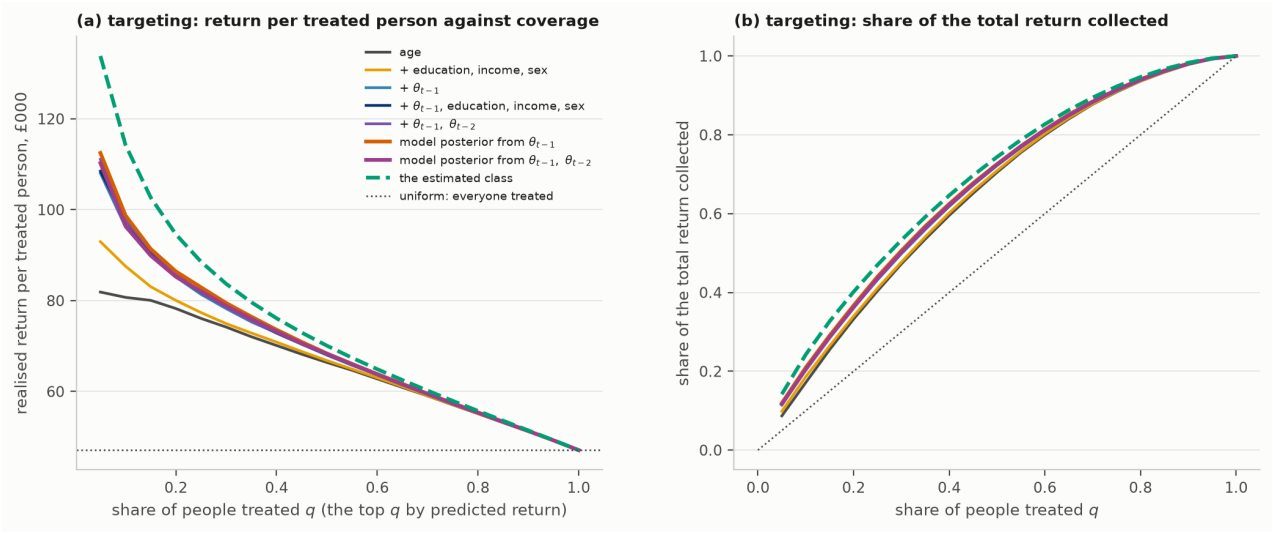


Figure G.3: What to target on when the type is not observed. The return to prevention by class, $P_j(r)$, at each person's age at $t-1$, on the $K=3$ persistent–transitory classes fitted with the first two rows held out (29,899 people), so that t is a person's first fitted row and $t-1$ the later of the two held-out rows. Each rule predicts a person's return from what is known at $t-1$: age, education, income and sex, θ_{t-1}, and θ_{t-1} and θ_{t-2} (a multinomial logit of the class on a quadratic in age plus the information set shown, fitted on a 70% split; the model's own posteriors from those readings and the estimated class, the posterior from the whole fitted window, need no regression), treats the top share q of the other 30% by predicted return, and is scored by the realised return, the fitted-window class posterior times $P_j(r)$. (a) Return per treated person against coverage; dotted, everyone treated. (b) The share of the total return collected. The age-only rule is the benchmark, since returns fall with age; what the health reading and the observables add on top is the value of type information.

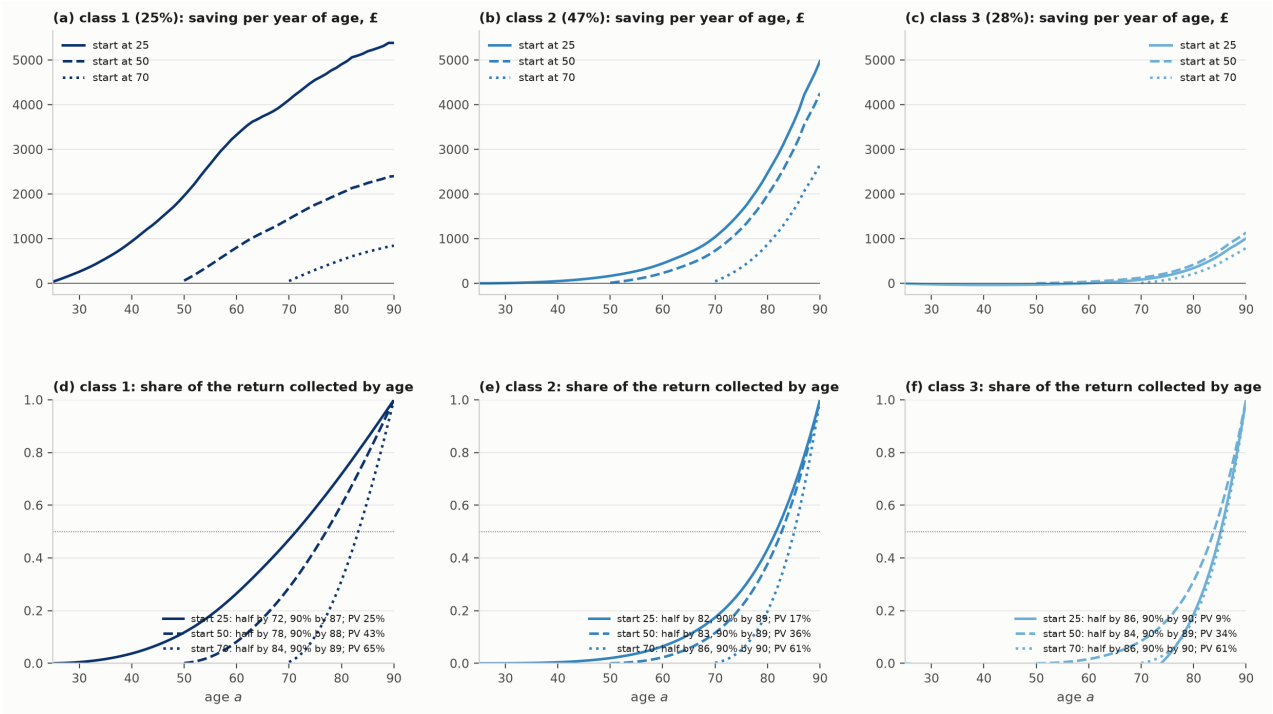


Figure G.4: When the saving arrives, by class. Top: the saving in each year of age a from prevention started at 25, 50 and 70, $s_j(a \mid r) = E[-c'(h_a) \mid j](\bar{h}_{j,r-1} - \bar{h}_{j,a})$, whose sum over $a \geq r$ is $P_j(r)$. Bottom: the share of $P_j(r)$ collected by age, with the ages by which half and 90% has arrived and the present value at 3.5% a year as a share of the undiscounted return.

G.3 K-Means Types under the Estimated Cost Curve

Figure G.5 prices the partial K-means types with the same cost curve.

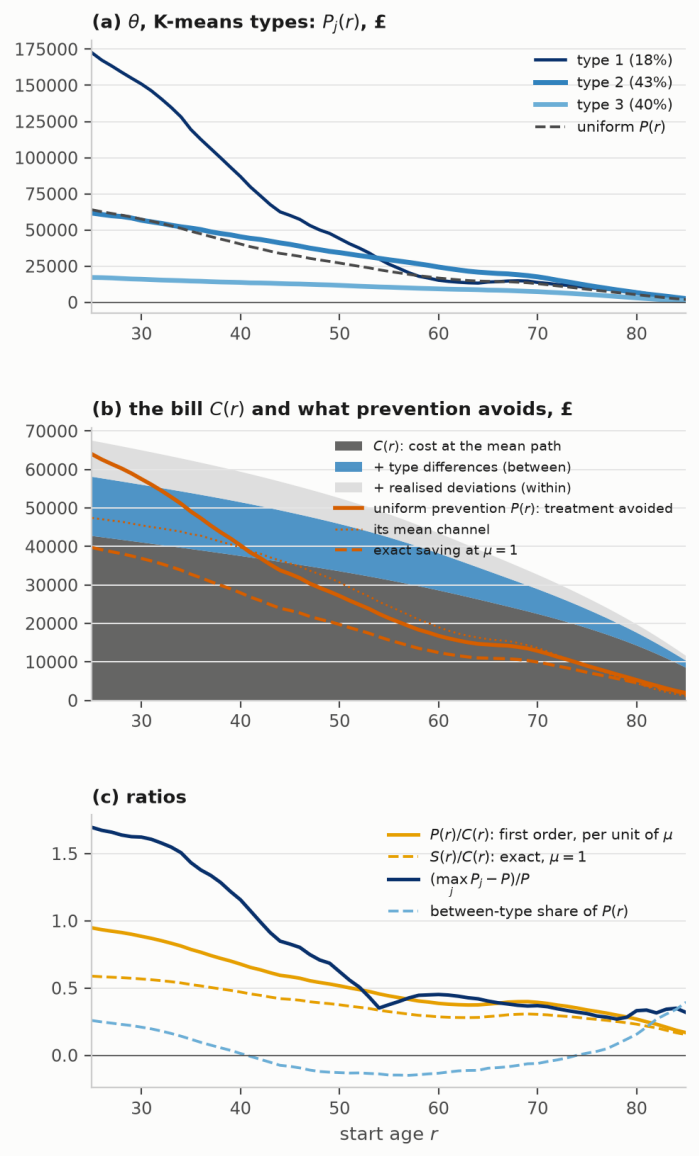


Figure G.5: As Figure G.2, on the partial K-means types on θ: type shares and smoothed type means, with the same cost curve.

- On the K-means types $P(r)/C(r)$ is 0.95 at 25, 0.52 at 50 and 0.39 at 70, below the 1.17, 0.78 and 0.45 of the persistent–transitory classes, and the exact saving at $\mu = 1$ is 0.59, 0.38 and 0.30 of the bill; the within-type part of the bill is 14%, 13% and 11%.
- The between-type channel is +26% of $P(25)$ and turns negative from about 40 (−13% at 50, −6% at 70), as on the transitory-only classes: the K-means types converge after midlife, where the persistent–transitory classes keep fanning out.
- Type targeting gains 170% over the uniform return from 25, 62% from 50 and 37% from 70 (186%, 92% and 35% at $\mu = 1$).

G.4 The Capped Cost Curve

Figure G.6 prices the classes with the curve fitted to capped costs.

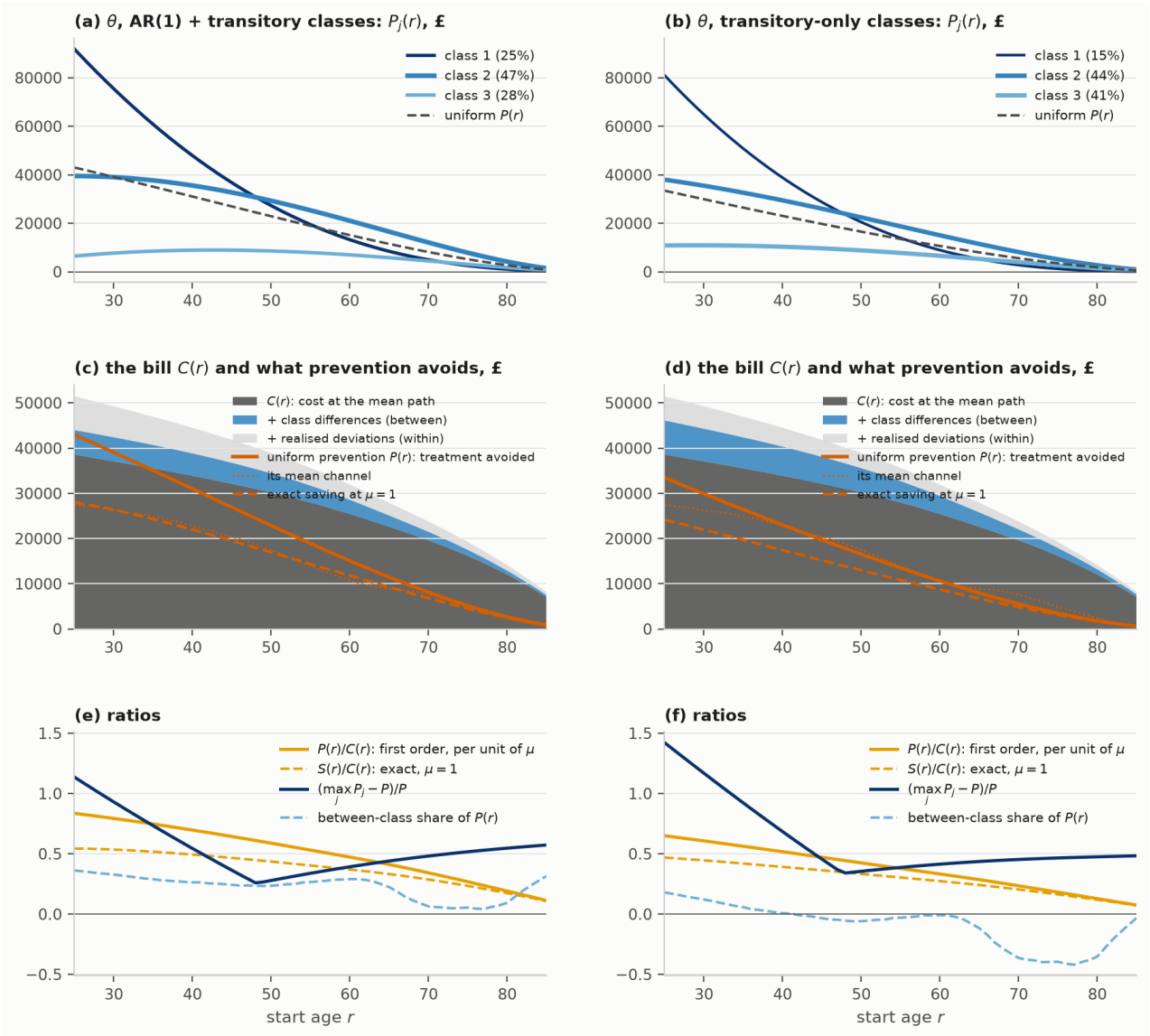


Figure G.6: As Figure G.2, with the cost curve fitted to costs capped at the 99th percentile.

- With costs capped at the 99th percentile the bill from 25 falls from £67,500 to £51,500 and uniform prevention from £79,200 to £43,000, so P/C falls to 0.84, 0.59 and 0.34 at 25, 50 and 70 on the persistent–transitory classes (the cap removes most of the sick tail, which is where prevention's pounds are); the within-class part of the bill is 14%, 11% and 10%; the between-class shares of P are +36%, +24% and +6%, and the class-targeting gains 114%, 28% and 48%. The capped curve is a quadratic, so its exact saving at $\mu = 1$ is $P(r)$ less the sum of squared declines times the constant curvature: £28,000 from 25 against £43,700 on the cost index, 0.54, 0.44 and 0.29 of the capped bill.
- The one change is at 50, where the middle class rather than the worst has the highest first-order return under the capped curve; at $\mu = 1$ the worst class keeps the highest saving there too.

G.5 The Return by Class under Other Typologies and Measures

Figure G.7 prices six typologies on θ with the same cost curve, and Figure G.8 repeats the persistent–transitory returns on each of the four measures at $K = 3$, 4 and 5.

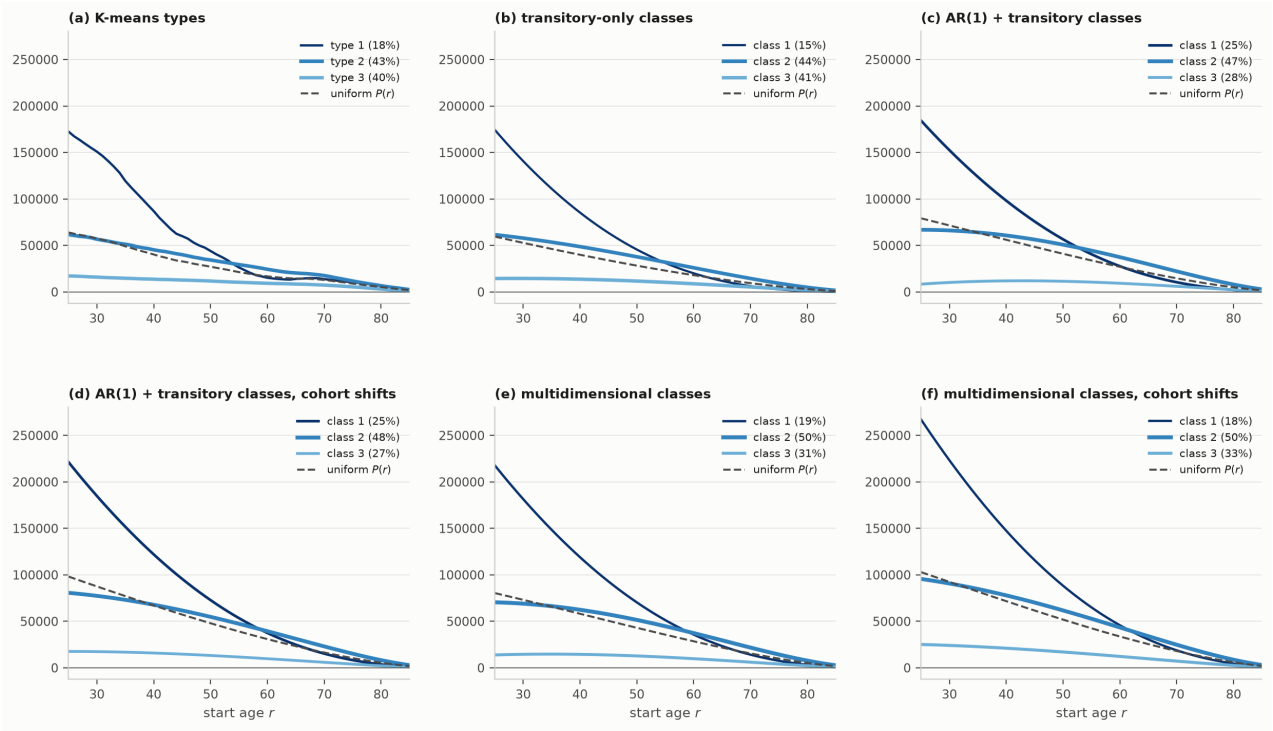


Figure G.7: The return to prevention by class, $P_j(r)$, on θ under six typologies, all $K = 3$ and all priced with the same cost curve: (a) the partial K-means types; (b) the transitory-only mixture; (c) the persistent–transitory mixture of Figure 11; (d) the same with birth-decade shifts, paths at the 1950s (the preventable decline is shift invariant); (e), (f) the physical channel of the multidimensional fits (θ, the mental GRM and mortality) without and with the shifts, on the multidimensional contract's rows. Line width is proportional to the class share; dashed, the uniform return; one vertical scale.

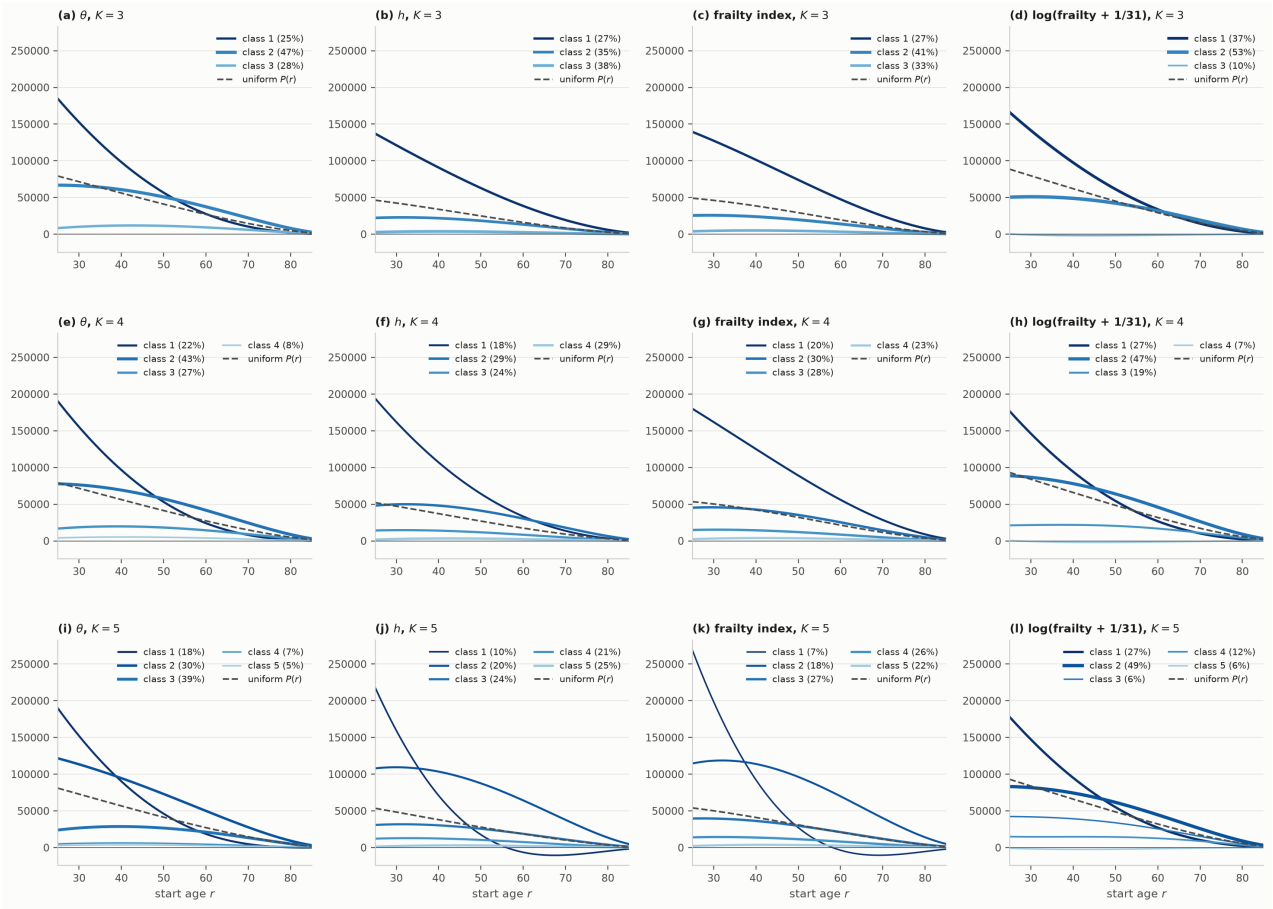


Figure G.8: The return to prevention by class under the persistent–transitory mixture on each of the four measures (columns: θ, h, the frailty index, log frailty), at $K = 3$, 4 and 5 (rows), each measure with its own cost curve in pounds. On the frailty indices higher is frailer, so the preventable decline is the class's rise in frailty and class 1 is the frailest; the frailty indices' $K = 5$ fits are read at the single chain with the highest log posterior (Figure I.6). One vertical scale.

- **The shape of the return by class is the same under every typology** (Figure G.7): the worst class's return falls steeply from about £170,000–270,000 at 25 to meet the middle class's between 53 and 62, the best class's is flat and small, and the uniform return tracks the middle class until the worst class's return crosses below it. The K-means types give £173,000 for the worst type at 25 against £184,000 for the persistent–transitory classes and £174,000 under the transitory-only specification; the uniform returns are £64,000, £79,000 and £60,000.
- **The birth-decade shifts raise every return, by about a quarter at 25**: £98,000 uniform at 25 against £79,000 without them on the single-channel classes, £103,000 against £80,000 on the multidimensional ones, with the worst class at £221,000 and £267,000. This is the age–cohort collinearity of Section 4.3 read through the returns: once the shifts carry the younger cohorts' lower level, the within-cohort age path is steeper, and prevention scales that steeper decline. The multidimensional classes return more than the single-channel ones at every start age because their worst class is a little lower and smaller.
- **Across measures the worst class carries the return everywhere, but the bounded measures move it around** (Figure G.8, each measure priced with its own cost curve). On h and on the frailty index the uniform return at 25 is £46,000–49,000 against £79,000 on θ, and the worst

class's £137,000–139,000 against £184,000, because the collapsed healthiest class (persistence 0.25 on h, 0.36 on frailty) has almost no preventable decline and the middle class is flatter too. Log frailty gives the largest uniform return, £89,000, with a middle class worth £50,000 and a small healthiest class worth nothing (its path barely moves), and its worst class £166,000. The ranking of the classes is common to all four. On θ and log frailty the worst class's return falls below the middle class's from a start at 53 and 62; on h and the frailty index it stays above it at every start age. The level of the return depends on which measure's decline the cost curve is read against, which is the cardinality point of Section 3.

- **Adding classes splits the return, it does not create it.** On θ the uniform return at 25 is £79,000–81,000 at every K. At $K = 4$ the worst class is unchanged (£190,000, 22%) and the middle class of $K = 3$ splits into one worth £78,000 and one worth £17,000; at $K = 5$ the worst class splits into two, £190,000 (18%) and £122,000 (30%), and the next class is worth £24,000. On h the same happens with a smaller base (£46,000 to £54,000 uniform), and at $K = 5$ the smallest class (10%) has a return that turns negative after 56: its path improves with age in old age, the collapsed-class pathology of the bounded scale again. The frailty indices at $K = 4$ repeat the pattern: on the index the uniform return moves from £49,000 to £53,000 while the worst class (20%) is worth £180,000 and the old middle class splits into £45,000 and £15,000; on log frailty the uniform return moves from £89,000 to £93,000, the worst class (27%) is worth £177,000 and a large second class (47%) £89,000, with the two healthiest classes again worth little or nothing. At $K = 5$ the frailty index behaves like h: the uniform return is £54,000, and a small, very frail class (7%) is worth £269,000 from 25 but turns negative from a start at 59; on log frailty the uniform return stays at £93,000.

G.6 Life Years

Figures G.9 to G.14 repeat the life-years exercise of Figure 12: in detail, at $\mu = 0.1$, on the multidimensional classes and on the other three measures. The solid lines of Figure G.11(a) and (b) are the life expectancy by class of Section 4.

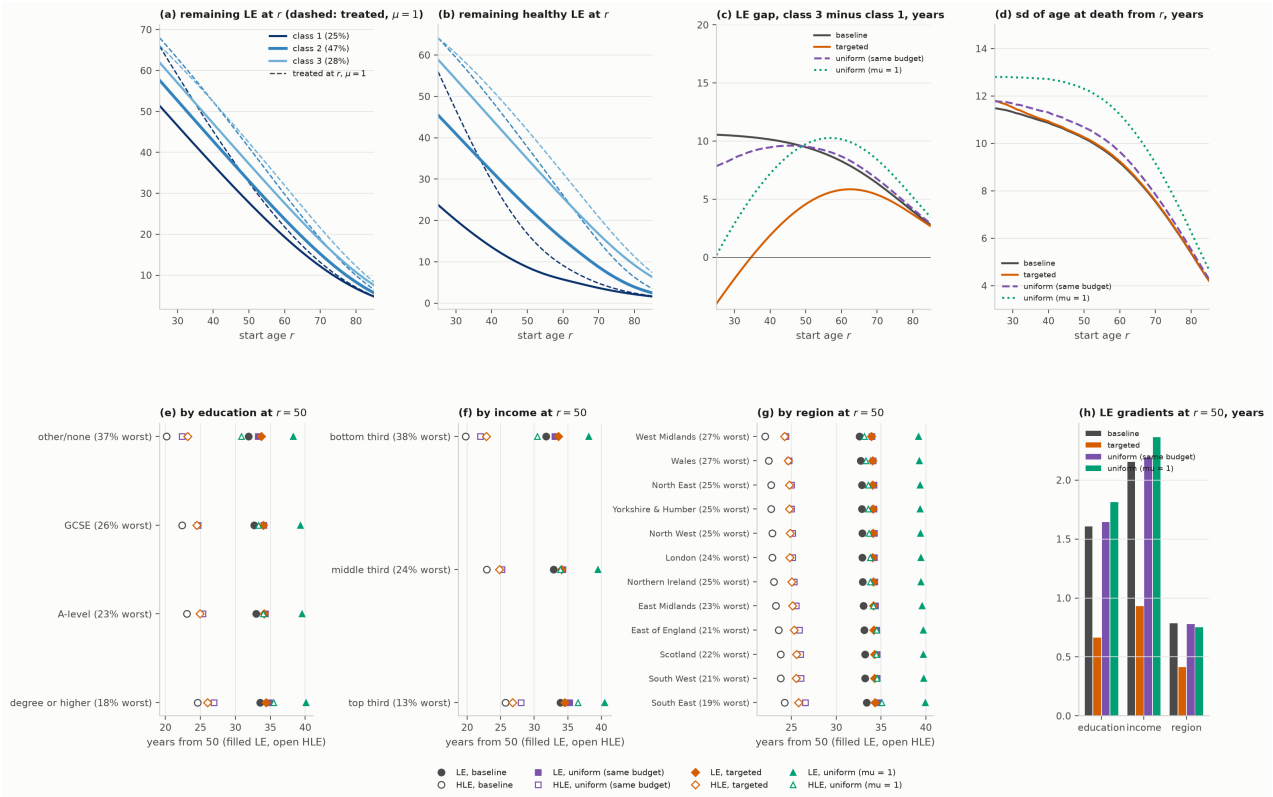


Figure G.9: The detailed version of Figure 12, on the same univariate classes, hazards and policies, with uniform prevention at $\mu = 1$ throughout. (a), (b) Remaining LE and HLE by class at every start age, and each class treated on its own at $\mu = 1$ (dashed). (c) The LE gap between the best and the worst class. (d) The standard deviation of age at death over the population mixture. (e)–(g) LE (filled) and HLE (open) from 50 by education, income third and region, each group's class composition among the person-waves within five years of 50 times the class values, with the group's worst-class share in the label. (h) The LE gradients of Figure 12(d).

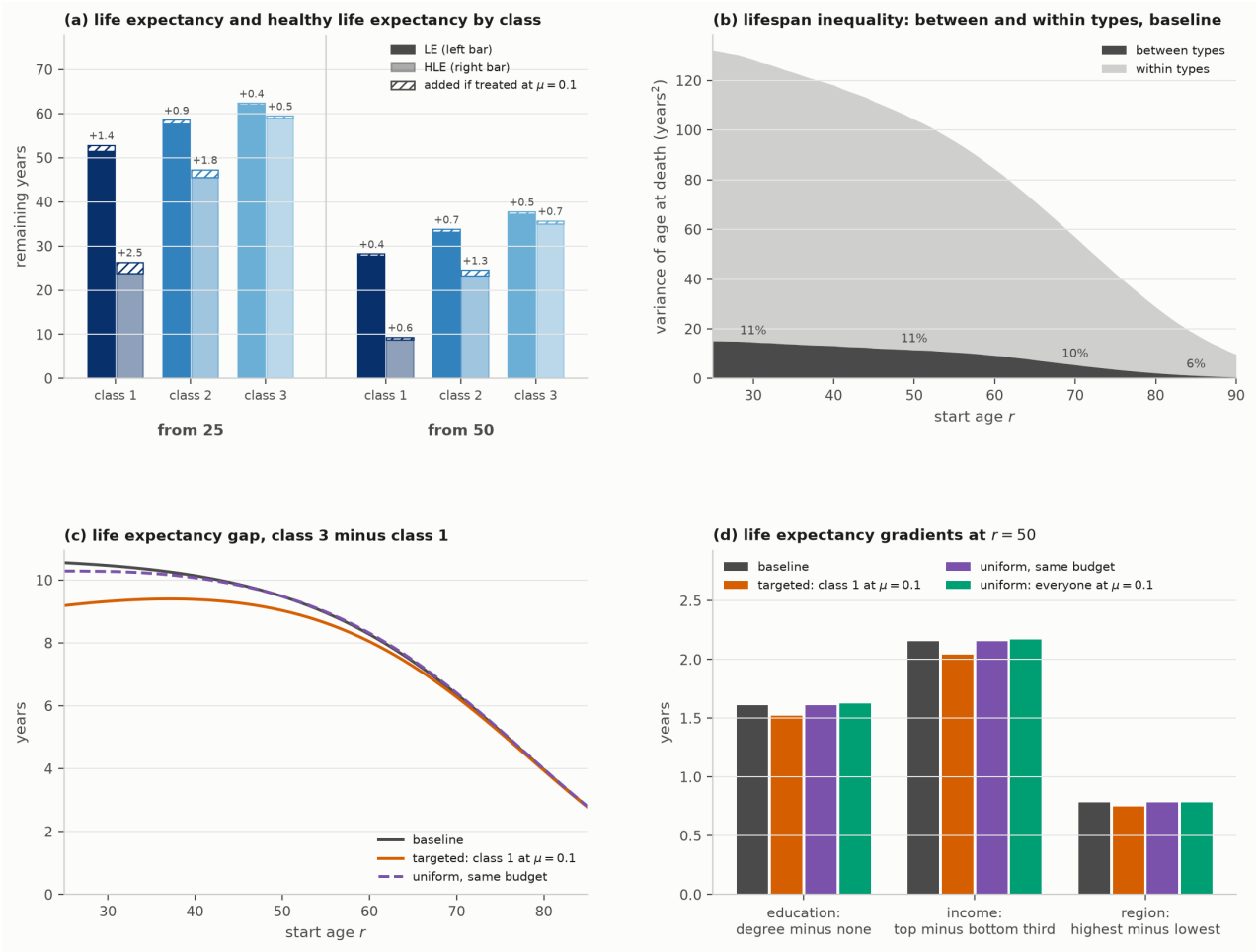


Figure G.10: As Figure 12, at $\mu = 0.1$: targeted, class 1 at $\mu = 0.1$; uniform with the same budget, everyone at $\mu = 0.1\,\pi_1(r)$; in (d), uniform, everyone at $\mu = 0.1$. (a) hatched: what each class gains if treated on its own from r at $\mu = 0.1$. (b) is the baseline, as in Figure 12.

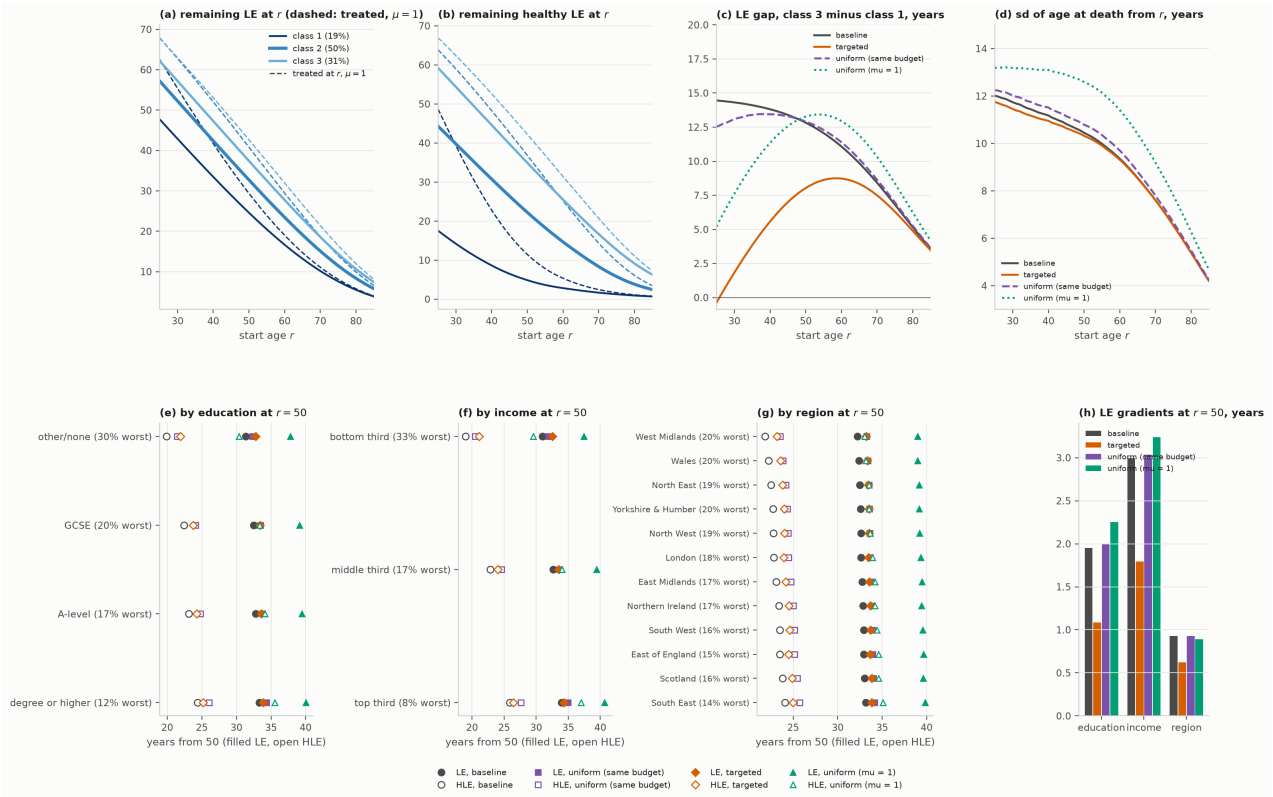


Figure G.11: As Figure G.9, on the $K = 3$ persistent–transitory classes of the multidimensional fit (θ, the mental GRM and Gompertz–Makeham mortality), whose class hazards are part of the fit, calibrated to the ONS tables in the same way. The same budget is $\mu = \pi_1(r)$ on these classes (0.17 at 50).

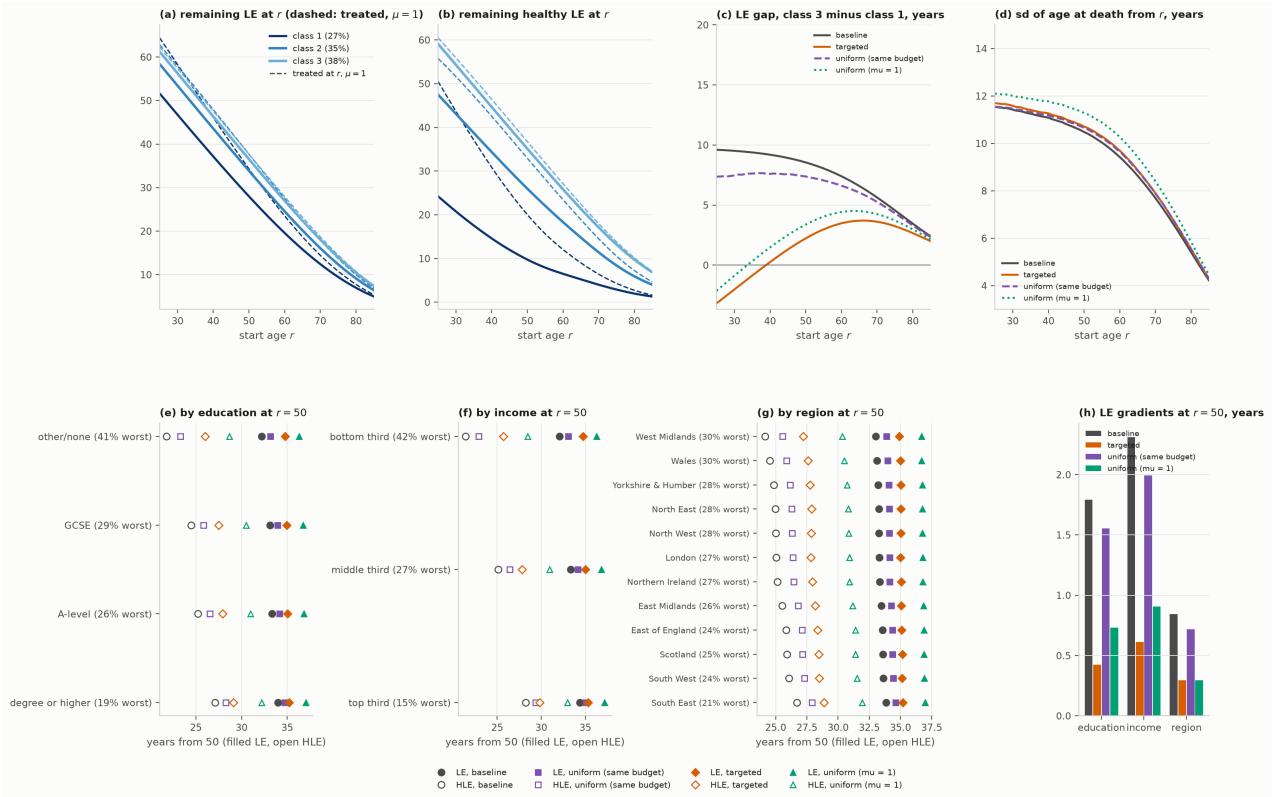


Figure G.12: As Figure G.9, on the univariate persistent–transitory classes on h: class hazards estimated for those classes, and the gradients of mortality and of the healthy share re-estimated in h.

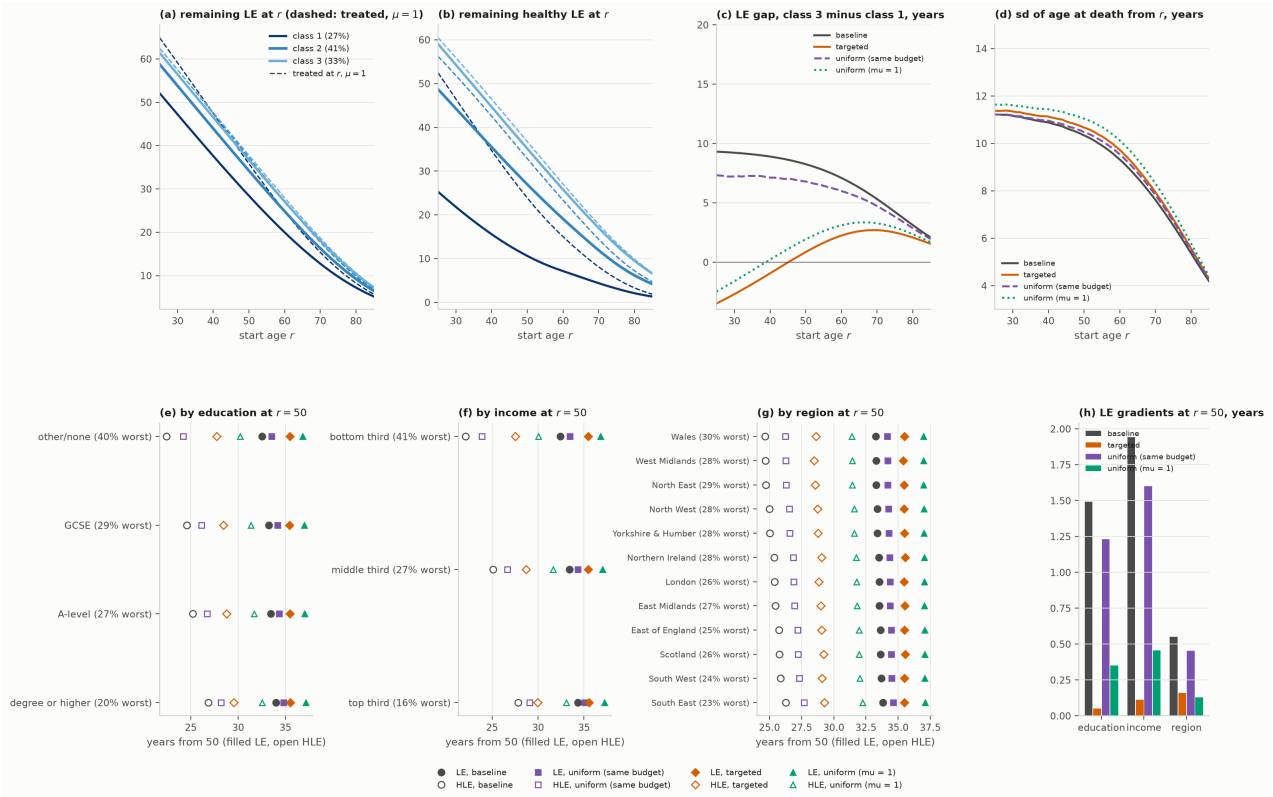


Figure G.13: As Figure G.9, on the univariate persistent–transitory classes on the frailty index (class 1 the frailest; the held-back change is a rise in frailty, and the gradients are re-estimated in the index).

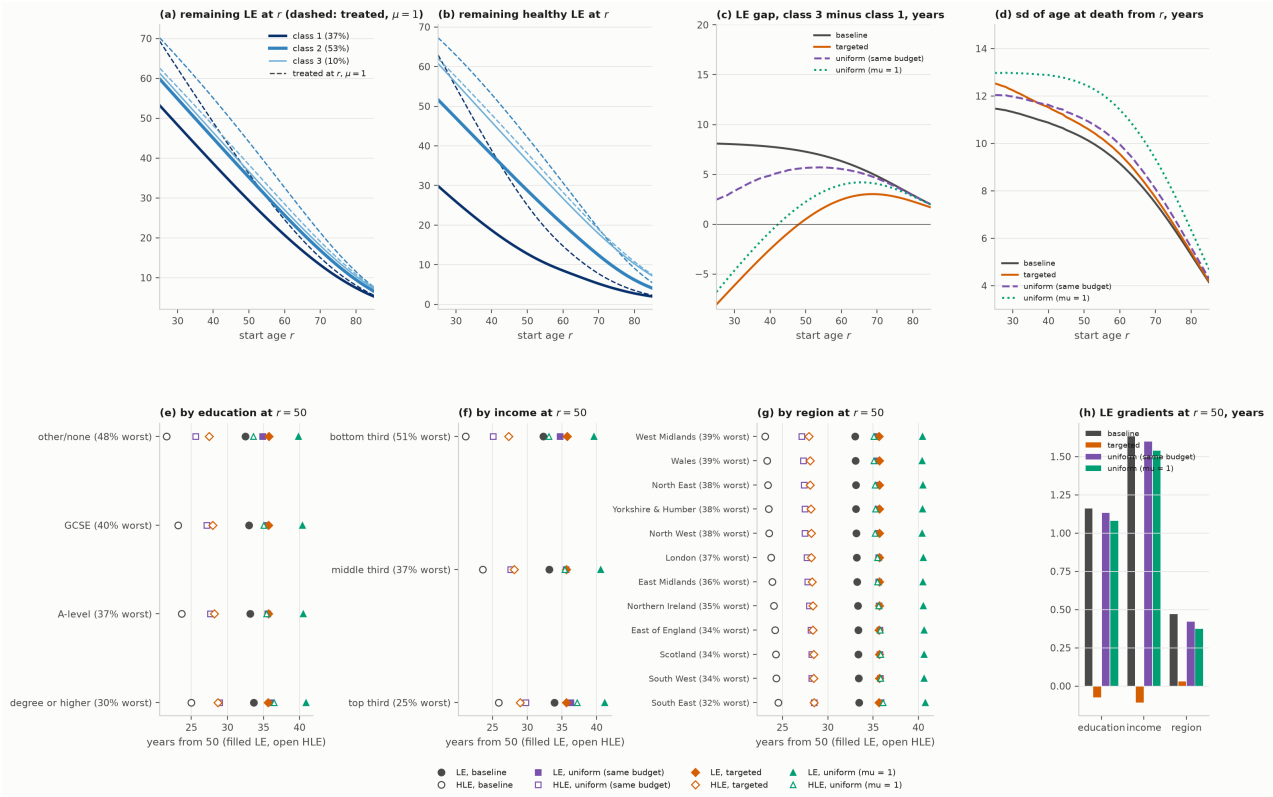


Figure G.14: As Figure G.13, on log frailty.

- **On the multidimensional classes the gaps are wider, and the ranking of the policies is the same** (Figure G.11). The multidimensional worst class is smaller (19% against 25%) and more sharply drawn, since the fit's mortality channel pulls the people who go on to die into it, and its hazards are part of the fit rather than estimated afterwards. The LE gap between the best and the worst class from 50 is 12.8 years against 9.5 on the univariate classes (from 25, 14.5 against 10.6), and the composition gradients the types carry are larger (2.0 years by education, 3.0 by income and 0.9 across regions from 50, with worst-class membership running from 30% among people with no qualifications to 12% among graduates and from 33% in the bottom income third to 8% in the top). Treating the worst class at $\mu = 1$ from 50 takes the type gap to 8.0 years (the HLE gap from 30.1 to 23.5) and the gradients to 1.1, 1.8 and 0.6; the same budget spread evenly ($\mu = 0.17$) again leaves them where they are; population LE rises by 0.8 years under targeting and 1.0 under the even spread; and the standard deviation of age at death barely moves (10.4 years at baseline, 10.3 and 10.8). From 25 the targeted policy takes the worst class level with the best (62.6 against 62.2 years) rather than past it. Here the mortality gradient in θ is at or below the between-type gradient the fit's own class hazards imply at every age, so the years are if anything conservative, where on the univariate classes the late-life years are if anything generous.
- **On the other measures the type gaps are smaller still, and targeting closes them almost entirely** (Figures G.12–G.14). The LE gap between the best and the worst class from 50 is 8.5 years on h, 8.3 on the frailty index and 7.3 on log frailty, against 9.5 on univariate θ and 12.8 on the multidimensional classes; the gradients the types carry shrink in step (1.8, 1.5 and 1.2 years by education, 2.3, 1.9 and 1.6 by income). Treating the worst class at $\mu = 1$ from 50 takes the type gap to 2.2, 0.9 and 0.5 years and the gradients to within half a year of zero, because on the bounded measures the worst class holds nearly all the preventable decline: the best class

sits at the ceiling of h or the floor of frailty with almost nothing to prevent, which is why under uniform prevention at $\mu = 1$ on log frailty the middle class ends up ahead of the best (44.1 against 38.3 years). The even spread barely moves the gradients, and the standard deviation of age at death from 50 rises by at most half a year under targeting and 0.8 years under the even spread. The ranking of policies is therefore the same on every measure; what the measure changes is how much of the decline the worst type owns, as in Figure G.8. The gradients of mortality and of the healthy share in h and in the frailty index are steep per unit because the scales are bounded, so these versions lean more on the logit links at the ends of the scales.

H Mental Health

The mental GRM is the second channel of the multidimensional model of Section 4.3: the three SF-12 mental testlets and two GHQ-12 testlets, with no depression diagnosis, higher is better, standardised on the pooled sample. Here it is on its own, on the multidimensional contract's rows (38,181 people), so it shares a sample with the multidimensional fits. Two findings matter for the paper. Half of its variance is one-period noise (Figure H.1), and once that noise is separated the mixtures sort people by how persistent their score is rather than by its level (Figure H.2). The mental score is therefore too noisy to carry a level typology of its own, which is why the multidimensional classes are the physical classes with a mental path attached (Appendix F.4).

The cost and mortality gradients look like those on θ but weaker. On capped costs the curvature in mental health has $t = 18.9$ against 33.5 on θ, and the slope is about half of θ's (£239 against £469 per standard deviation at the mean); the log-odds of death fall by 0.57 per standard deviation against 0.90 on θ. Partial K-means on the mental score finds types with shares close to those on θ (.20/.42/.38 against .18/.43/.40) but flat, parallel paths, and they are different people: the adjusted Rand index with the θ types is 0.14 (58% in the same type), against 0.32–0.61 between any two of the physical measures.

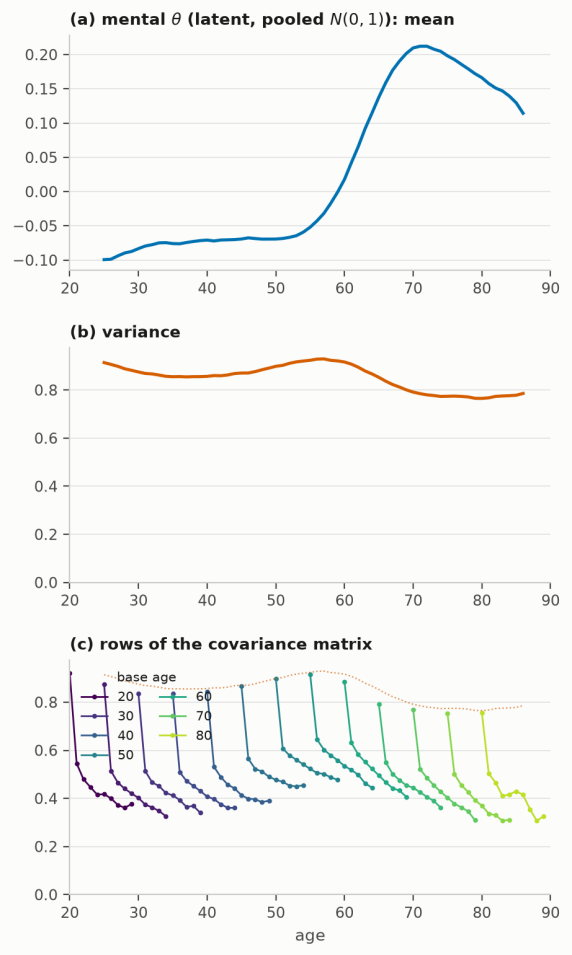


Figure H.1: As Figure 2, on the mental GRM: mean path, variance profile and covariance rows, model-free.

- The mean path is nothing like the physical one: flat at about −0.08 from 20 to 55, then up by 0.3 sd to a peak at about 70, then down again. The variance is flat at 0.85–0.93 across the whole age range, against the hump on θ.
- Roughly half of the variance is one-period: every covariance row drops from about 0.9 on the diagonal to 0.45–0.55 a year later, then decays slowly. On θ the first-year drop is about a fifth. This is the fast mental channel of the multidimensional fits, where its transitory share is 0.34–0.55 by class.

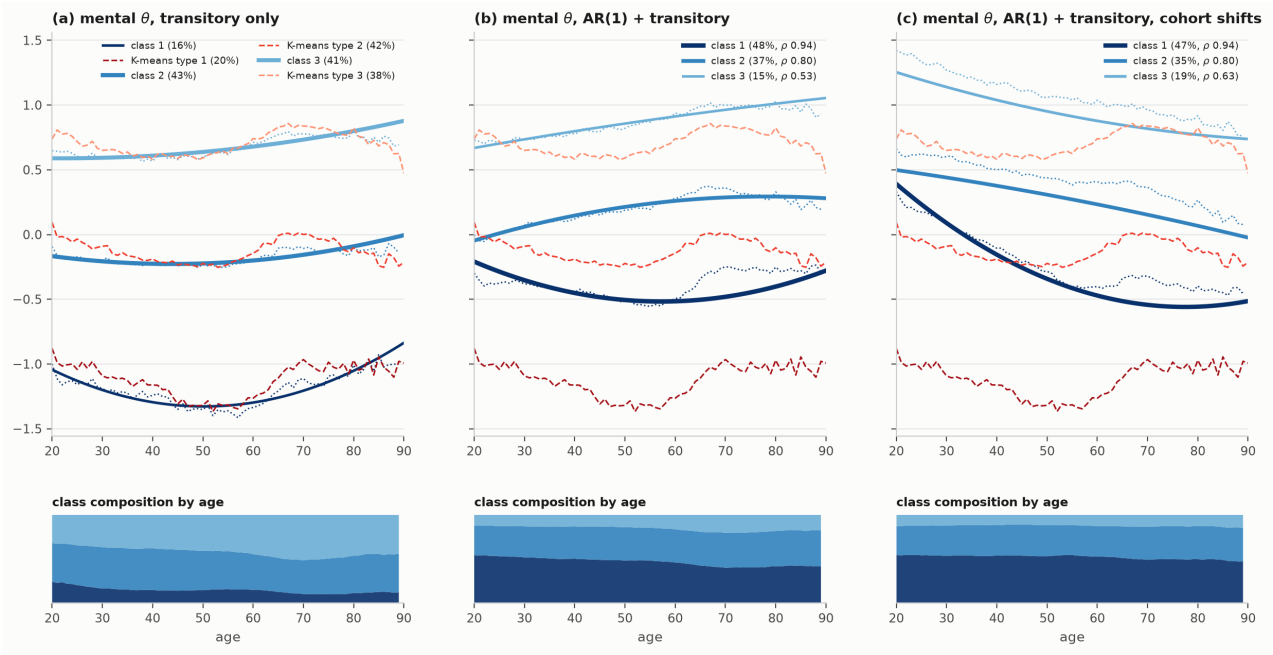


Figure H.2: As Figure 6(a) and (d), $K = 3$ mixtures on the mental GRM, on the multidimensional contract's rows: (a) transitory only, (b) persistent–transitory, (c) the same with one level shift per birth decade common to the classes, paths and observed class means at the 1950s with every score net of its decade shift. Dashed red: the partial K-means types on the same rows.

- Under the transitory-only specification the mixture reproduces the K-means types: shares .16/.43/.41 against .20/.42/.38, class paths at about -1.3, -0.2 and $+0.6$ at 50, each with the dip to 50 and the rise after 60.
- Under the persistent–transitory specification the classes are a different partition. The persistence is .94/.80/.53 with a transitory scale of 0.54, so the signal shares are 0.72/0.45/0.29, and the largest class (48%) is the lowest, running from -0.2 at 20 to -0.5 at 55 and back; nothing sits near the K-means worst type at -1. With half of the variance one-period, the mixture separates people by how persistent their mental score is rather than by its level: the small top class (15%) is mostly transitory. The class composition is flat in age.
- With birth-decade shifts the shares and persistence barely move (.47/.35/.19; .94/.80/.63), but the shifts are large: relative to the 1950s, $+0.2$ for the 1920s–40s and -0.9 for the 2000s, 1.1 sd across the 1940s–2000s, twice the physical shift. At the 1950s every class path then declines with age, so the rise after 60 in Figure H.1 is being read as the older cohorts' higher level. The age–cohort caveat of Section 4.3 applies with more force here, since the whole post-60 rise sits inside the fifteen-year window where age and birth year are nearly collinear.

I The Main Figures on the Other Measures

This appendix repeats the main-text figures on h, the 31-deficit frailty index and $\log(\text{frailty} + 1/31)$, defined in Section 3 and Appendix B.3, in the order of the main text. Figures I.1 to I.5 repeat the descriptive and K-means figures on all three. Figure I.6 shows the persistent–transitory mixtures on the two frailty indices at $K = 3$, 4 and 5. Figures I.7 to I.15 repeat the mixtures, the observables, prediction and the returns on h, and Figure I.12 predicts the cost proxy rather than health.

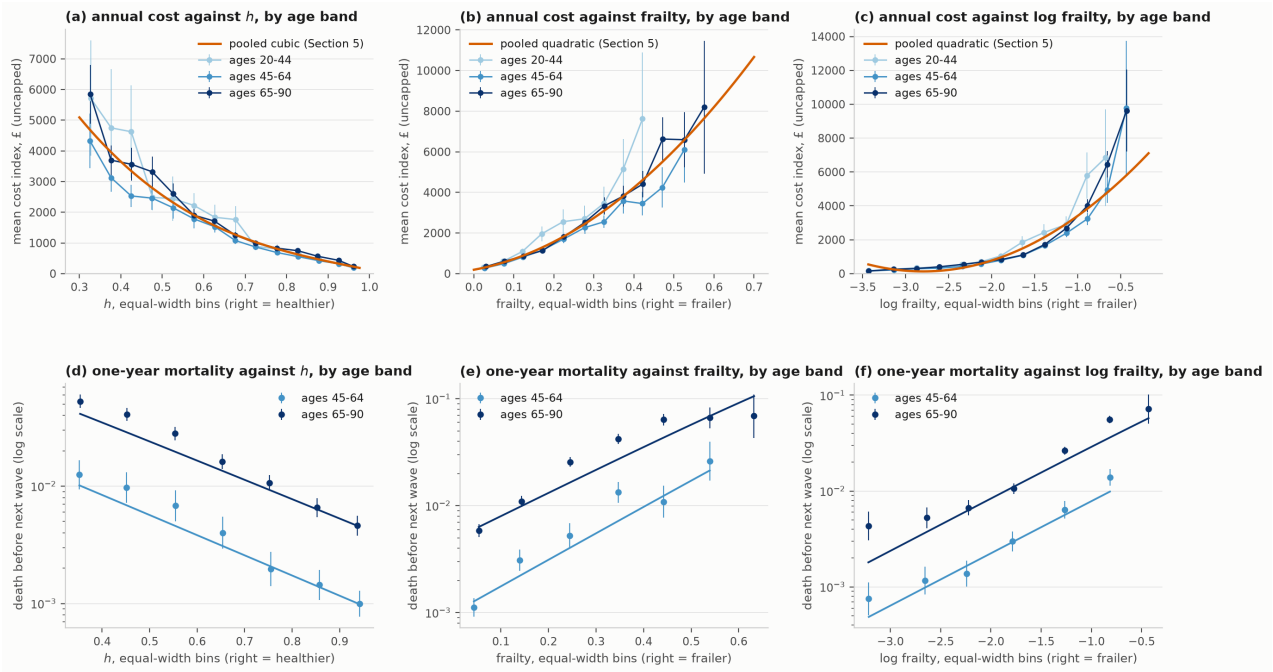


Figure I.1: As Figure 1, on h (left), the frailty index (middle) and log frailty (right). Top: mean annual cost in equal-width bins within three age bands; bottom: the one-year death rate on a log scale with the within-band logit. Higher is healthier on h and frailer on the two frailty indices.

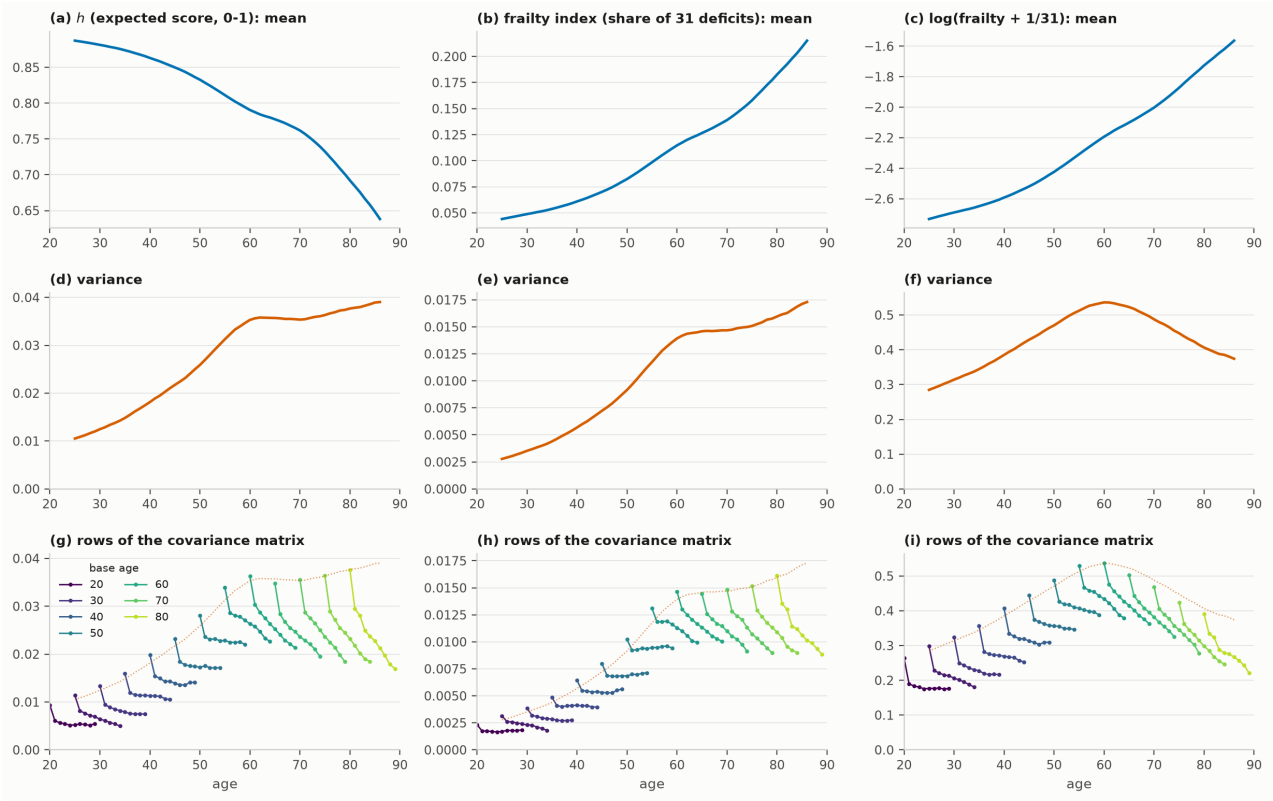


Figure I.2: As Figure 2, on h (left), the frailty index (middle) and log frailty (right): mean path, variance profile and covariance rows, model-free. Higher is healthier on h and frailer on the two frailty indices.

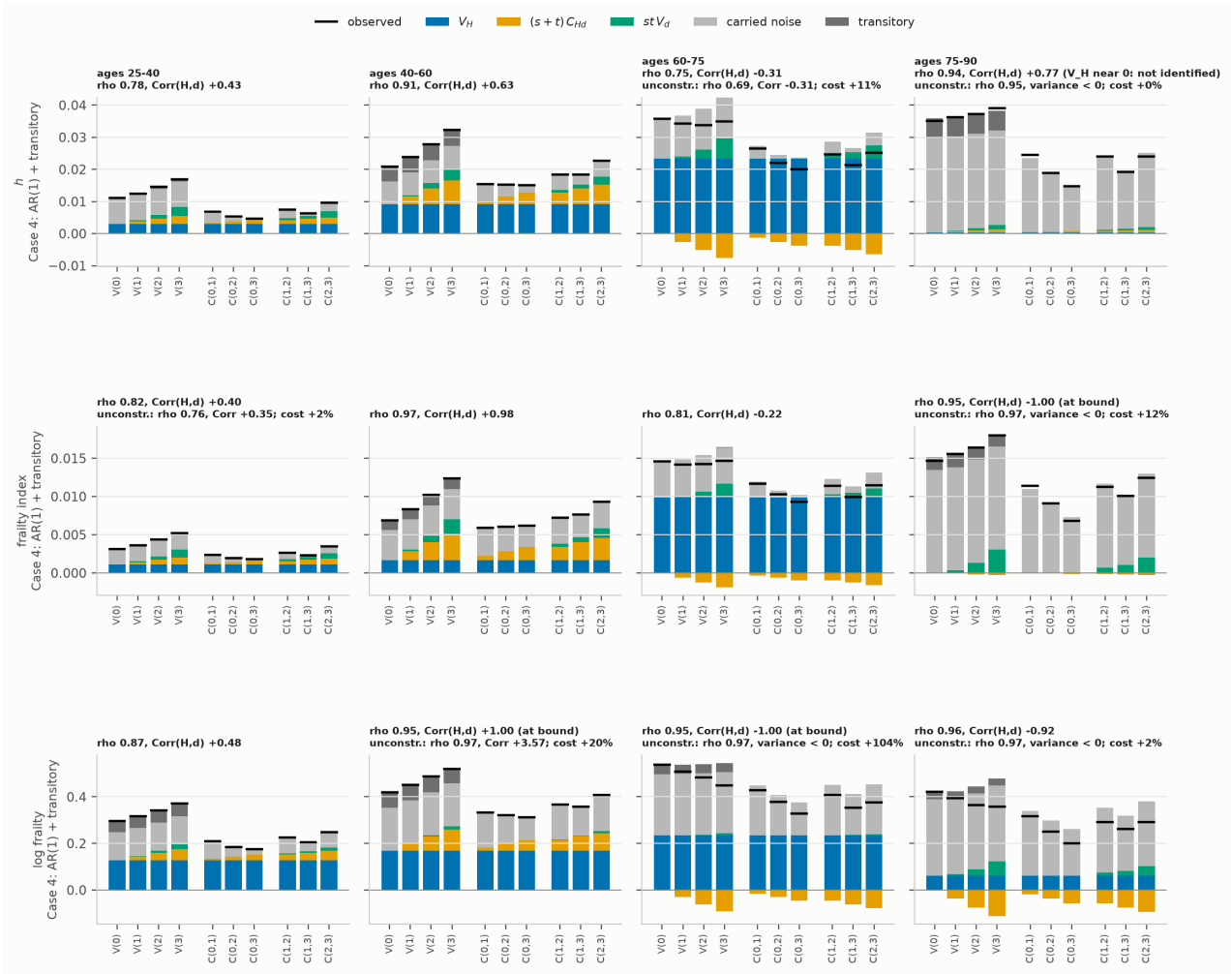


Figure I.3: As Figure 3, Case 4 on h (top), the frailty index (middle) and log frailty (bottom). Titles flag the bands where the admissibility constraint binds or V_H is below 5% of the first variance.

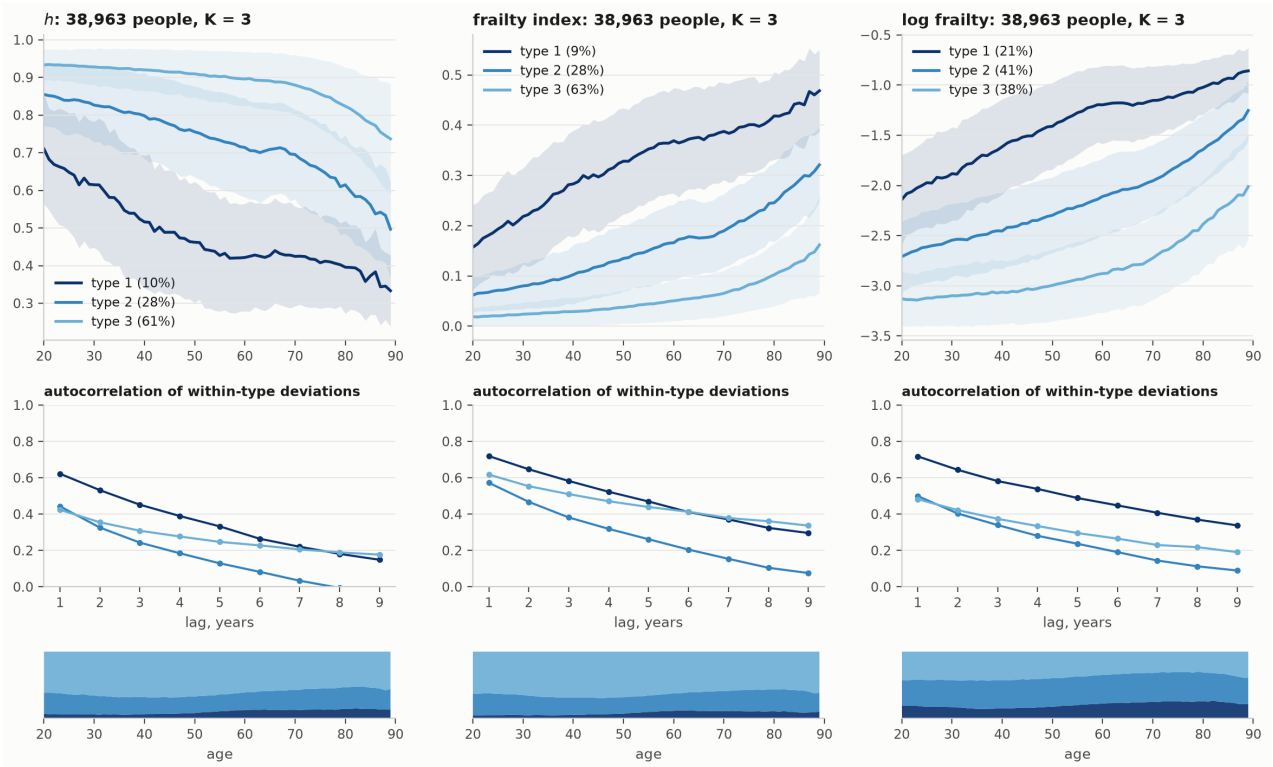


Figure I.4: As Figure 4, the partial K-means types on h (left), the frailty index (middle) and log frailty (right). Type 1 is worst health on every measure, so it is the top line on the two frailty indices.

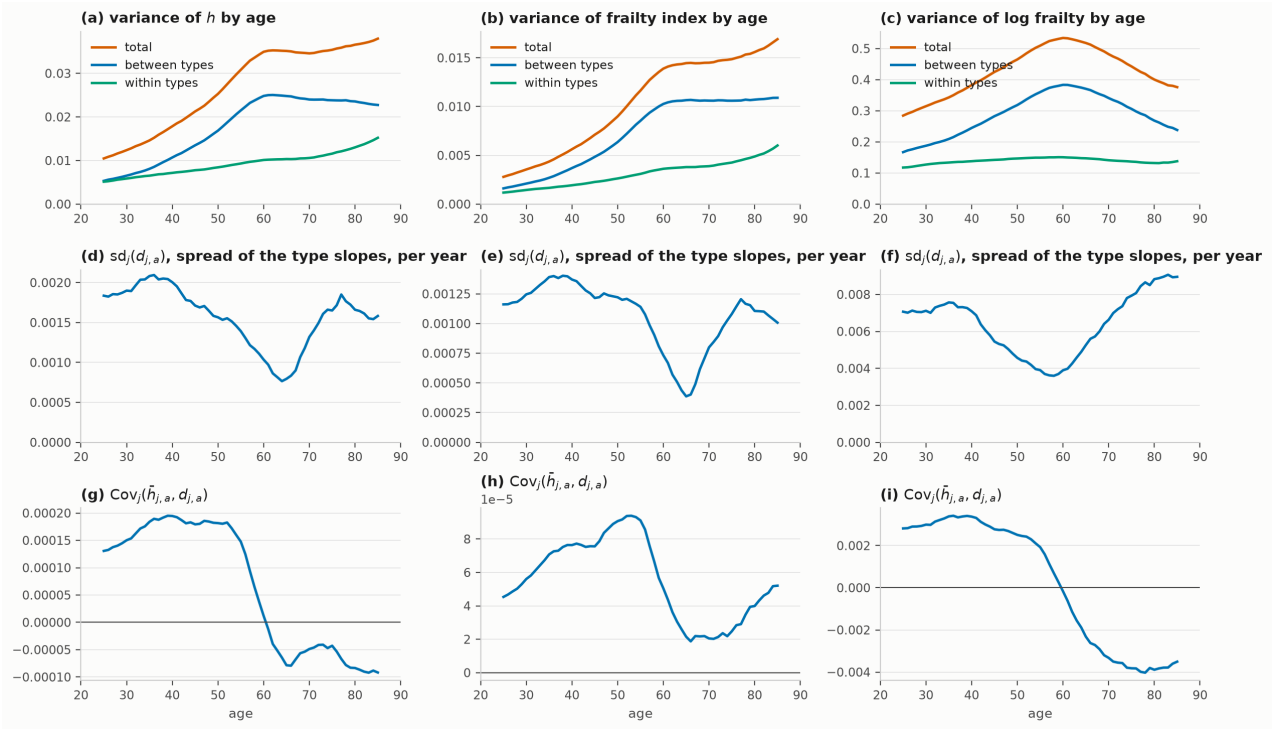


Figure I.5: As Figure 5, between versus within on h (left), the frailty index (middle) and log frailty (right).

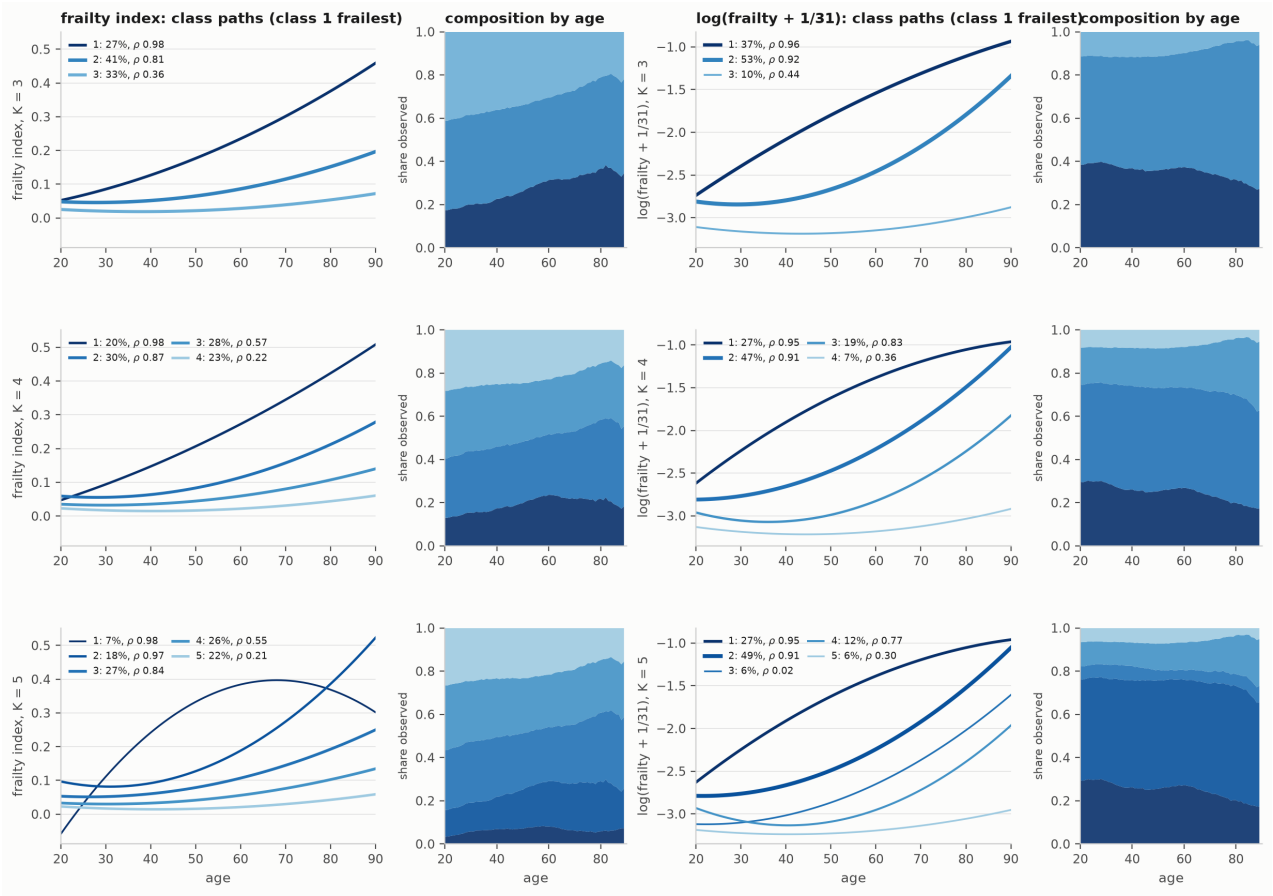


Figure I.6: The frailty index and log frailty under the persistent–transitory specification by number of classes: class paths and the posterior class composition by age at $K = 3$, 4 and 5, class 1 the frailest. On the frailty index $K = 4$ splits the $K = 3$ middle class into a steeper riser (30%, persistence 0.87) and a flatter one (28%, 0.57); the frailest class keeps its path and shrinks from 27% to 20%, and the healthiest stays at the floor with a persistence of 0.22. The frailest class's share of the person-waves peaks around 60 and falls after, the mortality selection the index sees. On log frailty $K = 4$ splits the healthy end instead: the frailest class shrinks from 37% to 27% and the middle from 53% to 47%, a new class (19%, persistence 0.83) rises late from near the floor, and the floor class falls from 10% to 7% (persistence 0.36). At $K = 5$ the frailty index splits its frailest class into a small, very frail one (7%, persistence 0.98) and a larger one (18%, 0.97), and keeps the other three close to $K = 4$ (27%, 26%, 22%; persistence 0.84, 0.55, 0.21). Log frailty at $K = 5$ keeps its frailest and middle classes (27% and 49%) and splits the late riser into a class with almost no persistence (5.5%, 0.02) and one at 0.77 (12%), the floor class at 6%. The frailty $K = 4$ fit is read over the three chains that share its mode; the log-frailty $K = 4$ fit has all four chains at one mode. Both $K = 5$ fits found three modes across their four chains and are read at the single chain with the highest log posterior (chain 1 on the frailty index, about 530 above the next; chain 4 on log frailty, about 30 above).

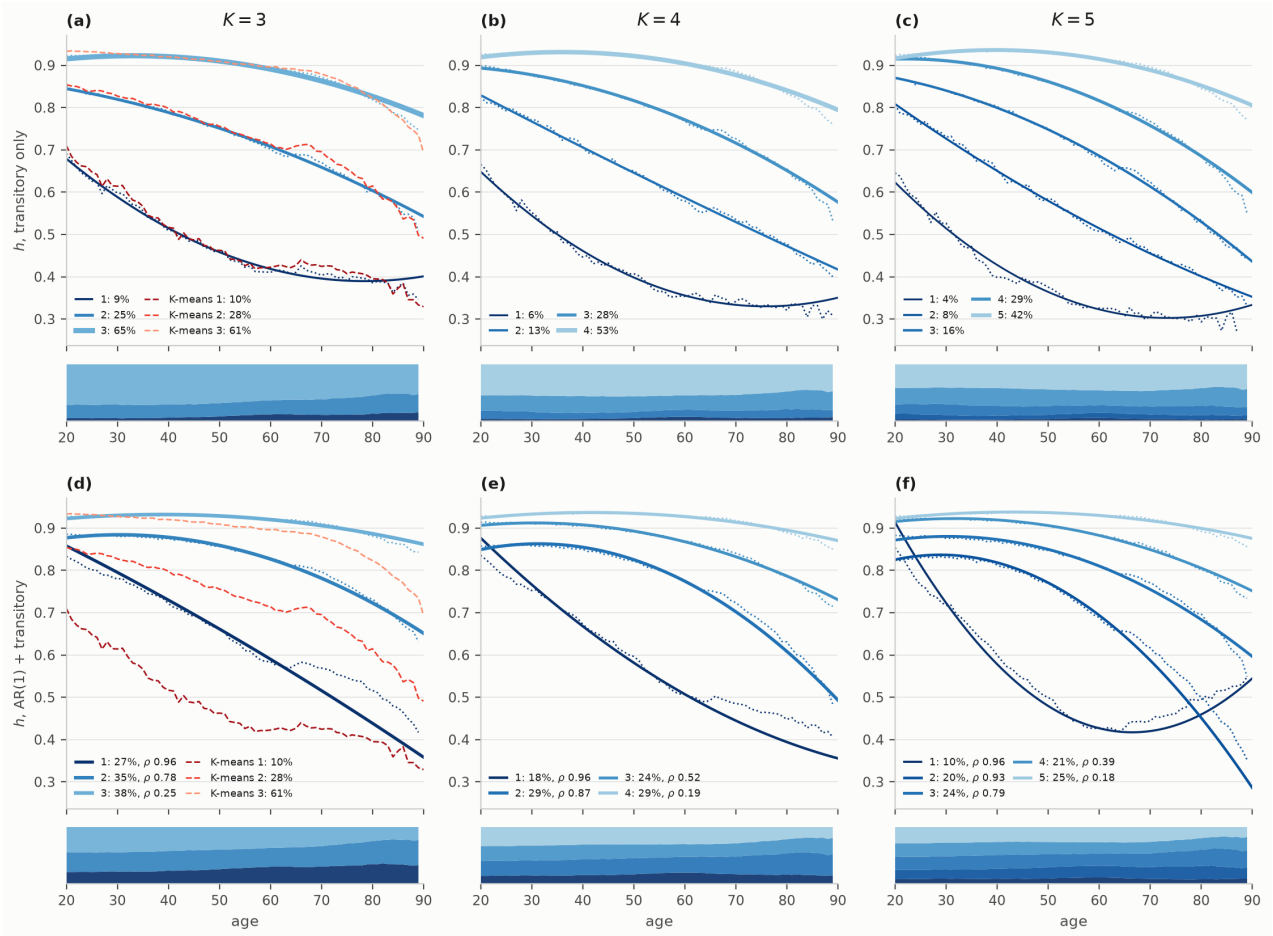


Figure I.7: As Figure 6, on h. Under the persistent–transitory specification at $K = 3$, (d), the healthiest class collapses to a persistence of 0.25. The $K = 5$ fit under the persistent–transitory specification is read over the three chains that share a mode; the fourth sat in a mode about 310 log-posterior units worse.

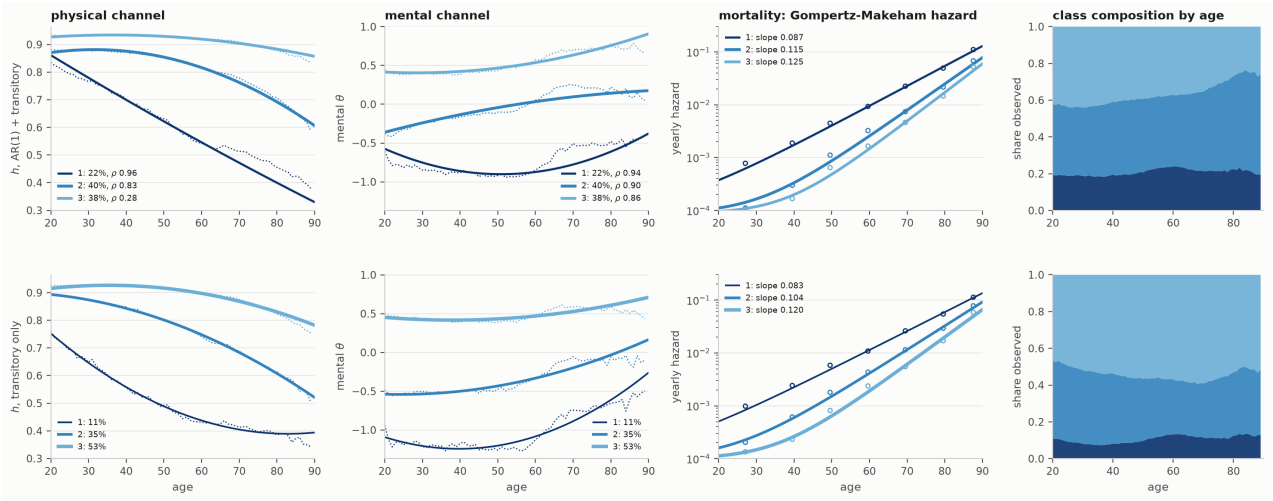


Figure I.8: As Figure 7, on h. Under the persistent–transitory specification one of four chains sat in the mode with the class 2 and 3 intercepts pressed together on the ordering constraint, about 2,950 log-posterior units below the other three; the fit is read over the three chains that agree (split $\hat{R}$ 1.003).

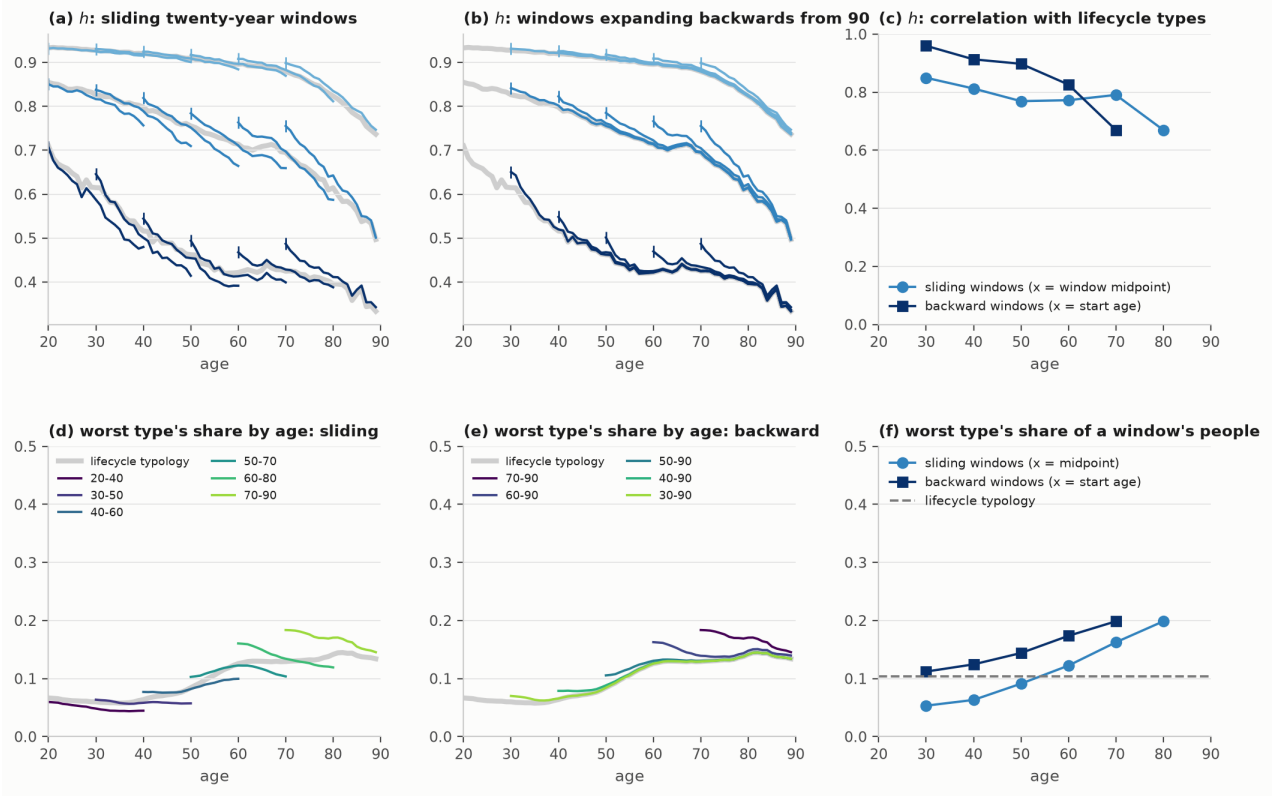


Figure I.9: As Figure E.2, the partial K-means types on h over sliding and backward-expanding windows.

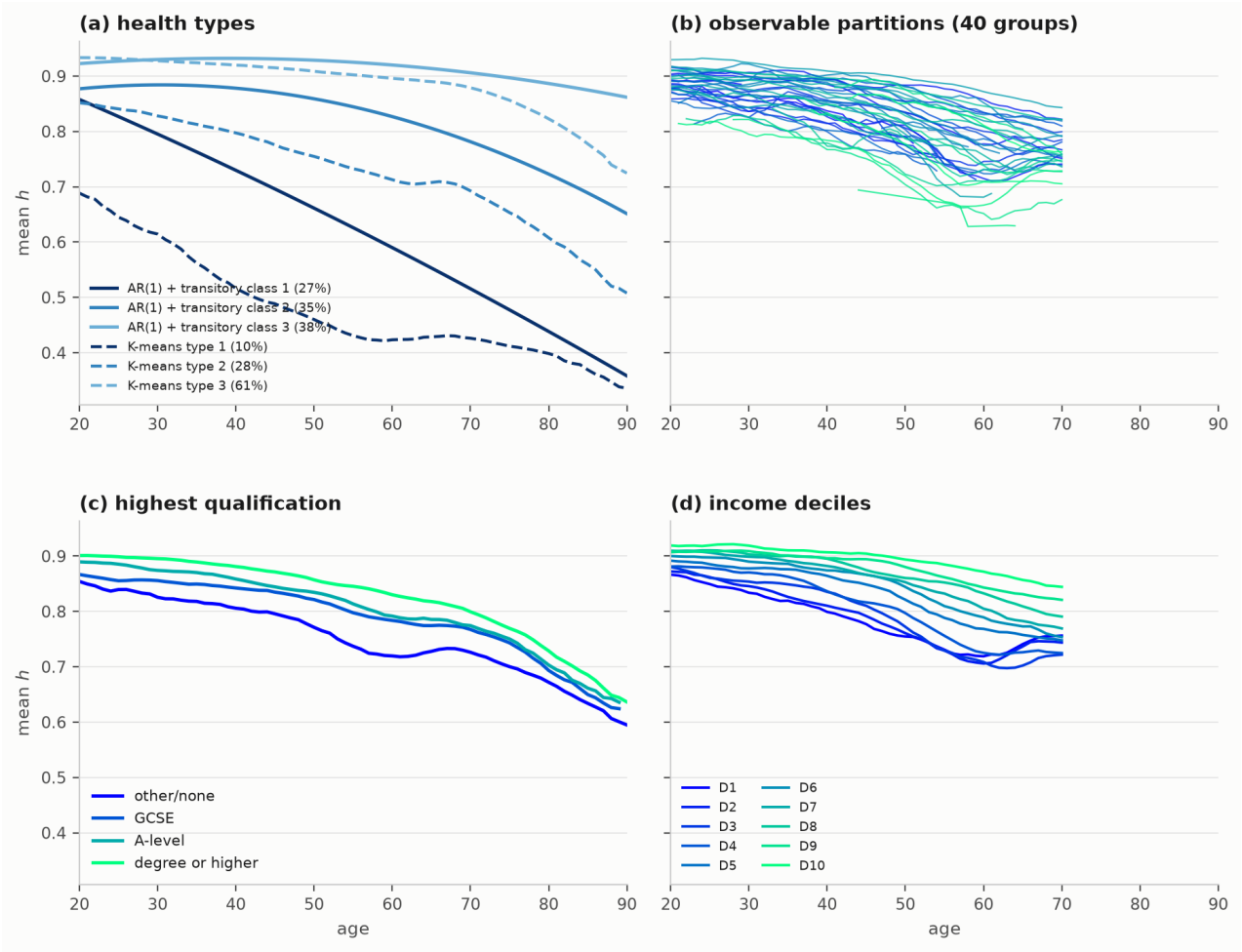


Figure I.10: As Figure 9, on h.

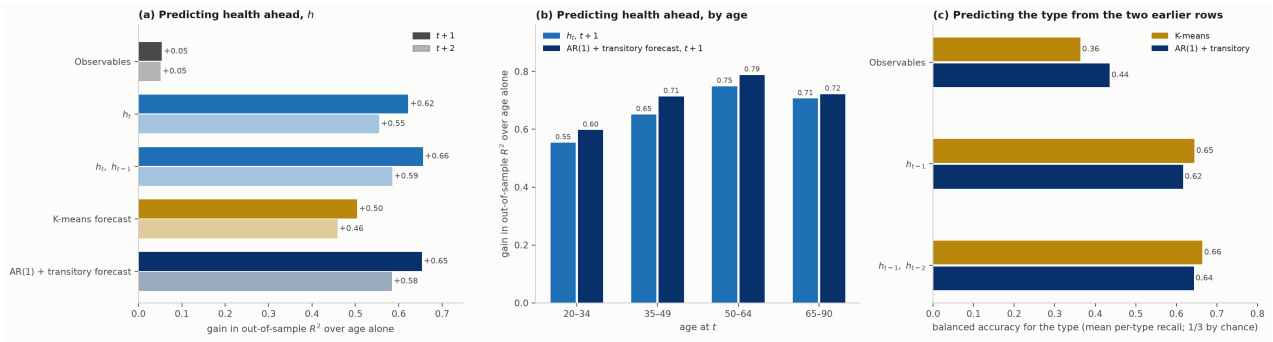


Figure I.11: As Figure 10, on h: (a) and (b) as its panels (a) and (c), (c) as its panel (b), for the single-channel typologies only (h is not fitted in the multidimensional model).

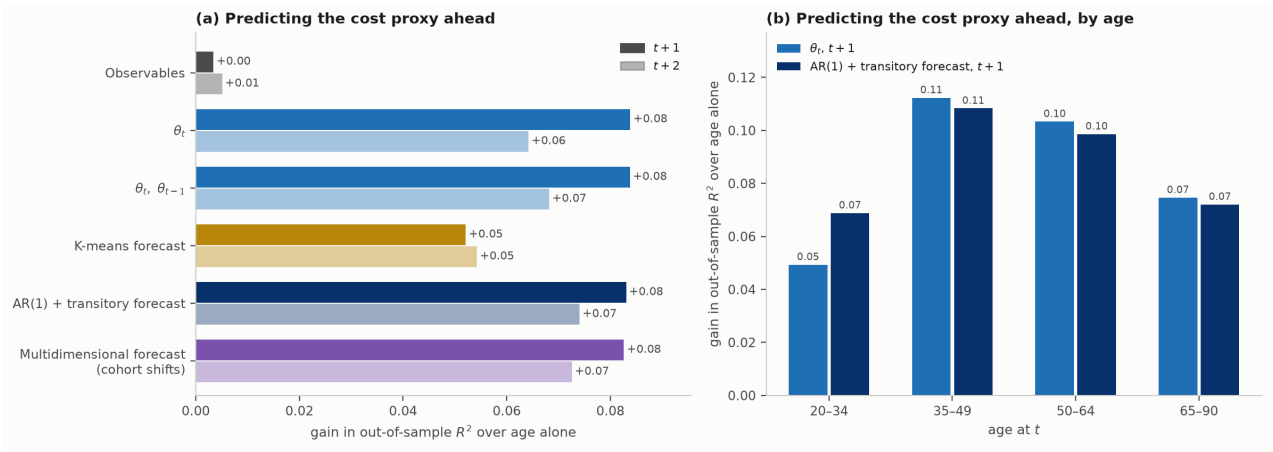


Figure I.12: As Figure 10(a) and (c), predicting the cost proxy: (a) one (full bars) and two (light bars) waves ahead (23,555 and 24,191 people); (b) one wave ahead within age bands at t.

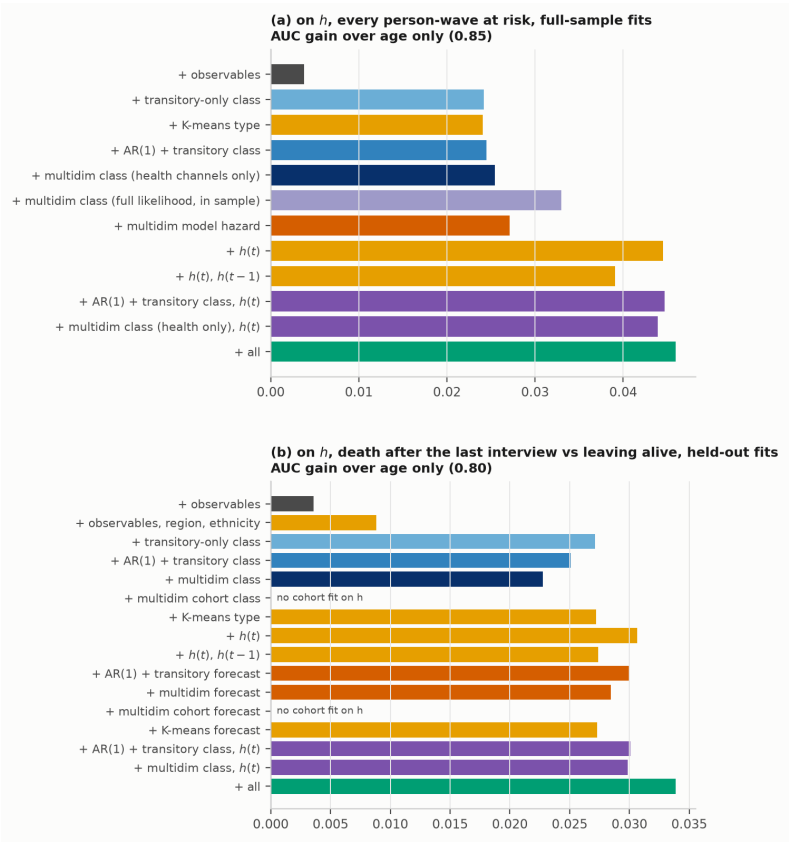


Figure I.13: Predicting death on h, AUC gain over an age-only quadratic. (a) Every person-wave at risk on the full-sample fits: death before the next wave; the single-channel classes never see the death record, and the multidimensional class is shown from the health channels only and from the full likelihood, which has seen the decedent's event. (b) The held-out fits: death after the last interview against leaving the panel alive, predicted from the last fitted row. The θ version of (a) is Figure F.9.

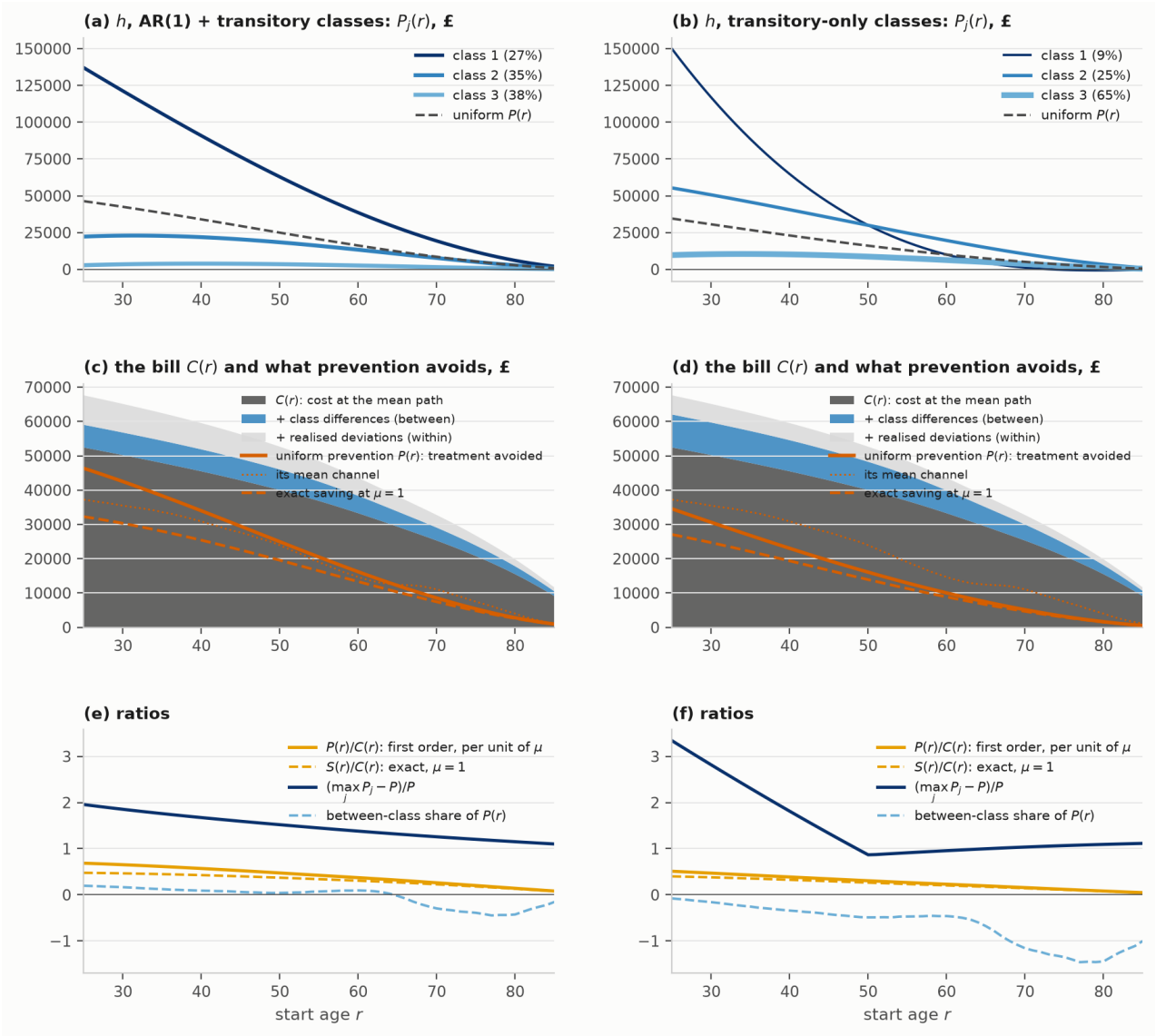


Figure I.14: As Figure G.2, on h, with the cost curve estimated on h.

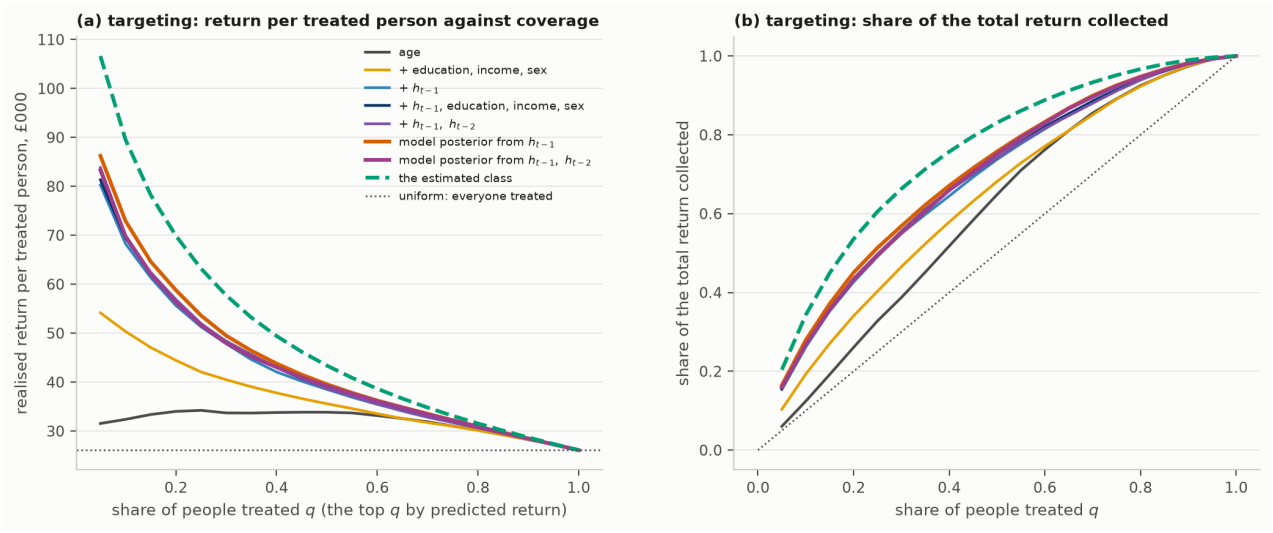


Figure I.15: As Figure G.3, on h, with the return by class priced on h (29,899 people, the first two rows held out).